UNIT 1: INTRODUCTION TO PSYCHOTHERAPY

PSYCHOTHERAPY

Psychotherapy is the general umbrella term for an enormous range of interventions, modalities, and integrative strategies employed in the service of improving quality of life and health.

The word psychotherapy means caring for another person's soul or being, as derived from the Greek word ***psyche*** meaning "soul" or "being" and ***therapeutikos*** meaning "caring for another" (Kleinke, 1994).

Anna O., an early psychoanalytic patient of Breuer, referred to the treatment she received as "***the talking cure***." This is an elegant, albeit vague description of psychotherapy.That is talking, expressing, verbalizing, or somehow sharing one's pain is, in and of itself, potentially healing.

However, this definition does not satisfy contemporary psychotherapy researchers, but it does provide an historical and foundational frame to consider.

There are many definitions of psychotherapy:

There are some favourite definitions of psychotherapy, some of which are imprecise, but others nearly reach the standard of scientific precision. These are:

1. "A conversation with a therapeutic purpose" (Korchin, 1976)
2. "The purchase of friendship" (Schofield, 1964)
3. "A situation in which two people interact and try to come to an understanding of one another, with the specific goal of accomplishing something beneficial for the complaining person (Bruch, 1981)
4. "When one person with an emotional disorder gets help from another person who has less of an emotional disorder (J. Watkins, 1983)
5. "Psychological treatment of emotional problems in which a trained person deliberately establishes a professional relationship with the patient in order to : (a) remove or modify or retard existing symptoms; (b) mediate disturbed patterns of behavior; (c) promote positive personality growth and development" (Wolberg, 1995).

CONTEMPORARY PSYCHOTHERAPY

Contemporary psychotherapy combines the synergistic power of a wealth of biological, psychological, and social resources to create a multidimensional and integrative treatment requiring a great deal of creativity, competence, and collaboration on the part of the clinician.

Definition : The treatment of mental and emotional disorders through the use of psychological techniques with the goal being relief of symptoms, changes in behavior leading to improved social and functioning, and personality growth.

A useful definition, developed by Norcross (1990), follows:

Psychotherapy is "the informed and intentional application of clinical methods and interpersonal stances derived from established psychological principles for the purpose of assisting people to modify their

behaviors, cognitions, emotions, and/or other personal characteristics in directions that the participants deem desirable" (p. 218).

While psychotherapy implies being treated by someone to cure or care for problems, counseling refers to providing advice and suggestions.

Psychotherapy is a unilateral professional relationship that is circumscribed by limits on time, frequency of contact, content of discussions, and level of intimacy in which a person may talk over problems with a specialist in human behavior.

Confidentiality is assured in a professional relationship, so that a client or patient can express whatever is on his or her mind without concern that the information will be disclosed to others. There are ethical and legal limits to confidentiality, including serious and immediate danger to self and others.

A professional relationship also must not be compromised by the development or maintenance of unethical dual relationships.

A psychologist will not be effective if he or she tries to be a friend, a colleague, or a lover. One of the differences between talking to a friend and talking to a psychologist is that the professional relationship is a primarily unilateral, or "oneway," relationship in which the problems or concerns of the patient or client are discussed, not the needs or concerns of the psychologist.

Thus, psychotherapy involves consulting with a mental health professional to obtain assistance in changing feelings, thoughts, or behaviors that one experiences as problematic and distressing.

Psychotherapy utilizes theories of human behavior as well as a carefully integrative plan to improve the psychological and behavioral functioning of a person or group.

An extensive review of all the research studies concludes that those who choose to receive psychotherapy from a mental health professional tend to benefit from the experience. Generally 70% to 80% of those treated in psychotherapy are improved (M. Smith et al., 1980).Most people who participate in a psychotherapy experience are satisfied with their treatment. Some psychologists are critical of psychotherapy, feeling that much more effort should be directed toward prevention rather than treatment of psychological problems.

Thus, psychotherapy has been described as including four major factors:

1. A relationship in which the client has confidence that the therapist is competent and cares about his or her welfare.
2. The expectancy or hope shared by the therapist and the client that benefits will result from the process.
3. An underlying conceptual framework that provides a rationale for understanding and a set of procedures designed to alleviate the client's distress and functional impairment that require the active participation of both client and therapist.
4. A practice setting that is socially defined and accepted as a place of healing.

Additional factors that influence the process and outcome include:

- Aspects of the client (e.g., nature of the problem, ability to understand and engage in the process, motivation for change, and resources outside of the psychotherapeutic relationship such as parental involvement and support); and

- Characteristics of the therapist (e.g., competence and personality features).

Psychotherapy is intended to enhance the client's mental health, flexibility, quality of relationships, and well-being.

This occurs through a process of development and change in cognition, perception, self-understanding, relational capacities, and overt behavior in the client that promotes a decrease in distress and an increase in adaptive functioning.For children and adolescents, collaborative work with families and schools are designed to promote and maintain complementary change as well.

How have historical and theoretical perspectives affected the development of psychotherapy?

All schools of psychotherapy are bound together by rationales that include theoretical formulations related to personality and psychopathology, and a set of principles and techniques to apply in practice.

The most influential schools have been based on four major theories of personality development and psychopathology:

1. Psychoanalytic theory (and its derivatives)
2. Behavioral theory (including behavior modification, social learning, and cognitive–behavioral)
3. Humanistic theory
4. Family systems theory

Psychoanalytic psychotherapy was the exclusive form of psychotherapy during the early part of the 20th century. Early psychoanalytic theories of treatment sought to enable the client to become aware of suppressed memories as well as the consequences that past experiences asserted upon the client's present psyche. In so doing, it would then be possible to work toward the client's realization of the connection between his or her unconscious past and the conscious reality he or she was presently experiencing.

Eventually, psychoanalytic psychotherapy evolved to place greater importance upon interpersonal and social determinants of personality and psychopathology. This shift was paralleled by an increased focus on the importance of the therapeutic relationship as an effective therapeutic agent. In the course of this shift, certain key concepts, such as transference and countertransference, were reconceptualized to reflect the impact of early relationships on current relational functioning.

Transference refers to the thoughts and/or feelings directed toward the therapist (e.g., child feels love toward the therapist). In countertransference the therapist projects his or her feelings upon this client (e.g., caring for the patient).

Within this approach, which is often referred to as psychodynamic or interpersonal psychotherapy, the therapist seeks to understand the impact of early relationships on current relational functioning. The therapist then uses the current therapeutic relationship to "correct" earlier relational disappointments and harm by providing a corrective emotional experience; that is, the therapist responds to the client in ways that do not confirm the client's expectations based on earlier non-supportive or hurtful relationships.

The client internalizes the qualities of the therapeutic relationship to further develop his or her relational schemes in ways that provide the client with more adaptive choices and counteract earlier established maladaptive patterns of relating and reacting.

Behavioral therapy became recognized in the 1950s as an alternative to traditional psychotherapy.The behavioral perspective focuses on the present rather than past conditions to find factors that are maintaining current problematic behavioral patterns.Behavioral therapy targets the child's overt behavior and emphasizes treatment in the context of family and school. In the 1970s, many with a behavioral perspective began to move away from a strictly stimulus–response model (e.g., stimulus or environmental conditions give rise to behavior) to one that gave weight to the cognitive processes as well.

Cognitive–behavioral therapy is based on a problem-solving approach that emphasizes changing an individual's dysfunctional thoughts, emotions, and behavior. This approach places an emphasis on analyzing current problems, defining concrete goals, and developing strategies so that the client can achieve specific, measurable changes.

When working with children and adolescents from a cognitive–behavioral perspective, it important to support them through their thought processes, guide them in identifying alternate solutions, encourage and reward them for effort, and aid them in implementing and practicing the skills needed to manage problems and interactions with others.

Humanistic psychotherapy moved into the forefront in the 1960s as a reaction to the clash between those who adhered to psychoanalytic theories and those who followed a behavioral orientation. The concept that most characterizes this approach is dedication to a phenomenological approach. This approach subscribes to humanity's unique ability to possess an awareness of one's own thoughts and to use this ability to interpret one's own world, and use this analysis toward achieving the realization of one's own independent self. Moreover, it focuses on the individual's perception of reality in order to understand behavior.

All the theories under this orientation emphasize the process of actualization and growth. There have been several subsets of the humanistic approach, among the most common being Gestalt psychotherapy and experiential psychotherapy. An integrative approach to psychotherapy has been embraced in the past 20 years. Such an approach advocates not being confined by a single paradigm; rather it promotes constructing a theoretical orientation that draws from multiple theoretical traditions.

The integrative movement has been validated by research findings indicating that psychotherapy from different major theoretical perspectives is equally effective, as well as meaningfully more effective than no treatment or placebo conditions. Some attribute the effects to factors shared across the schools of psychotherapy discussed in the description of psychotherapy presented earlier.

GOALS OF PSYCHOTHERAPY

The list of possible goals of psychotherapy is endless.

Goals may include behavioral change, enhanced interpersonal relationships, insight, support, or concrete outcomes such as finding a job or a partner, staying out of the hospital, or staying alive. The goals may also be different for the patient, for the significant others in the patient's life (e.g., spouse, parents, coworkers, teacher), and for the psychologist.Whereas the patient may have one goal (e.g., reduction of anxiety), the psychologist may have another (e.g., increased insight), and the patient's spouse may have yet a different goal (e.g., increased independence and responsibility for the patient).

Kleinke (1994) described six fundamental therapeutic goals common to almost all psychotherapies:

- ✓ overcoming demoralization and gaining hope: ,
- ✓ enhancing mastery and self-efficacy,
- ✓ overcoming avoidance,
- ✓ becoming aware of one's misconceptions,
- ✓ accepting life's realities, and
- ✓ achieving insight.

Goals and objectives of therapy

Informed consent

What are the goals of psychotherapy, and who decides? If we apply the traditional medical ethical principles of beneficence and nonmaleficence, then psychotherapy, like any other medical treatment, should aim to help the patients with their problems, make them feel better and do them no harm. Most psychotherapists pursue this aim by working with patients to increase their capacity for self-reflection, and to help them become more aware of the links between their feelings and actions.

But this is not as simple as it seems. Although CBT can make people feel better in the short term by removing their symptoms, psychodynamic therapies may not do so. For example, patients with histories of exposure to traumatic events (whether in childhood or in adulthood) may want the therapy to take their horrible feelings away. However, psychotherapists do not take patientsâ€™ feelings and memories away, but try and help them to deal with them better. Both CBT and psychodynamic therapy seek to help regulate conscious negative feelings, and modify distorted and dysfunctional meanings of memories. Although these are perfectly reasonable goals for psychotherapists, they may not be what the patient sees as the goal of therapy. Patients may not accept that the meanings attributed to memory are distorted; they may also be unaware of the extent of their negative feelings, especially guilt, shame, and hatred. In CBT, the patient may want to remove all their negative emotion, while the therapist wants to decrease excess emotion.

There are practical techniques that all therapists employ to address these issues clinically. Ethically, however, there may still be uncertainty or conflict

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about what the goals of therapy are, and who should set them, which can make the issue of informed consent in psychotherapy particularly complex. The ethical principle that underpins the requirement for consent is respect for autonomy: patients should be free to choose or refuse for themselves what treatment they have. Legally, for consent to be valid it must be given voluntarily, by a patient who is competent to make that decision, and the patient must be adequately informed about what the treatment involves, including any possible negative side-effects. However, it may be difficult for therapists to advise patients exactly to what it is they are consenting. Therapists may not be able to predict what patients will experience during the course of therapy, or what the outcome will be. Should therapists inform patients about unconscious transference enactments (both positive and negative), and get specific consent for this possible â€˜side-effectâ€™? (Holmes and Lindley, 1989). My own view is that therapists need to warn patients that the process of psychotherapy is not always comfortable, and that many patients feel worse before they feel better (although this may not be true for CBT).

It must also be pointed out that some people do not get better with psychotherapy (Lambert and Bergin, 1994). Successful psychotherapy matches the right therapy and right therapist to the patient, which is why therapists need to develop good quality assessment skills, not just for their own discipline but for others. This means

being able to formulate problems in different ways, and think about the ways that this particular patient is most likely to make progress.

Autonomy and consent to therapy

For consent to be valid it must be given by competent patients, who are capable of exercising their autonomy in a way that expresses their values and interests, while giving weight to conflicting values and facts. Although in a general sense, most psychotherapy patients will be competent to make decisions about their own treatment, this may not be true for all. For some it is problems with the exercise of autonomy that have led them to seek therapy; for others, their psychological distress may affect their capacity to make choices about treatment. Obvious examples are those people who have very recently suffered psychological trauma or bereavement, or those who are experiencing psychotic symptoms.

There are other groups of patients who may lack competence to make treatment decisions, such as children, or patients with psychotic disorders or learning disabilities. The problem is usually not a global lack of competence, but rather that patients experience fluctuating levels of capacity, or experience rapid changes in how they make decisions. For example, it is hard to know what to make of a child's refusal to have therapy, which they agreed to only a day before. Can this be understood as an informed choice to refuse treatment? Or is this refusal merely evidence that the work has begun? There are also groups of patients are â€˜coercedâ€™ into therapy, such as children who only agree under pressure from family.

It may also be much harder to know how to think through issues of informed consent for family, marital, or group psychotherapy (Lakin, 1988). Children, or other family members may only â€˜agreeâ€™ to therapy because of pressure from others in their emotional network. Although such pressures are an essential part of intimate relationships, they raise dilemmas about the nature of â€˜trueâ€™ voluntariness. Voluntariness is also an obvious issue in the domain of forensic psychotherapy, as some patients may be mandated to have treatment, as an alternative to prison, or participation in therapy is expected as part of their detention. Clinically, most therapists in forensic or penal settings get to grips with this issue as part of the therapy from the start; but it does not sit easily with classical medical ethical accounts of informed consent.

Confidentiality and consent to disclosure

Clearly, negligence and incompetence are potential risks for all clinicians, and these can (and should) be addressed by training, licensing or registration, and supervision processes. But other types of harm are also possible in therapy, which are not so common in other types of medicine: for example, threats to confidentiality and boundary violations. I will deal with boundary violations in more detail below, but at this point it is relevant to consider the issue of breaches of confidentiality, or the boundary of therapeutic privacy, as a type of harm.

My own view is that the principle of confidentiality may be better understood as the principle of informed disclosure. Therapists are under an ethical duty to obtain their patient's consent before they disclose details of their psychotherapeutic treatment to anyone; including those close to the patient. Although this is undoubtedly a principle that is respectful of the patient's autonomy, it can be problematic when the patient discloses material, which indicates that someone else is (or has been) at risk of harm from them. Most professional codes and guidelines address this issue, and there is relevant case law (see below). In general psychotherapeutic practice, this is probably a rare event; more commonly dilemmas arise when the therapist perceives that it might be helpful to the patient for others to know that a patient is in therapy, and what has been discussed. Of course, the therapist can seek consent from the patient to disclose, but if the patient refuses consent, then the therapist may still face a dilemma. A nonpsychotherapeutic example occurs when an HIV-positive patient refuses to tell his or her partner (with whom they are still having a sexual relationship), and refuses to let the clinician inform the partner. In the UK, professionals are advised that it may be justified to breach confidentiality in the face of a competent refusal.

Clinicians may also come under pressure from others (family members or employers) who may contact them to discuss the patient, or to seek information about the therapy. Again, it may usually be possible for the therapist to seek consent to discuss some agreed upon material with others; ethical problems may arise when the third party asks the therapist not to tell the patient about the contact, usually because it will cause distress to the patient. In such circumstances, the therapist may have to balance the patient's claim to honesty and confidentiality against a possible harm to them.

Consent to use of patient information for research

Getting consent to any form of disclosure can be intrusive into the process of therapy, and this is a particular issue in relation to research. The ethical dilemma here is about whether therapists can use patient material without consent for teaching and research purposes. Traditionally, this has not been an issue for psychotherapists who have assumed some ownership over their experience in the therapeutic space, and who have presumably also assumed that the breach of confidentiality is justified for the public good that arises from teaching and research. The good consequences justification seems plausible enough; clinical material is essential for teaching trainees, and for research. What is different from 20 years ago is the increased social emphasis on respect for individual patient autonomy, in the form of control and ownership over anything personal, which means that therapists may be

unwise to assume that it is ethically unproblematic to use patient material for teaching and research without consent, even if it is disguised.

Although some journals do not require patient's consent to publication of their details, there are others that require that the patient has not only given consent, but has read the article in which their case is mentioned or described. Getting consent after therapy has ended is not necessarily the answer, as this might be just as intrusive or distressing for the patient (Winship, 2002). Then there is the question of content. Do patients have to agree with what is written? Can they disagree only with matters of fact, rather than opinion? And to what extent is the therapist allowed to own his or her own view of the therapy, and discuss it without the patient's permission?

The issue is further complicated because the therapist's capacity to have a personal and intimate relationship with the patient is part of the therapeutic process (Klauber, 1986). The therapist makes their mind available to the patient to assist them; and this involves taking seriously their own feelings and thoughts about the patient. Keeping a record is a way to do this; and also helps with the process of thinking and supervision. However, generally speaking, the therapist does not share her thoughts and feelings about the patient with the patient (the extent to which she might do this is a matter of technique, reflection, and supervision). It is unlikely that the patient will find it helpful to discover what their therapist thinks about them from simply reading the process notes. My own practice is to make a brief note in the medical record that the session has taken place, and then keep some brief notes of the main themes of the session in a file in my office. The American Psychiatric Association recommends this practice to psychotherapists, and this would be consistent with general advice on good record keeping from the UK Royal College of Psychiatrists. Legal jurisdictions, both in the UK and in the US, do, however, make it theoretically possible for the patient to have access to those notes, unless this access would constitute harm to them or another person. The ethical issue here is that therapists cannot assume that they own the notes of their meetings, and that only their views about the process notes need be consulted.

In CBT, it is common practice for the therapist to record the patient's experiences in therapy (thoughts, feelings, behaviors), in the same way as the patient does in homework assignments. For both therapist and

patient, sharing of these records and notes is often therapeutically helpful. What may be problematic is when the patient is recording information that may be misunderstood out of context (for example, in cases of sexual dysfunction, or violence). CBT therapists may also wish to record impressions of the patient and his/her progress that they do not wish the patient to see; and to which the patient may have legal access.

Clinical legal issues in psychotherapy

There are over 450 schools of psychotherapy currently in existence (Simon, 2001, p. 90). New schools continue to emerge. Amidst this diversity, basic clinicalâ€“legal principles generally apply to most, if not all, forms of psychotherapy. Psychotherapists need to have a working knowledge and comfort managing clinicalâ€“legal issues arising in psychotherapy to avoid maladaptive defensive practices that may interfere with treatment. Moreover, legal issues can often be turned to the benefit of the patient's therapy.
It is said that psychotherapy is an â€˜impossible taskâ€™, sometimes complic-ated by litigation (Simon, 1991). In recent years, negligence claims against psychotherapist have substantially increased. Table 41.1 lists typical malpractice claims filed against psychotherapist.

Confidentiality

Patients have the right and psychotherapists the duty to have spoken or written communications during the course of treatment kept confidential. In the US, there is no legal obligation requiring therapists to provide information, even to law enforcement officials, absent statutory disclosure requirements, or judicial compulsion (Simon, 2001, p. 90).
The therapist's duty to safeguard confidentiality arises from four sources:

- The ethical codes of the mental health professions.
- States recognize the right of confidentiality through provisions in professional licensure regulations or in confidentiality and privilege statues.
- The common law has long recognized an attorney-client privilege. Developing case law has carved out similar protection for psychotherapists, although not as stringent.
- The right of confidentiality derives from various constitutional guarantees. An explicit constitutional right of privacy does not exist.

In Jaffe v. Redmond (1996), the US Supreme Court ruled that communication between the psychotherapist and patient are confidential. The psychotherapist is not required to disclose them in federal trials. The decision does not, however, apply to state courts where most psychotherapistâ€“patient confidentiality matters take place.

Informed consent

As psychotherapy is a â€˜talkingâ€™ treatment, is informed consent necessary? The answer is yes, because all psychotherapies have risks and benefits that patients need to understand. Few therapists warn patients of the risks of a proposed psychotherapy, although the potential benefits may be emphasized. Untoward transference reactions, mismanagement of transference and countertransference, regressive dependency states, and general worsening of a patient's clinical condition are some of the risks of psychotherapy.
Informed consent is important in psychotherapy. Through increased participation of patientâ€”consumers in treatment decisions, the potential for the use of harmful treatments is lessened. Therapists must be prepared to consider thoughtfully the risks and benefits of any treatment they recommend to patients. Informed consent doctrine provides a basis for legal recovery of compensation for patients who are harmed by failures of

therapists to obtain informed consent. Some states that have informed consent statutes declare the failure to obtain informed consent to be negligence (Slovenko, 1989).

Ethics and psychotherapy

The term ethics can be defined in many ways. In the context of mental health care, any discussion about an ethical dilemma involves a special type of dialog; the discourse of ought and should in interpersonal relationships. Thus, when we talk about ethics in psychotherapy, we are talking about how therapists should behave in clinical practice with patients, and what the therapist ought to do in difficult interpersonal situations with patients and colleagues.

There is arguably a close relationship between ethics and psychotherapy, because just as ethical debate is all about how individuals should treat other people, and how we should act in relationship to each other, so psychotherapy explores interpersonal relating; how patients actually do treat other people, and relate to them. Additionally, all psychotherapeutic processes, regardless of school, utilize the therapeutic relationship between patient and therapist in some way, in order to understand and address the patient's problems.

This chapter is written largely from the perspective of psychoanalytic psychotherapy, with some additional reference to cognitive-behavioral therapy (CBT). But whatever the school of therapy, ethical dilemmas will arise that the therapist will have to reason and think about, and resolve; Tjelveit (1999) suggests that there are at least 14 different types of ethical reasoning that may be used in ethical dilemmas in psychotherapy (Box 40.1). A key theme here is that ethical dilemmas in psychotherapy have to be resolved, one way or the other; the overarching ethical duty of the therapist is to make the best quality decision that can be made.

Fourteen approaches to ethical reasoning (Tjvelveit, 1999)

- Casuistry: emphasize the specific circumstances
- Classic liberal individualism: emphasize autonomy and justice as ideals
- Communitarianism: emphasizes the interests of society
- Critical psychology: challenges psychology's claim to be ethically neutral
- Feminist ethics: the ethical character of therapy as gendered
- Hermeneutics: emphasizes interpretation, not explanation
- Narrative: understanding the right through stories
- Naturalistic: putting science and ethics together
- Pragmatic: emphasizes practical consequences
- Radical: the values of the left should be adopted
- Rational: ethics based on reason
- Religious: linked with varieties of religious tradition
- Romantic: linking ethics with the idea of a natural self
- Virtue ethics: understanding the character of the ethical actor.

Some guidance about the ethical duties of therapists can be found in the codes and guidelines that professional bodies hold to define their professional identity. Further guidance can be found in legal statutes and cases that have examined ethical dilemmas in psychotherapy. In this chapter, I will discuss some important legal cases from both US and English jurisdictions, because these give an indication of how the courts resolve ethical dilemmas. However, these cases should not be understood as legal advice (as the law is always subject to

interpretation and review); nor does the law always provide an ethically justifiable source of guidance, as is clear if we remember the impact of both the Nazi and South African race laws.
Legal and professional advice may not provide all the answers, and therapists will still have to do some ethical reasoning for themselves. Specifically, therapists are likely to face dilemmas relating to:

- goals and objectives of therapy
- the boundaries between their different identities
- the social and political frameworks in which they work.

Most of the classical ethical dilemmas can be understood in these three domains.

Boundaries in psychotherapy
Perhaps one of the few ethical precepts that all trainees learn at medical school is that the Hippocratic Oath forbids doctors from sexual relationships with their patients. However, there is rarely any accompanying discussion about why, or what this proscription represents. This part of the Oath, however, is perhaps the first recorded acknowledgment that the doctor who is working with a patient as a professional cannot also be that patient's lover; that there is a boundary between the two identities that should not be crossed. Such a boundary applies to all professional carers, and not just physicians.

Boundary crossings and violations
Crossings (may or may not be intended or initiated by either party)

- time keeping: lateness, earliness, alteration, or cancellation of sessions without notice
- self-disclosure (verbal and nonverbal)
- discussions of patient material with others, even with consent
- arguments or jokes with patients
- accidental/unexpected contact outside sessions (common in institutional settings)
- any physical contact

Violations (intended and initiated by the therapist)

- abrupt termination of therapy by therapist without warning
- excessive self-disclosure; especially of therapist's distress or anxiety
- prolonged or repeated angry outbursts with patient
- speaking or responding in ways, which humiliate or demean
- coercive behavior (verbal or nonverbal, including financial)
- financial exploitation
- planned contact outside therapeutic setting
- all physical contact that is prolonged or repeated
- any sexual or erotic contact between therapist and patient
- negligent therapy

Social and political frameworks
Social and cultural issues in psychotherapy

Just like any other form of medical treatment, psychotherapy is not practiced in a social and legal vacuum. One of the enduring myths of medicine is that practitioners use their skills and knowledge in a value-free way, for no other purpose but the patient's good. However, there is good reason to think that medical diagnosis and treatment are value-laden processes from start to finish, and mental health is no exception (Fulford, 1989). The values of the social groups and cultures to which the therapist and patient belong will infuse the dialogue that takes place in therapy. Tensions may particularly emerge in relation to ethnicity and racial identity, gender of either therapist or patient and religious or political beliefs.

Legal frameworks for therapy

The legal framework in which both patient and therapist function is also relevant here. Psychotherapeutic relationships are not outside the law, either civil or criminal. Existing statutes and case law that apply to medical treatments will also apply to therapy. Thus, psychotherapists acquire a legal duty of care when they work with a patient, and that duty requires them to practice in accordance with a reasonable body of medical (psychotherapeutic) opinion. This means that they must exercise reasonable professional skills, make logical treatment decisions, conform to professional ethical guidance where it exists, gain informed consent to treatment, and tell the truth when asked. They also have to abide by the laws of the land; so as described above, psychotherapists in the USA are mandated to report suspected child abuse, even when the information comes from their patient, who may be the abuser. In such circumstances, the therapist may feel a conflict between respect for the law, and the avoidance of harm to children, and concerns that disclosure that lead to harm to the patient.

UNIT 3 – INTERVIEWING

Clinical interviewing has been defined in many different ways. Some prefer a narrow, straightforward definition: An interview is a controlled situation in which one person, the interviewer, asks a series of questions of another person, the respondent. (Keats, 2000, p. 1).

An interview is an interaction between at least two persons. Each participant contributes to the process, and each influences the responses of the other. However, this characterization falls short of defining the process. Ordinary conversation is interactional, but surely interviewing goes beyond that. (Trull & Phares, 2001).

Table 3.1. Summary of Nondirective Listening Responses and Their Usual Effects

Nondirective Listening Response	Description	Primary Intent/Effect
Attending behavior	Eye contact, leaning forward, head nods, facial expressions, etc.	Facilitates or inhibits spontaneous client talk.
Silence	Absence of verbal activity.	Places pressure on clients to talk. Allows "cooling off" time. Allows interviewer to consider next response.
Clarification	Attempted restating of a client's message, preceded or followed by a closed question (e.g., "Do I have that right?").	Clarifies unclear client statements and verifies the accuracy of what the interviewer heard.
Paraphrase	Reflection or rephrasing of the content of what the client said.	Assures clients you hear them accurately and allows them to hear what they said.
Sensory-based paraphrase	Paraphrase that uses the client's clearly expressed sensory modalities.	Enhances rapport and empathy.
Nondirective reflection of feeling	Restatement or rephrasing of clearly stated emotion.	Enhances clients' experience of empathy and encourages their further emotional expression.
Summarization	Brief review of several topics covered during a session.	Enhances recall of session content and ties together or integrates themes covered in a session.

Table 3.2. Summary of Directive Listening Responses and Their Usual Effects

Directive Listening Response	Description	Primary Intent/Effect
Interpretive reflection of feeling	Statement indicating what feelings the interviewer believes are underlying the client's thoughts or actions.	May enhance empathy and encourage emotional exploration and insight.
Interpretation	Statement indicating what meaning the interviewer believes a client's emotions, thoughts, or actions represent. Often includes references to past experiences.	Encourages reflection and self-observation of clients' emotions, thoughts, and actions. Promotes client insight.
Question	Query that directly elicits information from a client. There are many forms of questions.	Elicits information. Enhances interviewer control. May help clients talk or encourage them to reflect on something.
Feeling validation	Statement that supports, affirms, approves of, or validates feelings articulated by clients.	Enhances rapport. Temporarily reduces anxiety. May cause the interviewer to be viewed as an expert.
Confrontation	Statement that points out or identifies a client incongruity or discrepancy. Ranges from very gentle to very harsh.	Encourages clients to examine themselves and their patterns of thinking, feeling, and behaving. May result in personal change and development.

Types of Questions

Interviewers have many types of questions at their disposal. It is important to differentiate among them because different types of questions tend to produce different client responses and response patterns. The most common questions used by interviewers are open, closed, swing, indirect or implied, and projective.

Open Questions

Open questions are designed to facilitate verbal output. By definition, open questions require more than a single-word response; they cannot be answered with a simple yes or no. Ordinarily, open questions begin with the word *How* or *What.* Writers sometimes classify questions that begin with *Where, When, Why,* and/or *Who* as open questions, but such questions are really only partially open because they don't facilitate talk nearly as well as *How* and *What* questions (Cormier & Nurius, 2003; Hutchins & Cole, 1997).

Closed Questions

Closed questions can be answered with a yes or no response (Hutchins & Cole, 1997). Although some people classify them as open, questions that begin with *Who, Where,* or *When* direct clients toward very specific information; therefore, we believe they generally should be considered closed questions (see Putting It in Practice 4.1). Closed questions restrict verbalization and lead clients toward more specific responses than open questions. They can serve as a technique for reducing or controlling how much clients talk. Restricting verbal output is useful when interviewing clients who are excessively talkative. Also, getting clients to

describe their experiences in a particular way can be helpful when conducting diagnostic interviews (e.g., in the preceding example about a panic attack on the New York subway, a diagnostic interviewer may ask, "Did you feel lightheaded or dizzy?" to confirm or disconfirm the presence of symptoms associated with panic attacks).

Table 4.1. Question Classification

Word Question Begins With	Type of Question	Usual Client Responses
What	Open	Factual and descriptive information
How	Open	Process or sequential information
Why	Partially open	Explanations and defensiveness
Where	Minimally open	Information pertaining to location
When	Minimally open	Information pertaining to time
Who	Minimally open	Information pertaining to a person
Do/Did	Closed	Specific information
Could/Would/Can/Will	Swing	Diverse info, sometimes rejected
I wonder/You must	Indirect	Exploration of thoughts and feelings
What if	Projective	Information on judgment and values

ATTENDING BEHAVIOR

A. Ivey and Ivey (1999) consider *attending behavior* the foundation of interviewing. They define attending behavior as "culturally and individually appropriate . . . eye contact, body language, vocal qualities, and verbal tracking" (p. 15). To succeed, interviewers must pay attention to their clients in culturally and individually appropriate ways. If interviewers fail to look, sound, and act attentive, they will not have many clients. Most clients quit going to counseling if they think their interviewer is not listening to them. It is refreshing to find a concrete principle in psychology and counseling on which virtually everyone agrees. Attending behavior, and the importance of listening well, is spectacularly uncontroversial (Cormier & Nurius, 2003; Goldfried & Davison, 1994; Wright & Davis, 1994). Perhaps the only helping professionals who minimize the importance of body language are staunch psychoanalysts who continue to use the couch in psychotherapy. Yet, even psychoanalysts pay very close attention to their own vocal qualities, to tracking client verbal behavior, and to the general principle of listening well (Chessick, 1990; Geller & Gould, 1996). Attending behavior is primarily nonverbal. Edward T. Hall claimed that communication is 10% verbal and 90% a "hidden cultural grammar" (1966, p. 12). Others suggest that 65% or more of a message's meaning is conveyed nonverbally (Birdwhistell, 1970).

Positive Attending Behavior

Many authors have described different positive and negative attending behaviors (A. Ivey & Ivey, 1999; Pipes & Davenport, 1999; Shea, 1998). Positive attending behaviors open up communication and encourage free expression. In contrast, negative attending behaviors inhibit expression. When it comes to identifying positive and negative attending behaviors, there are few universals because cultural background and previous experiences affect whether clients view a particular attending behavior as positive or negative. Ivey and Ivey (1999) identify four dimensions of attending behavior, which are simple and have been studied, to some extent, cross-culturally. They include:

- Eye contact
- Body language

• Vocal qualities
• Verbal tracking

Eye Contact

The eyes have been called the windows of the soul. Cultures vary greatly in what they regard as appropriate eye contact. There is also much individual variation in eye contact patterns. For some interviewers, sustaining eye contact during an interview is natural. For others, it can be difficult; there may be a tendency to look down or away from the client's eyes because of shyness. The same is true for clients; some prefer more intense and direct eye contact; others prefer looking at the floor, the wall, or anywhere but into your eyes. For most clients, it is appropriate to maintain more constant eye contact when they are speaking and less constant eye contact when you are speaking. Some research suggests that clients' pupils tend to dilate when they are emotionally aroused or interested and constrict when they are bored or uncomfortable (Hess, 1975; A. Ivey & Ivey, 1999). However, for most people, eye contact is just a method of making personal contact with someone else. It usually does not involve intense scrutiny of the other person's physical characteristics.

Body Language

Aspects of communication that involve what most people, refer to as body language are known technically as *kinesics* and *proxemics* (Birdwhistell, 1952; E. Hall, 1966; Knapp, 1972). *Kinesics* denotes variables associated with physical features and physical movement of any body part, such as eyes, face, head, hands, legs, and shoulders. *Proxemics* refers to personal space and environmental variables such as the distance between two people and whether any objects are between them. Positive interviewer body language includes the following (derived from Walters, 1980). These positive body language examples are based on mainstream cultural norms. In practice, you will find both individual and cultural variations on these behaviors:

• Leaning slightly toward the client.
• Maintaining a relaxed but attentive posture.
• Placing your feet and legs in an unobtrusive position.
• Keeping your hand gestures unobtrusive and smooth.
• Minimizing the number of other movements.
• Making your facial expressions match your feelings or the client's feelings.
• Seating yourself at approximately one arm's length from the client.
• Arranging the furniture to draw you and the client together, not to erect a barrier.

Mirroring, as an aspect of body language, involves synchrony or consistency between interviewer and client. When mirroring occurs, the interviewer's physical movements and verbal activity is "in sync with" the client. Mirroring is a relatively advanced nonverbal technique that potentially enhances rapport and empathy, but when done poorly, can be disastrous (Banaka, 1971; Maurer & Tindall, 1983).

Vocal Qualities

Effective interviewers use vocal qualities to enhance rapport, communicate interest and empathy, and emphasize specific issues or conflicts. In general, interviewers' voices should be soft yet firm, indicating both sensitivity and strength. As with body language, it is often useful to follow the client's lead, speaking in a volume and tone similar to the client. Meier and Davis (2001) refer to this practice as "pacing the client" (p. 9). On the other hand, interviewers can use voice tone, as with all interviewer responses and directives, to lead clients toward particular content or feelings.

Verbal Tracking

It is crucial for interviewers to accurately track what clients say. Although eye contact, body language, and vocal quality are important, they do not, by themselves, represent effective listening. Interviewers demonstrate

an ability to track the content of their clients' speech by occasionally repeating key words and phrases. In most cases, clients do not know if you are really hearing what they're saying unless you prove it through accurate verbal tracking.

Negative Attending Behavior

It has been said that familiarity breeds contempt. When it comes to attending skills, it might be more accurate to say that overuse breeds contempt. It can be disconcerting when someone listens too intensely. Positive attending behaviors, when overused, are obnoxious. Interviewers should avoid overusing the following behaviors:

• ***Head nods***. Excessive head nods can be bothersome. After a while, clients may look away from interviewers just to avoid watching their heads bob. One child client stated, "It looked like her head was attached to a wobbly spring instead of a neck."

• ***Saying "Uh huh."*** This is a very overused attending behavior. Both novices and professionals can fall into this pattern. While listening to someone for two minutes, they may utter as many as 20 "uh huhs." Our response to excessive "uh huhs" (and the response of many clients) is to simply stop talking to force the person to say something besides "uh huh."

• ***Eye contact.*** Too much eye contact causes people to feel scrutinized or intimidated. Imagine having a therapist relentlessly stare at you while you're talking about something deeply personal, or while you're crying. Eye contact is crucial, but too much can be overwhelming.

• ***Repeating the client's last word***. Some interviewers use a verbal tracking technique that involves repeating a single key word, often the last word, from what the client has said. Overusing this pattern can cause clients to feel overanalyzed, because interviewers reduce 30- or 60-second statements to a single-word response.

• ***Mirroring***. Excessive or awkward attempts at mirroring can be damaging. We recall a psychiatrist who used this technique with disturbed psychiatric inpatients. At times, his results were astoundingly successful; at other times, the patients became angry and aggressive because they thought he was mocking them. Similarly, clients sometimes worry that counselors use secret techniques to exert special control over them.

Additionally, research indicates that clients perceive the following interviewer behaviors as negative (Cormier & Nurius, 2003; Smith-Hanen, 1977):

• Making infrequent eye contact.
• Turning 45 degrees or more away from the client.
• Leaning back from the waist up.
• Crossing legs away from the client.
• Folding arms across the chest.

UNIT 4 : AFFECTIVE PSYCHOTHERAPIES

The development of the humanistic psychotherapies

Although the humanistic psychotherapies are often stereotyped as a rebelling 'Third Force', emerging in the 1960s as a reaction against the 'old guard' of psychoanalysis and behaviour therapy, in fact it is probably more realistic to see them as one of many developments and evolutions, with strong roots in psychodynamic theory. As we have seen, the history of psychoanalysis and psychodynamic psychotherapy has involved many divergences and developments that, in their day, were as radical as the development of the humanistic psychotherapies. Key ideas such as the role of real experience in the development of psychological problems,

and the curative importance of the actual here-and-now contact between therapist and client, were identified as significant by these psychoanalysts. These clinical ideas, together with a number of philosophical principles, are the defining features across the whole range of humanistic therapies.

Philosophical aspects of the humanistic psychotherapies

Human Potential And Freedom :- Humanistic psychotherapists generally share an optimistic view of human beings, recognising that people have a 'lifeforce' that moves them towards health and well-being, rejecting the view that people are innately 'bad' or 'flawed'. While recognising that people can be extremely troubled and seemingly self-destructive, the humanistic view is that people have freedom and responsibility to choose how they create their lives and they can be enabled to find their own internal resources even in very adverse circumstances. This is sometimes misunderstood as ignoring genetic endowment, or life circumstances, or as promoting unrealistic omnipotence. In fact it is more a recognition that people can find self-worth and dignity, despite the limitations they meet.

Holism :- People are seen as indivisible from their context and indivisible in themselves, an idea drawn from Kurt Lewin's field theory (Parlett, 1991). So for the humanistic psychotherapist, the individual is unique and cannot be separated into, for example, mind and body, or thinking and feeling. The person is a unity of mind, body, action, feelings and spirit. Every aspect of the person is contingent upon every other aspect and cannot be separated off. Equally, the person's context (life situation, culture) cannot be ignored without inevitably misunderstanding or misperceiving the person.

Relationship:- Another core value is the centrality of human relatedness. Human beings are viewed as essentially social, and there is a recognition that problems often have their source in human relating, and that the human relationship offered by the psychotherapist is the most potent healing aspect in the psychotherapeutic process. Clearly these ideas exist in other psychotherapeutic theories (e.g. Bowlby, Kohut) – however the humanistic psychotherapies played a significant role in defining the therapeutic relationship itself as a central source of resolution and healing (Rogers, 1951/1990).

Phenomenology:- The humanistically based psychotherapist is interested in meeting the client's experience without bringing personal or theoretical assumptions that might cause them to pre-judge the client's difficulties or their causes. By encouraging the client to pay attention to all aspects of his or her experience equally, new meaning and significance can emerge. While each of these values is by no means unique to the humanistic psychotherapies, the constellation of them, taken together, and their centrality in defining the humanistic paradigm is what gives these

approaches a common ground and also what differentiates them from the other major 'schools' of psychotherapy. Perhaps it is because many of the principles of humanistic psychotherapy derive from philosophical values, and include the spiritual dimension of human experience, that these therapies have emerged so strongly over the past 50 years, gaining wide recognition and acceptance with clients. It is perhaps also for this reason that the development of a research evidence base for the humanistic psychotherapies has been a particular challenge.

Humanistic and integrative psychotherapy

Humanistic and integrative psychotherapy embraces many different schools of psychotherapy in which there is not one accepted theoretical approach. There are, to date, 22 different organisational members within the Humanistic and Integrative section of the United Kingdom Council for Psychotherapy. Most of these 22 claim a unique theoretical and methodological approach to psychotherapy. So to condense this great variety into a few paragraphs is problematic. We can say that a humanistic approach emphasises self-actualisation with the therapist facilitating that actualisation (Rowan, 2001).

Key concepts

The world of humanistic and integrative psychotherapy is conceptually highly diverse. It will be more useful here to look at some of the general principles that can be found to lie in all schools. This approach emphasises the idea of psychotherapy as being intrinsically involved in facilitating the client's development of their potential. This potential is realised paradoxically. Essentially change involves the acceptance of who one is (and therefore 'no change') and yet also full acceptance of oneself may involve personal change. In other words someone *accepting* that they feel bad about themselves results in them developing the capacity to change and to feel good about themselves. The concepts and practices within humanistic and integrative therapy grapple with these seemingly irreconcilable dimensions of human experience. This approach has an essentially optimistic view of human potential. People are regarded fundamentally as OK, all problems and difficulties are regarded as potentially solvable and everyone has the capacity to improve the quality of their life. Emotional problems are regarded as attempts to get on with life and as such need to be respected and understood within this framework. Only then will people be able to more effectively live their lives. People are conceived as whole persons. So conceptually these various approaches take a wide view of people. From a humanistic perspective people are feeling, thinking and acting entities with equal importance attached to all three processes. Also spiritual dimensions of human experience are incorporated into this conceptualisation of people. From the cradle to the grave people are engaged in making meaning of their experience and using this meaning to realise their potential. A recent very important development in humanistic psychotherapy is the emergence of integrative psychotherapy. Safran and Messer (1997) have identified three popular forms that psychotherapy integration has taken. First, there is a 'technical eclecticism' approach in which the practitioner seeks 'to determine the core ingredients different therapies might share in common, with the eventual goal of developing more efficacious treatment based on these components' (Norcross and Grencavage, 1990: 8). This is a technique-based form of integration in which the practitioner seeks to utilise in her work a variety of approaches. Reconcilable and irreconcilable theories are brought together and the practitioner selects techniques and approaches that are effective in treatment. Secondly, Safran and Messer identify a 'common principles' approach in which common factors identified in a variety of psychotherapeutic approaches are brought together. For example Nuttall (2002) argues that one such 'common principle' is the primacy of the therapeutic relationship as a factor for change. Therefore an integrative practitioner utilising this approach will pay particular attention to developing the therapeutic relationship. The last form of psychotherapy integration identified by Safran and Messer is 'theoretical integration'. The practitioner utilising this approach seeks to construct their own theory of psychotherapy that draws on other approaches. Here the practitioner seeks to devise a general unified theory of psychotherapy. Common to most integrative practitioners is a focus on integration itself. Integration is described as 'the act of combining or adding parts to make a unified whole' (Collins, 1999). The practitioner emphasises this unifying principle in which the client is understood to be embarking on an integrative, or 'making whole', process which the therapist facilitates.

Therapeutic application

Client Group:- Humanistic and integrative psychotherapy is practiced in group, couple and individual settings. Initially found exclusively in the private sector it is starting to establish itself in the public sector as more and more clients request a humanistic style of psychotherapy. Practitioners tend to work with reasonably functional clients who have a problem with a particular area of life such as relationships or work. It is, though, being found to have effectiveness with more deep-rooted problems such as personality disorder (Johnson, 2000; Bulmer, 2003; Shah, 2004). Frequency varies from a number of times a week to fortnightly or even monthly. The therapy itself can also be applied briefly or for long periods of time.

Early forerunners: Jaspers and Buber

As psychoanalysis was developing, the philosophy of existentialism, which emerged in nineteenth-century Europe, was beginning to influence the field of psychiatry (van Deurzen-Smith, 1997). In 1913, Karl Jaspers, a German psychiatrist and philosopher, was incorporating existential ideas from contemporary philosophers into an approach to human suffering and healing. In 1913 he published *General Psychopathology* in which he explored the psychological significance of these ideas. He identifies the role of phenomenological observation, and is perhaps the first to recognise the importance of 'empathic understanding', in enabling us to connect with and understand the inner world of the patient: 'We sink ourselves into the psychic situation and *understand genetically by empathy* [*sic*] how one psychic event emerges from another' (Jaspers, 1913/1959: 301). Thus Jaspers identifies the importance of human relatedness, seeing it as an essential human need and a key therapeutic tool. In Jaspers' work we thus find perhaps the earliest exploration of some of the ideas underpinning humanistic psychotherapy. In the 1920s the centrality of human relatedness was further articulated by Martin Buber, a philosopher who had a major influence on the development of existential psychotherapy, and also Gestalt therapy. Buber proposed that human existence is essentially dialogical and that we only exist in relation to the other. He identified two modes of relating, both of which are essential to life: the contactful and authentic 'I–thou' relationship, and the practical, goal-directed 'I–it' relationship (Buber, 1923/2004).

Carl Rogers and person-centred psychotherapy

Humanistic psychotherapy became more established through Rogers' work in the United States from the 1930s onward. Both Adler and Rank were involved in the child guidance movement at
that time, and it is within this field, and influenced by Rank (Cain and Seeman, 2001), that Rogers began to develop his 'Clientcentred therapy'. Rogers believed that people 'have within themselves sufficient drive towards health' (Kirschenbaum, 1979: 75), which may be thwarted by life experiences, but that, given the right conditions, the person will find within themselves the capacity to move forward and resolve difficulties. Rogers' landmark presentation on 'Newer Concepts in Psychotherapy', given at the University of Minnesota in 1940 (Cain and Seeman, 2001) critiqued existing psychotherapeutic approaches, and offered a radical new perspective. At the core of Rogers' approach is the idea that the provision of a respectful, empathic and authentic
relationship will in itself enable the client to activate his or her capacity for 'responsible self-direction' (Rogers, 1951/1990: 64) in solving his or her own problems. Rogers' core conditions of 'empathic understanding', 'acceptance' and 'genuineness' (Rogers, 1961/1989: 61–2) have become central to most humanistic psychotherapies, and are increasingly becoming integrated into other approaches. Rogers' theory and clinical practice has often been perceived as 'intuitive' or as little more than a set of beliefs or a philosophy, and this belies the clinical rigour of his work. Throughout his career Rogers was committed to the rigorous observation and evaluation of his ideas in clinical practice, and his work represents some of the earliest systematic research into psychotherapy (Cain and Seeman, 2001).

Rogers and Buber

In 1957 Rogers and Buber explored their ideas through a theoretical dialogue. Rogers particularly resonated with Buber's notion of 'confirming the other', by which Buber meant confirming the client's positive potentialities. However Rogers saw in this an implicit nonacceptance of 'negative' aspects of the client, which, for Rogers, was a form of judgement and objectification which undermines the client's 'self-actualising tendency' and is inimical to the principles of person-centred psychotherapy.

Existential psychotherapy

Existential psychotherapy has its roots in the philosophical ideas of Kierkegaard (1813–1855), Nietzsche (1844–1900), Heidegger (1889–1976), Husserl (1859–1938), Merleau-Ponty (1907–1961), Sartre (1905–1980) and Tillich (1886–1965) (van Deurzen-Smith, 1997). As early as 1908 the psychiatrist J.L. Moreno was drawing on existential ideas of personal responsibility in his use of drama techniques in psychotherapeutic groups to enable patients to process early
family difficulties and conflicts (Clarkson, 1992). As we have seen, Buber was also a key figure in the development of existential psychotherapy. Another significant contribution came from Viktor Frankl, who from his experience in the Nazi concentration camps developed 'logotherapy' (Frankl, 1946/2004), an existentially based psychotherapeutic approach focusing on the 'search for meaning' as the key to psychological survival and well-being. Frankl proposes that the focus in psychotherapy must be on the here-and-now and on life to come, rather than on early experience, and that owning freedom of will and finding meaning in life are central to psychological health. In the UK Ronald Laing (1927–1989) was strongly influenced by existentialism in the development of the 'anti-psychiatry' movement, which sought to address psychotic disturbances such as schizophrenia without the use of medication to interfere with the patient's self experience. He considered that psychotic experiences could be understood as 'an extreme form of existential misery' (van Deurzen- Smith, 1997: 163), and could be amenable to psychotherapeutic help. Also working in the UK, Emmy van Deurzen has done much to develop existential psychotherapy, and in the United States, Rollo May and Irvin Yalom have made significant contributions.

Gestalt therapy

Gestalt is a word derived from the German – it means whole, form, shape, body, framework. Gestalt therapy was originated by Fritz Perls and his wife Laura. Perls became a neuropsychiatrist, and briefly had analysis with Karen Horney, following which he trained as a psychoanalyst himself and worked in Berlin. During the 1920s he came into contact with gestalt psychology and existentialism. In 1933 he fled from fascism in Germany and moved to South Africa, where he founded the psychoanalytic society (1936). He began to develop his own ideas, departing from traditional psychoanalytic theory, but they were not well received. He moved to the US and in 1947, with Laura, began the first therapy group from which emerged gestalt therapy. In 1951 *Gestalt Therapy: Excitement and Growth in the Human Personality* (Perls et al., 1951/1990) was published. The goals of gestalt therapy are based in phenomenology, focusing on raising awareness, enlarging choice and facilitating the client to make full contact with themselves and the environment. There is no focus on change or problem solving; rather the view is that by focusing on 'what is', the client inevitably becomes aware of alternatives, which he or she is then free to choose (Beisser, 1970). Here we can see the humanistic principle that human beings are self-regulating organisms with an inbuilt movement towards development and the meeting of present needs. Change is inevitable because it is the normal, natural state of affairs when a person is in contact with him or herself and their environment. When contact is interrupted the person becomes unable to meet his or her needs. This may happen in early life, resulting in what gestalt therapists call 'unfinished business' – repetitive patterned interruptions to their 'organismic flow'. Addressing these interruptions is the primary focus of gestalt therapy, and this is done by paying attention to and exploring current experience. For Perls the therapist acted as a facilitator, and the focus was on the client's phenomenological world, rather than on the relationship with the therapist. He emphasised self-support, challenging the client who looked to the therapist to provide any external support. Having rejected the alienated relationship of traditional psychoanalysis, Perls appeared to have arrived at another kind of alienation,
based on existential aloneness. Gestalt theory since Perls has developed to incorporate the concept of the dialogic relationship from Buber's work (Richard Hycner, 1985; Lynne Jacobs, 1989), and current gestalt

therapeutic practice thus emphasises that phenomenological exploration takes place through the medium of the dialogic relationship between therapist and client (Yontef, 1993). Another recent development has been a move towards the integration of psychodynamic and self-psychology theories with Gestalt therapy (e.g. Hycner and Jacobs, 1995).

Other humanistic approaches

The emergence of the humanistic psychotherapies gained momentum after the Second World War, and it became clear that many different theories were emerging. Moreno's early work described above evolved into modern psychodrama, body-focused humanistic psychotherapies have emerged from Wilhelm Reich's work, while the transpersonal psychotherapies, including psychosynthesis, have explored the domain of human soul and spirit (e.g. Rowan, 1993; Wellings and McCormick, 2000).

The personal as political: equality and oppression

It is within the humanistic psychotherapies that perhaps most attention has been paid to issues of equality and oppression. Carl Rogers was very interested in the political power of personal change (Rogers, 1978). The women's movement in the 1960s and 1970s coined the phrase 'the personal is political' and focused on raising self-awareness (consciousness-raising) as a political tool. In 1981, feminist therapists Sheila Ernst and Lucy Goodison published *In Our Own Hands: A Book of Self-help Therapy* which sought to wrest psychotherapy from the hands of 'the experts' and make it accessible to women. Feminist therapy has since developed into a therapeutic model in its own right, across humanistic and psychodynamic fields. It seeks to challenge the patriarchal assumptions that are seen to be inherent in most psychotherapeutic approaches and to empower women to develop their self-esteem, assertiveness and capacity to author their own lives (McLellan, 1995). Similarly the 'pink therapy' movement (Davies and Charles, 1996) focuses on psychotherapy with lesbian, gay and bisexual clients. Undoubtedly the humanistic psychotherapy movement represents a rich diversity of psychotherapeutic theory and practice that seeks to address the full range of human experience. It is characterized by openness and flexibility to explore and widen the horizons of psychotherapy; however this has at times been viewed as a dilution and oversimplification of the complex task of addressing human distress. John Rowan in *The Reality Game* (1983), gave a comprehensive critical overview of the humanistic psychotherapy movement in Britain in the early 1980s, recognising both its strengths and its shortcomings.

Transactional Analysis (TA)

Developed in the 1950s and 1960s in the United States by Eric Berne, a Canadian doctor and psychoanalyst, TA is rooted in the humanistic and existential values. Its core theory, the ego-state model, is a sophisticated model of intrapsychic structure and psychodynamics evolved from psychoanalysis (Berne, 1961/2001). Berne also drew on learning theory and concepts of 'reinforcement' in developing his ideas about repetitive self-defeating strategies that maintain the person's 'life script' (Clarkson, 1992). Since Berne, TA, while maintaining its own distinct identity, has incorporated theory and technique from diverse sources, examples being the integration of techniques from gestalt therapy (Goulding and Goulding, 1979), or of self-psychology theory and neuroscientific findings (Hargaden and Sills, 2002). There is considerable justification therefore for regarding it as an early and evolving example of 'assimilative integration' (Arkowitz, 1997). Recent publications in the UK (Palmer and Woolfe, 1999; Lapworth et al., 2001) give a much stronger account of the place of the humanistic psychotherapies within psychotherapy integration, and both texts include a focus on the development of a 'personal integration' by the psychotherapist. This is an interesting concept, which acknowledges the theoretical and clinical sophistication of the psychotherapist, as well as the uniqueness of the human relationship that lies at the core of psychotherapeutic practice. It implies greater diversification of practice, and presents interesting challenges to the researcher. Despite the development of integrative psychotherapy, the field continues to be characterised by theoretical and methodological distinctions, and psychotherapy as a profession in the UK is structured to highlight difference rather than similarity. However,

this is changing. Recent trends in Britain, such as the exercise of developing criteria for National Vocational Qualifications in Psychotherapy, have begun to highlight the common ground across modalities, and there is increasing dialogue between different psychotherapeutic 'schools'. Indeed the series of books Core Concepts in Therapy (Open University Press) aims to explore the meaning of key elements
of psychotherapeutic theory across explicitly distinct psychotherapeutic models – *The Therapist's Use of Self* (Rowan and Jacobs, 2002), for example, draws on a wide range of humanistic and psychodynamic perspectives to develop a rich account of its subject. As the psychotherapy profession in the UK moves towards statutory regulation, the felt need to retain, value and protect what is distinctive in each approach is being challenged by the imperative of professional unity in order to secure the future of psychotherapy as a valid and accountable healing force.

Psychodynamic psychotherapy

A psychodynamic approach has its roots in the work of the early psychoanalysts including, of course, Sigmund Freud. This tradition and long history means that there is enormous diversity in the application of a psychodynamic approach. For example practitioners will
vary in their emphasis on the unconscious, on dreams as a means of understanding unconscious processes or on their attention to the evolving therapeutic relationship. Some practitioners will purely focus on the 'here and now' therapeutic relationship while others will make the presence of the past in the therapeutic relationship their primary focus.

Key concepts

A psychodynamic approach holds to a number of key principles. A central tenet is the universal existence of conflict and how people manage that conflict. Inner conflicts between thoughts, feelings, desires, ideas and so on are emphasised. A young child will want the security-invoking presence of their mother but at the same time will want to be rid of her when she prevents that child from having an experience they want. A young child has intensely destructive feelings towards his mother when she denies him a chocolate bar when he is hungry. The child tries to manage what he is driven to do, think or feel (eat and enjoy the chocolate bar) and what he wishes to do, think or feel (maintain his mother's love and affection). From time to time these are in conflict with each other and with society as a whole. Mental aliveness is a continual adaptation to these conflicting forces as the individual compromises desire with social and familial appropriateness. In order to contain intolerable states of anxiety, which arise through the experience of internal conflict, we develop a system of defences. These defences have the function of protecting us from the anxiety that arises through experiencing these unacceptable wishes. A psychodynamic approach proposes a basic structure of the mind within which the management of these conflicts occur. This structure was described by Freud as the ego, id and superego. Essentially the id contains unconscious material and the ego and superego contain conscious and preconscious material. A key concept here is the idea of the unconscious – processes that influence us outside of awareness. A principle arising from this concept is the
movement of unconscious material into consciousness or awareness leading to greater freedom and authenticity . Past relationships, particularly with early caregivers, are considered to be of primary importance in understanding and helping the client develop healthily. These early relationships are viewed as a template or prism through which we view the world. We construct
meaning and develop relationship patterns based on these early relationships. If our caregiver withheld interest in us whenever we were strongly upset we are likely to experience 'upsetness' as undesirable or wrong and prevent ourselves from this type of experience. We develop a pattern of closing down whenever we feel upset. These early patterned styles of relationships are identified as transferential relationships by the

psychotherapist. The early pattern is utilised by the client in the present when the other is perceived by the client as similar to the early caretaker. So as the client starts to feel upset in psychotherapy they shut down on this upset feeling and instead are experienced by the therapist as closing down. This is a very important concept in psychodynamic therapy as the therapeutic relationship itself and the patterns contained within it are understood as giving the client direct insight into their past experiences and how they are influencing them now.

Therapeutic application

Client Group:- A psychodynamic approach has application in individual, couple and group psychotherapy. A session will last 50 minutes and the frequency of sessions varies from weekly to as much as five times a week. Malan (1979) identified a number of key characteristics in clients for this approach to be successful. These can be divided into two areas: features of the client's lifestyle and capacities the client demonstrates in the therapeutic relationship. In terms of lifestyle the client needs to have at least one important sustained personal relationship and be able to show the capacity to hold back from impulsive behaviour such as suicidal acts or violent acts towards self or others. The conflicts the client experiences need to be at a neurotic level – that is, not all-pervasive in the client's reactions to the world. The client needs to actively engage themselves in the therapeutic process by staying focused on one theme. The client must be motivated to engage in their own development with the therapist and be able to withstand the pressures of interpretation and confrontation that the therapist uses in their work.

Relationship style

Psychodynamic therapeutic relationship is very much influenced by Freud. He thought it was very important that the therapist avoid criticism, encouragement, reassurance and advice giving. Hence the relationship does not have the normal dynamics of a conventional relationship. This unusual way of being with another influences the therapeutic relationship such that often the relationship is experienced as rather cold and uninviting. This often changes as the therapeutic benefits of the work start to be experienced by the client. The therapist will not try to establish a friendship. While there will not be an absence of basic warmth, friendliness is not a priority. The therapist will primarily look to interpret and confront the actions of the patient. Because the psychodynamic model places a lot of importance on conflict and attempts to resolve it, the therapist is interested in how conflict develops within the therapeutic relationship and will not shy away from conflictual experiences. Actually, the therapist may pay a lot of attention to attempts by the client to control the therapeutic relationship. Indeed Cooper goes so far as to say 'in successful therapy the therapist maintains control of the kind of relationship that will operate' (Cooper, 2002).

Brief therapy

Brief therapy, sometimes also referred to as short-term therapy, is a generic label for any form of therapy in which time is an explicit element in treatment planning. There is no uniform dividing line between brief and nonbrief therapies. Rather, the range of interventions described as brief range from single session therapies in the strategic and solution-focused literature to episodes of 20 or more sessions in the short-term psychodynamic tradition. It is not at all unusual, for example, to find models of cognitive restructuring therapy that last for 12–15 sessions. This might be viewed as brief treatment for a client with a personality disorder, but not one with an adjustment problem. There is Simon Budman and Alan Gurman's important distinction between therapies that are brief by design and those that are brief by default. A hallmark of all brief treatments is their planned nature. Many clients drop out of therapy for a variety of reasons, including a lack of resources, low motivation, disenchantment

with the therapist, or satisfaction with a small number of visits. Indeed, approximately one-half of all terminations appear to be unilateral, initiated by the client. Although such cases turn out to be of short duration, they are not brief in the planned sense. True brief therapy makes an active effort from the outset to streamline change processes. Defined in this manner, brief therapy is not a separate school of treatment, but an approach that is applicable to any school. There are several elements common to brief therapies that distinguish them from treatments that do not explicitly address the component of time. These include the following:

• ***Focus****:* Brief therapies typically target specific patterns rather than attempt broader personality changes. Such a focus is explicitly addressed in treatment planning and generally is part of a mutual understanding between therapist and client.

• ***Activity level of the therapist****:* Brief therapists characteristically adopt active methods to maintain the focus of treatment and promote self-understanding and change. The role of the brief therapist is more akin to a catalyst than to the blank screen described by Freud.

• ***Activity level of the client****:* The efficient utilization of time requires that clients be absorbed in change efforts between sessions, as well as during them. Brief therapies, therefore, generally make use of explicit tasks, homework exercises, and between-session efforts to apply skills and insights obtained during meetings.

• ***Client selection****:* The great majority of brief therapists recognize that brief therapies are not appropriate for chronic and severe mental illnesses, although techniques borrowed from brief approaches may be of value. Accordingly, therapy typically begins with an assessment to determine the appropriateness of brief treatment for a particular client.

• ***Enhanced experiencing****:* People appear to be more receptive to change when they are in altered and enhanced states of experiencing. Many methods utilized by brief therapists, including hypnosis, relaxation techniques, introspection, confrontation, exposure, and role playing, enable individuals to experience themselves and their problems in new ways.

Emphasis upon readiness for change*:* Following the work of James Prochaska and Carlo DiClemente, brief therapists typically target changes for which clients are ready. When clients are not aware of changes that need to be made or committed to making these, brief therapy goals will typically focus on advancing readiness for change rather than working through long-term resistance.

• ***Emphasis on impact of intervention****:* Brief therapists typically regard the emotional power of interventions, rather than their absolute duration, as critical to change. Accordingly, they tend to emphasize the promotion of corrective emotional experiences rather than exhaustive insight into past conflicts. By actively maintaining focused efforts at change and maximizing the involvement of both parties within and between sessions, brief therapists attempt to make optimal use of time in treatment. There are, however, a range of persons and problems for which brevity is not feasible and, indeed, can even exacerbate problems. Among the dimensions that are important in determining whether or not treatment can be brief are:

1. ***Duration****:* Although there is no one-to-one correspondence between the duration of a presenting problem and treatment length, it is fair to assume that chronic, long-standing patterns have been over learned and may require more intensive and extensive work on discrepancy and consolidation than recent, situational adjustment problems.

2.***Interpersonal history****:* A precondition of brevity is the formation of a quick, positive, and durable therapeutic alliance. This is most likely to be possible among clients who have not experienced significant interpersonal traumas. When clients have grown up within abusive, neglectful, or highly

inconsistent homes, the capacity for trust and bonding may be impaired. Such individuals may need many months of meeting before they can feel sufficiently comfortable to open up about their true concerns. This greatly extends the time needed to effect change.

3. ***Severity:*** When presenting problems are so severe and complex that they greatly impair emotional, vocational, and interpersonal functioning, it may be difficult to maintain the singular focus that is the hallmark of brief treatments.

4. ***Understanding:*** As suggested earlier, clients are most appropriate for brief work when they are aware of a problem in their life and committed to doing something about it.

5. ***Capacity to tolerate discomfort****:* Much of the brevity of therapy is accomplished by provoking client patterns within sessions and creating active opportunities for new learning. Such corrective experiences may be highly threatening to a client with poor coping ability and/or one who is in a state of crisis.

In the broadest sense, there are three overarching models of brief therapy: contextual, learning, and relationship. These make different assumptions regarding the scope and genesis of problem patterns, the role of the therapist in addressing these patterns, and the techniques to be utilized in effecting change.

A. Contextual Therapies

The contextual therapies owe their genesis to the pioneering work of Milton Erickson and the subsequent elaboration of his therapy in strategic-systems and solution- focused therapies. These are among the briefest of the brief therapies, sometimes addressing change in a single session. Common to the contextual brief therapies is the notion that problems are constructed and not intrinsic to the individual. Once people construe a set of life outcomes as problems, they typically attempt solutions to these problems, sometimes making the initial difficulties worse in the process. The result is a circular difficulty in which problems and solutions become self-reinforcing and amplifying. A common and simple example of this is the insomniac client who tries as hard as he can to fall asleep, only to find that repeated trying keeps him awake. Contextual brief therapy proceeds from the recognition that relatively small shifts in the context of the client's problem can be sufficient to interrupt these self-reinforcing cycles and set the client along a new trajectory. The emphasis is thus not on analyzing problems or their historical roots, but in finding contextual
parameters that no longer sustain the unwanted behavior patterns.

B. Learning Therapies

The learning models begin with a different set of assumptions, defining problem patterns as learned, maladaptive responses to situations. The goal of therapy from this perspective is to unlearn these self-defeating responses and replace them with more constructive alternatives. This places the client in the role of student and the therapist in the role of teacher. Instead of affecting client constructions with novel framings and experiences, the therapist engages in active instruction. These nascent skills can then be rehearsed in situations that have led to problems in the past, eventually becoming part of an established repertoire.

C. Relationship Therapies

Whereas the learning-based brief therapies rely heavily on skills-teaching and homework-based rehearsal, the relationship brief therapies emphasize interpersonal experience as a change vehicle. Central to these approaches is the idea that individuals internalize their interpersonal experiences,

cementing their identity in the process. When these experiences have been conflicted, the person internalizes a fragmented, negative, and/or inconsistent sense of self. This disrupts mood and action and potentially impairs future relationships.

Unlike the contextual therapist, the relationship therapist emphasizes the internal experience of clients and its social origins. Unlike the learning therapist, the relationship therapist stresses the curative impact of the helping relationship, rather than didactic instruction. The brief relationship therapies differ from their longer-term siblings—client-centered and psychoanalytic treatment—in their focus on specific relationship patterns and relative emphasis upon the present.

Adlerian psychotherapy

The primary indication of mental health in Adlerian psychotherapy is the person's feeling of community and connectedness with all of life. This sense of embeddedness provides the real key to the individual's genuine feeling of security and happiness. When adequately developed,it leads to a feeling of equality, an attitude of cooperative interdependency, and a desire to contribute. Thus, the central goal of psychotherapy is to strengthen this feeling of community. The major hindrance to a feeling of equality and the development of the feeling of community is an exaggerated inferiority feeling for which the individual attempts to compensate by a fictional final goal of superiority over others. Thus, the therapeutic process is simultaneously focused on three aspects of change. One is the reduction of the painful, exaggerated inferiority feelings to a normal size that can be used to spur growth and development and a healthy striving for significance.

A second is the dissolution of the patient's corrosive striving for superiority over others, embodied in a compensatory style of life. A third is the fostering of equality and feeling of community. Underlying this work is a firm belief in the creative power of the individual, to freely make choices and correct them, thus providing an encouraging perspective on responsibility and change. Adlerian psychotherapy is a creative process in which the therapist invents a new therapy for each client. Six phases of the therapeutic process are as follows:

(1) establishing the therapeutic relationship, (2) assessment, (3) encouragement and clarification, (4) interpretation, (5) redirection of the lifestyle, and (6) meta-therapy. These are briefly offered with the caveat that for any particular client, the actual therapeutic process may look quite different.

Phase I: Establishing the Therapeutic Relationship

Developing a cooperative working relationship is fundamental for any meaningful therapeutic progress. A warm, caring, empathic bond, established from the very beginning, opens the door for gradual, positive influence. Initially, the client may need to express a great deal of distress with little interruption. In response, therapist offers genuine warmth, empathy, understanding, and empathy. An atmosphere of hope, reassurance, and encouragement enables the client to develop a feeling that things can be different.

B. Phase II: Assessment

A through assessment is a critical step in Adlerian psychotherapy, for it will guide much of the therapeutic process. Although it is generally conducted during the first part of the treatment, information obtained throughout treatment may be used to refine and even correct initial impressions and interpretations. The objective of the assessment process is to conduct a comprehensive analysis of the patient's personality dynamics and the relationship among the—what Adler called the style of life.

At a minimum, this analysis includes an identification of the patient's inferiority feelings, fictional goal, psychological movement, feeling of community, level and radius of activity, scheme of apperception, and attitude toward the three life tasks of occupation, love and sex, and other people. A central assessment technique that Adler pioneered is the projective use of early memories. These memories—whether they are "true" or fictional—embody a person's core beliefs and feelings about self and the world and reflect the core personality dynamics. In addition to these early memories, the therapist uses the following: (1) a description of symptoms and difficulties, the circumstances under which they began, and the client's description of what he would do if not plagued with these symptoms; (2) current and past functioning in the domains of love relationships, family, friendships, school, and work; (3) family of origin constellation and dynamics, and extended family patterns, (4) health problems, medication, alcohol, and drug use, (5) previous therapy and attitude toward the therapist; (6) night and day dreams, and (7) information about the larger contexts in which the patient is embedded (e.g., ethnic, religious, class, gender, or racial contexts).

Phase III: Encouragement and Clarification

An ongoing central thread throughout the entire therapeutic process is encouragement. The therapist cannot give clients courage; this feeling must develop through the gradual conquest of felt difficulties. The therapist can begin this process by acknowledging the courage the client has already shown, for example, in coming to therapy. Then the therapist and client together can explore small steps that, with a little more courage, the client might take. For many clients, this is equivalent to doing the "felt impossible." During and after these steps, new feelings about efforts and results are acknowledged and discussed.

Phase IV: Interpretation

After the client has made some movements toward change and she and the therapist have examined the meaning of her movements and immediate goals, they eventually engage in an interpretation of the client's style of life. Discussing and recognizing these core personality dynamics, such as the inferiority feeling or the goal, can be both painful and even embarrassing. The interpretation process requires diplomacy, exquisite timing, and sensitivity. Doing this interpretation too soon is discouraging. The style of life is interpreted gradually, as the client gains success and strength in a new direction, discovers capacities that she has neglected, and begins to correct what she has omitted in her development. Once she has moved sufficiently in a new direction, the results of her new and old attitudes are then compared.

Phase V: Style of Life Redirection

This phase represents the depth work that is done for the client to redirect the lifestyle.This requires reducing and using inferiority feelings, redirecting the superiority striving, changing the fictional final goal, and increasing the feeling of community.

Phase VI: Meta-Therapy

A few clients may reach the quest for full personal development. The challenge is to stimulate each client to become her best self in the service of others, to awaken her inner voice, and to fully use her creative powers. Müller described the last phase of therapy as a "philosophical discourse." For those clients who need and desire this experience, Classical Adlerian psychotherapy offers the psychological tools and philosophical depth to realize their quest.

Applications and Exclusions

The strategies of Classical Adlerian psychotherapy are similar in individual, couple, family, and child psychotherapy. The central dynamic is the encouragement of each individual to develop his or her

capacities so as to reduce the inferiority feeling, to feel more equal with others, to become more cooperative, and to contribute to the improvement of all relationships for mutual benefit. In order to accomplish this, the style of life of each person usually needs to be redirected. Abbreviated adaptations of Classical Adlerian psychology have also been developed for use in brief therapy, career assessment and guidance, organizational consulting, and child guidance for parents and teachers.

Reality therapy

Founded by William Glasser reality therapy has its roots in the work of Alfred Adler, who emphasized that human beings are social in nature and that behavior is goal centered. Glasser extended his early ideas to include genetic instructions or human needs as sources of human behavior. Accordingly, human beings originate their own behavior. It is not thrust on them by their families, their environment, or their early childhood conflicts. Rather, behavior is seen as chosen. As in any therapy the therapist listens to clients' stories presented in their own words and seeks to become part of their inner discourse. In the language of choice theory the therapist becomes part of the clients' quality world (i.e., someone who is capable of providing needed help). The procedures are the specific tools for helping clients clarify and prioritize their wants, evaluate their actions and self-talk, and finally, make plans for effective change.

Theoretical Bases: Choice Theory

The practice of reality therapy is based on choice theory. Previously lacking a theoretical framework for reality therapy, Glasser employed the relatively obscure principles of control system theory to explain its effectiveness, and extended the theory to provide a basis for clinical practice by presenting a detailed explanation of human needs, total behavior (actions, thinking, feelings), perceptions, and inner wants or "quality world," the phrase used to describe our specific wants and intense desires. Control system theory is based on the principle that living organisms originate their behavior from the inside. They seek to close a gap between what they have and what they perceive they need at a given moment. This discrepancy, called a "perceptual error," sets the behavioral system in motion so as to impact the external world. Human organisms act on their external worlds to satisfy needs and wants. They gain input from and generate output toward the external world. Because of the emphasis on inner control and especially because of the emphasis on behavior as a choice, the theory was renamed choice theory in 1996.

WDEP

The delivery system of reality therapy, signifying wants, direction and doing, self-evaluation, and planning.

W: Explore Wants, Needs, and Perceptions

Essential to the process of change, as well as facilitating the relationship, is a clear determination and definition of clients' wants or desires. They are asked to describe current pictures or to insert firmly in their "quality worlds," exactly what they want. Using the analogy of wants as pictures, it is evident that clients often have blurred wants. They are unclear about what they want, so when they are asked, "What do you want from your job, from your spouse, from your parents, from your children?" the answer is,

"I don't know" or "I'm not sure." An adolescent often wants "my parents off my back" or "to be left alone" but is unable to provide a detailed and unambiguous description of this desire. Consequently, the reality therapist helps clients clarify and define wants, which is the process for the beginning of effective action on the part of clients.

D: Doing (Total Behavior)

Behavior is composed of four aspects: doing, thinking, feeling, and physiology. A popular misconception is that reality therapy neither deals with nor allows for a discussion of feelings and emotions. This erroneous perception is perhaps derived from the accurate statement that in reality therapy the action aspect of the behavioral system is emphasized (although not to the exclusion of the other components). Still, there are two important aspects to this procedure: exploration of overall behavioral direction and specific actions or choices. The therapist encourages clients to be specific in the discussion of behaviors, such as exploring a specific segment of time: a day, a morning, an hour, an incident, or an event. Although it is important to examine the overall direction of total behavior, direction will change only with small measurable changes made one at a time. Thus, therapists help clients become a television camera describing specific rather than typical events.

E: Self-Evaluation

If the entire process of environment and procedures is a cycle, the procedures appear as an arch with its keystone self-evaluation. This component is a prerequisite for change in human behavior. The term "Evaluation" has a meaning in reality therapy that is different from its meaning in other theories. In reality therapy, the procedure described here is not an assessment evaluation or "clinical diagnosis." Rather, it is a series of value judgments, decisions, and changes in thought made by the client. In the restructuring of thought, clients come to the conclusion that their life direction is not where they want to go, that a certain exact and specific current behavior is not useful or not helpful, that what they want is not attainable or helpful, that a perception is not effective, and that a future plan of action represents a more need-fulfilling behavior.

P: Planning

If evaluation is the keystone of the procedures, planning is the superstructure or the goal. A plan of action is crucial to change. It can sometimes be complicated and sometimes simple. There must always be a plan. People who go through life without some sort of long-term plan, are like ships floundering without rudders. This procedure is the easiest to bring about if the therapist has prepared the way by the effective use of the more subtle procedures and environmental components already described. Nevertheless, if the plan is to be effective, it should be characterized by as many as possible of the following qualities summarized by the acronym SAMIC3.

S = Simple: The plan is clear and not complicated.

A = Attainable: Realistically doable rather than grandiose and impossible.

M = Measurable. An effective plan is precise and exact.

I = Immediate. Implementation immediately after or even during the therapy session is desirable.

C = Controlled by the planner. A plan should not be contingent on the behavior of another person.

C = Committed to. The reality therapist elicits a firm commitment.

C = Consistent. The plan should be repeated.

The WDEP system should be seen as a unit, a system in which one component affects the others, and so, the subsystems W, D, E, and P are not isolated steps that must be followed one after another. Rather, it is more appropriate to extract from the system whatever component is most relevant at the moment.

Through listening, practice, and supervision, a user of reality therapy can develop a sense of where to start and how to proceed through the "Cycle."

UNIT 5: BEHAVIOR THERAPIES

Emergence of behavioural and cognitive therapies

Alongside the development of psychoanalytic psychotherapy with its focus on instinctual drives, early experience and unconscious processes, a very different paradigm for psychotherapy was emerging. This approach grew from advances in behavioural science, learning theory, cognitive psychology and experimental psychology between 1910 and 1920 (Fishman and Franks, 1992). It encompasses a range of therapies including behaviour therapy, cognitive therapy, cognitive behavioural therapy and rational emotive behaviour therapy. Broadly defined these approaches seek to use evidence-based techniques with the goal of producing constructive change in cognitions and/or behavior that solve the presenting problem.

Learning theories

The basis for these therapies is in the behavioural learning theories developed by Watson, Pavlov and Skinner. John Watson (1878–1958) developed behaviour theory and in 1913 defined psychology as 'a purely objective experimental branch of natural science. Its theoretical goal is the prediction and control of *behaviour* [*sic*]' (Watson, 1913: 158). These psychologists conducted experiments with animals that demonstrated that behaviour can be manipulated. Ivan P. Pavlov (1849–1936) demonstrated in 1903 that the reflex of a dog to salivate in the presence of food, can be 'conditioned' so that if a bell is rung at the same moment, eventually the dog will salivate when a bell is rung and no food is present (Lovell, 2000). This became known as 'classical conditioning' of behaviour. In 1938 B.F. Skinner (1904–1990), published his research demonstrating that a specific behaviour could be increased or reduced depending on the immediate consequence of that behaviour (operant conditioning). These consequences are known as 'reinforcers' and may be positive (presence of reward) or negative (absence of reward) (Skinner, 1938/1999).

Conditioning and behaviour therapy

In the UK, Hans Eysenck, a psychiatrist, had become disillusioned with traditional psychoanalytic approaches and in his 1952 study he attempted to discredit psychotherapy. This critique of the scientific basis for psychoanalysis led him to explore and research behavioural

techniques at the Institute of Psychiatry at the Maudsley Hospital in London (Eysenck, 1959), and he was one of the first to use the term 'behaviour therapy'. The focus for Eysenck and his colleagues was on applying learning theory and conditioning techniques to treat psychiatric problems, enabling the patient to develop more adaptive behaviours. Early critics of behaviour therapy, and particularly of 'operant conditioning', were concerned that a therapy designed to change behavior could be used to gain social and political control. B.F. Skinner's

Walden Two (1948) is a Utopian vision of social manipulation of human behaviour – however it is evident that the same techniques could be misused for political ends. It is perhaps because early work in this field emerged from psychiatry and was conducted with mentally ill or learning disabled people, including children, that concerns about consent became a focus. Fishman and Franks (1992) therefore make the point that therapeutic change should be considered

desirable and ethical by the patient as well as those around him or her. *Joseph Wolpe and Arnold Lazarus* Another early pioneer of behaviour therapy was Wolpe, a South African, who in 1958 developed the intervention of systematic desensitisation as a way to treat phobias. This involved exposing the patient to a manageable level of the fear stimulus, until the fear response subsided. The level of exposure was gradually increased until the patient could tolerate full exposure without an associated fear response (Tallis, 1998). These ideas were developed further by Arnold Lazarus, who studied with Wolpe. He used Wolpe's methods of systematic desensitisation, along with existing psychotherapeutic methods of support and insight, to develop a therapy that could be used to treat a range of psychological problems (Fishman and Franks, 1992). Lazarus is also credited with being one of the first to use the term 'behaviour therapy' to describe his approach (Lazarus, 1958), and he certainly played a major role in the further development and establishment of the behavioural psychotherapies when he and Wolpe moved to the United States. In the 1960s and 1970s behavioural therapies became well researched and well established. As well as the exposure techniques such as that described above, interventions included the modeling of the desired behaviour, training and coaching in social skills, and the use of self-help strategies (Glass and Arnkoff, 1995). Neither Eysenck nor Wolpe considered the therapeutic relationship to be a curative factor in itself, and initially it was very much downplayed in order to highlight the contrast with psychoanalytic models. More recently the presence of a supportive relationship has been acknowledged as significant to positive outcomes. In the later 1970s it gradually became clear that while research showed behaviour therapy to be effective for symptom relief in many anxiety-based behavioural problems (such as phobias), it had more limited success with problems (such as depression) where non-observable, internal processes are significant. This was a key factor in the emergence of the cognitive therapies.

Biofeedback

The concept of *biofeedback* is simple and appealing. A biofeedback practitioner measures a biological process such as muscle tension by means of an external sensor and provides an immediate visual or auditory display of this signal to the subject. The *feedback* of the biological signal enhances the individual's awareness of his or her own body and enables the individual to establish control over the physiological system. The individual observes the biofeedback display, becomes more aware of the bodily process, and learns to control the bodily process. In each case, human beings are empowered when they are able to bring disturbed bodily symptoms or mind-body processes under their own control. This experience becomes a prelude to increased control over emotions, work activity, relationships, and other areas of life. Thus, the modality of biofeedback naturally supports a philosophy of self-regulation and the acquisition of voluntary controls over one's own body and life. The effectiveness of biofeedback in teaching awareness and control has been shown with several physiological processes: muscle tension, skin temperature, respiration, autonomic nervous tension, heart rate, electrical wave activity in the brain, and brain blood flow. Biofeedback has well documented applications in health care, mental health, rehabilitation, education, sports psychology, and the performing arts. Within the mental health practice, there are a variety of well-documented applications: anxiety (Moss, in press; 2002) depression (Rosenfeld, in press), attention deficit disorders (Lubar & Lubar, 1999), alcoholism and addictions (Peniston & Kulkosky, 1990; Walters, 1998), and general psychotherapy (Moss & Lehrer, 1998; Wickramasekera, 1988). Early researchers and practitioners made many contributions to the foundations of biofeedback (Moss, 1998, pp. 146 153). For instance, Edmund Jacobson, a Harvard physician and

physiologist, developed progressive muscle relaxation. During the 1920s and 1930s, he refined this technique as a treatment for neurotic tensions and functional medical disorders (Jacobson, 1938). Johann Schultz, a German physiologist, developed autogenic training in the 1930s, a technique for creating a state of low physiological arousal, producing deep feelings of relaxation in the subject (Schultz & Luthe, 1959). Early behavioral researchers, including B. F. Skinner, Joseph Wolpe, and Albert Bandura, extended the lessons of experimental research on animals to create the new disciplines of operant conditioning and behavior therapy (Skinner, 1969; Wolpe & Lazarus, 1966). Three researchers played major roles in the emergence of biofeedback as a research-based approach: Joe Kamiya, Neal Miller, and John Basmajian.

Basic Biofeedback Modalities

Modality	Abbreviation	Biological process measured	Common applications
Electromyograph	EMG	Electrical activity of muscles	General relaxation training, headache, chronic back pain, stroke and neuromuscular rehabilitation
Electrodermal response Also referred to as: Skin conductance level Galvanic skin response	EDR SCL GSR	Electrical conductivity in palmar surface of fingers, EDR reflects sweat gland activity and mental activation	Cognitive relaxation, monitoring anxiety and cognitive activation in psychotherapy
Thermal biofeedback	TEMP	Provides index of peripheral blood flow and vascular dilation or constriction	General relaxation training, migraine headache
Respiration strain gauge	RESP	Monitors the activity and rate of breathing	General relaxation, anxiety, asthma, chronic pain, irritable bowel syndrome, psychotherapy
Capnometer	CAP	Measures CO_2 in expiration, detects hyperventilation	Anxiety disorders, asthma, respiratory illness
Heart rate Electrocardiogram Photoplethysmograph	HR EKG PPG	Monitors frequency and variability of electrical activity of heart. The EKG is more precise than PPG	Anxiety, cardiac rhythm disorders, IBS, pain disorders
Electroencephalograph	EEG	Measures the electrical activity of cortical surface of brain, shows activation of brain regions	Index of state of consciousness; attention deficit hyperactivity disorder, learning disability, alcoholism and addictions, neuropathy and psychotherapy
Hemoencephalograph	HEG	Measures hemodynamics, cerebral blood flow, volume, oxygenation, and temperature; shows activation of brain regions	State of consciousness and arousal. ADHD, LD, alcoholism, addictions, migraine, psychotherapy

Contraindications for Biofeedback Treatment

A number of conditions raise caution concerning the use of biofeedback, including patient age, mental retardation, psychotic disorders, the presence of life-threatening medical disorders, and personality disorders. These conditions remain cautions, however, not absolute contraindications. Biofeedback can be tailored to the intelligence level, degree of insight and motivation, and level of cooperation of a specific patient.

Behavioral Assessment

Behavioral assessment is the identification and measurement of a behavioral problem and its controlling variables (environmental, biological, or personal) for the purposes of manipulating variables relevant to target responses and, ultimately, evaluating and measuring change. From this commonly accepted definition two important characteristics of behavioral assessment arise: it is strongly linked to the clinical context, and it has emerged from a specific theoretical approach of clinical psychology. Behavioral assessment begins with the identification of the subject's problematic situation and the identification of its controlling, maintaining, or explaining

conditions (Fernandez-Ballesteros and Staats 1992). Thus, the first task is the specification of a person's problematic behaviors in term of deficits, excesses, or inappropriateness parameters (frequency, duration, etc.), and of the circumstances in which the behavior began and has occurred since. Functional analysis also demands identification of the relevant conditions that, hypothetically, explain, control, or maintain the behavioral problem. These conditions should be assessed, at this stage of the process, in terms of their observed association and} or interaction with the subject's problematic target behaviors, but are selected because of their well established relationships with the problematic situation. Thus, assessment begins with a case formulation or the conceptualization of the problem (Haynes and Follette 1993). The following five traits could be considered the essence of behavioral assessment: functional analysis, triple response modes and multicausality, idiographism, multimethodism, and the experimental method.

Functional Analysis

The main tool of behavioral assessment is functional analysis. It was defined by Skinner (1953) as the (stipulation of) external variables of which behavior is a function provide for what may be called a causal or functional analysis. We undertake to predict and control the behavior of the organism. This is our ``dependent variable "the effect for which we are to find the cause. Our ``independent variables "the cause of behavior are the external conditions of which behavior is a function' (p. 35). Over the history of behavioral assessment this definition has been extended, since other behaviors (besides overt behavior as dependent variable) and other sites of causation than external conditions (as independent variable) have been integrated; nevertheless, functional analysis continues to represent the core.

Triple Response System and Multicausality

The second characteristic of behavioral assessment arose from the complexity of behavioral problems both in their definition and their causation. Behavioral problems were to be defined through the triple response mode motor, cognitive, physiological and diverse potential causes multicausality were to be accepted.

Idiographism

The object of study of behavioral assessment is the behavior of a given person. This means identifying and defining behavioral problems and their controlling variables idiographically. In other words, psychologists should consider a single case and its specific circumstances.

Multimethodism

Methods in behavioral assessment are diverse. Nevertheless, observation (by experts or relatives) of motor behavior continues to be a commonly used method of assessment for assessing both overt behavior and its external antecedents and consequences. Other methods, such as self-report, self-monitoring, etc., are also administered for assessing public and private events (feelings of sadness, automatic thoughts, etc.).

Experimental Approach

Behavioral assessment provides the basis for behavioral change, behavioral change requires treatment treatment, and treatment demands experimental manipulations and evaluation (Hayes and Nelson 1986). This is why, throughout the history of behavioral assessment, the experimental method has been a constant among its basic characteristics.

Extinction

Operant extinction is a procedure in which reinforcement of a previously reinforced behavior is discontinued. Reinforcement occurs when a behavior is strengthened by its consequences. When such consequences no longer occur, the behavior eventually weakens and occurs rarely, if at all. This is operant extinction (hereafter called "extinction"). During extinction, emission or omission of the response in question produces neither a negative nor a positive reinforcer. Therapeutic applications of extinction involve four steps:

1. The behavior to be reduced must be defined and a system for accurately quantifying the behavior must be selected. This step is a prerequisite to treating any behavioral problem.
2. The events that follow and maintain the behavior, that is, its reinforcing consequences, must be determined. Isolating the consequences that reinforce a behavior, and the situations under which those consequences are evident, is termed functional assessment. Functional assessment is often, although not necessarily, accomplished through direct observation and interviews with care providers. It must be done prior to the use of extinction.
3. Conditions must be arranged so that the consequences that previously served as reinforcers no longer follow the inappropriate behavior. Moreover, the behavior must not be followed by new reinforcing consequences. The exact procedures required to arrange extinction depend on the nature of the reinforcing consequences and the situations in which they occur.
4. As with any treatment, the effects of extinction must be evaluated carefully. If necessary, the procedure should be modified to produce the desired effects. Although extinction is simple in principle, in practice it often is difficult to arrange and initial attempts frequently are unsuccessful. Moreover, extinction sometimes produces undesired and unacceptable changes in behavior that may require termination of the procedure.

Planned ignoring is a procedure related to extinction that is often used as an intervention for reducing inappropriate behavior in children. Planned ignoring involves the deliberate withholding of attention, including verbal exchanges and physical contact, for a brief period (typically no more than 1 or 2 minutes) following inappropriate behavior. This procedure involves extinction if the inappropriate behavior historically was maintained by events that are withheld during the period of planned ignoring and the behavior actually occurs during this period.

Theoretical Bases

Like punishment and reinforcement, extinction is a term that describes a particular relation among behavior and other environmental events. There is no "theory" of extinction as the term is commonly used. As discussed in the article "Operant Conditioning," it may eventually be possible to disclose the physiological changes responsible for the behavioral effects of extinction. For the most part, however, behavior analysts have been content to describe relations among behavior and other environmental events and to ascertain the variables that modulate those relations.

Exposure therapy

Exposure techniques are used to treat fear, anxiety, or other intense negative emotional reactions. The therapist encourages the patient to confront situations that give rise to negative emotion. Typically, the patient will erroneously believe that these circumstances are personally quite dire, and she will actively avoid and escape cues that signal them. Exposure to these feared or avoided situations allows the patient to gather data that are inconsistent with such beliefs. That is to say, she comes to realize that the feared situation is actually safer than she has previously thought. She also learns that avoidance and maladaptive anxiety-neutralizing or safety behaviors, such as ritualizing in obsessive-compulsive disorder (OCD) or taking antianxiety medication, are not required to cope with the anxiety. Exposure can be implemented in vivo or in imaginal mode. In vivo exposure involves actually encountering the feared situation or event, whereas imaginal exposure involves vividly imagining the event as if it were happening in the moment. The newest exposure method is virtual reality, which effectively produces vivid images and sensations of feared objects such as spiders (Garcia-Palacios et al., 2002), as well as feared situations such as airplane flight (Maltby et al., 2002), public speaking (Harris et al., 2002), or the Vietnam War experience (Rothbaum et al., 1999).

When planning exposure therapy, the therapist and patient identify a list of situations that are typically feared or avoided by the patient. The hierarchy should contain representative situations that are important to the treatment goals and the patient's functioning. The situations are then ranked in order of difficulty for the patient. The therapy begins with exposure to one of the easier items on the list, then, in a careful and concerted fashion, the patient and therapist move through the hierarchy until the patient has been exposed to the most difficult item on the list. Cognitive-behavioral applications include exposure to bodily symptoms in panic disorder and OCD, exposure to feared situations in posttraumatic stress disorder (PTSD) and social phobia, exposure to feared objects in specific phobia, exposure to traumatic memories in PTSD, and exposure to worry in generalized anxiety disorder (GAD).

A behavioral experiment (J. S. Beck, 1995) is a therapeutic technique much in the spirit of exposure methods for anxiety; however, it is a more versatile intervention, applying across a range of problems and areas of functioning. The main goal of a behavioral experiment, as with exposure, is to have the patient test out a specific, typically erroneous, belief or thought within a particular situation. When well-designed and carefully executed, such experiments play a pivotal role in the process of cognitive change (Newman et al., 2001). Thus, the depressed patient can, for example, discover the inaccuracy of her belief that exercise is useless or the belief that she won't enjoy a date (J. S. Beck, 1995). Likewise, a patient experiencing command hallucinations can discover the inaccuracy of his belief that the â€˜voiceâ€™ is all-powerful or all-knowing (Chadwick et al., 1996).

Flooding

Traditionally, flooding has been defined as a technique that involves prolonged exposure to feared stimuli or situations at full intensity, in an attempt to extinguish anxious responses. In other words, from the beginning of treatment, the client is exposed to stimuli that evoke maximal anxiety. In addition, the client is exposed to these stimuli for prolonged periods of time, until anxiety has begun to dissipate. Flooding typically is conducted *in vivo,* with live presentation of anxiety provoking stimuli. The duration of each session varies depending on the client's presentation, with a typical session lasting anywhere from 90 to 180 minutes. The length of the session is not predetermined but instead is dictated by the client's level of anxiety. In addition, the number of sessions is not fixed but instead is based on the client's anxiety level. Research suggests that significant symptom reduction generally occurs following 10 to 15 hours of exposure. Throughout the flooding procedure, anxiety symptoms should be consistently monitored via physiological assessment or client subjective ratings of distress. Sessions

typically are spaced close in time (e.g., daily or several times per week) rather than scheduled at weekly intervals.

A. *In Vivo* versus Imaginal Flooding

Clients may be exposed to actual feared stimuli (*in vivo* flooding) or may be asked to picture feared stimuli
(imaginal flooding). Deciding which procedure to employ requires clinicians to consider practical matters as well as prior research. Overall, studies have found that *in vivo* exposure is superior to imaginal exposure in reducing anxiety symptoms. In particular, for simple phobias, anxiety reduction is clearly enhanced with *in vivo* exposure. However, *in vivo* stimuli presentation has been found only marginally superior to imaginal exposure for obsessive–compulsive symptoms. Logistically, setting up *in vivo* exposure situations may be difficult when the fear involves remote or unpredictable stimuli (e.g., fear of earthquakes). Further, it may be impossible to arrange *in vivo* exposure to existential fears (e.g., fear of dying). Finally, due to the anxiety-inducing nature of treatment, clients may be more reluctant to engage in live exposure situations, at least initially. However, imaginal exposure also has its difficulties. First, not all clients are capable of evoking anxiety-inducing images in sufficient detail or for sufficient periods of time, which may necessitate time-consuming imagery training. Second, clinicians never will be able to fully ensure treatment adherence with imaginal exposure, as such procedures are assessed only via self-report.

Graduated versus Intensive Stimuli Presentation

Clients may be exposed to stimuli in a graduated fashion, beginning with a stimulus that induces little anxiety, or in an intensive fashion, beginning with a stimulus that is maximally anxiety-provoking. The decision on which format to follow should be based on prior research and practical concerns. Research has demonstrated that either format is effective in reducing overall anxiety symptoms, both immediately following treatment and after a short interval. However, one study found that intensive exposure appears more effective in the long run.

Response Prevention versus Exposure Alone

Typically, flooding involves both exposure to a feared stimulus and a response prevention component. In other words, once exposed to the stimulus, clients are prevented from responding with escape or avoidance behaviors. It was believed that allowing clients to avoid focusing on the anxiety or to escape the flooding situation prior to anxiety reduction would only increase anxiety symptoms. Prevention of escape or avoidance behaviors was once considered the hallmark of flooding procedures. However, more recent research has suggested that exposure alone may be as effective as exposure with response prevention. Studies on the role of avoidance behaviors have examined the use of distraction techniques (e.g., playing video games, spelling tasks) or medications (e.g., anxiolytics) during flooding procedures. Cognitive distraction techniques allow clients to avoid focusing on their feelings of anxiety.

Theoretical Bases

To understand how flooding is believed to work, acquisition and maintenance of fear or anxiety first must be considered. Orval H. Mowrer developed a two-factor theory that incorporates both respondent and operant conditioning principles. He posited that fear originally is learned through respondent conditioning but maintained through operant conditioning. A person learns to fear a previously neutral stimulus when it is paired over time with an aversive unconditioned stimulus. Once this conditioned

stimulus elicits fear, the person will engage in escape or avoidance behavior in an attempt to reduce fear or anxiety. Successful reduction in fear or anxiety reinforces escape or avoidance behaviors, increasing the likelihood that they will be repeated in the future.

Based on the two-factor model, flooding is believed to work to reduce anxiety by the process of extinction.

Extinction is defined as repeatedly presenting a conditioned stimulus without an unconditioned stimulus; over time, the conditioned response will diminish. During flooding, the person experiences no aversive event with the presentation of the feared stimulus. Repeated and continual presentation of the conditioned stimulus without the unconditioned stimulus results in extinction of the fear response over time.

Graded *in Vivo* Exposure

This is the method most commonly used to reduce fear. It has two components. First, the patient is instructed how to rate the intensity of discomfort using a Subjective Units of Distress Scale (SUDS). This measure of fear and distress ranges from 0 to 100, where 0 = none, 50 = moderate, and 100 = extreme. Second, the therapist and patient devise a hierarchy of real-life fear evoking stimuli, ranging from stimuli that evoke little or no fear or distress, to extremely frightening or upsetting stimuli. Patients begin by exposing themselves to items lowest on the hierarchy. Exposure to a given stimulus is repeated until the fear or distress abates. When fear of a given stimulus is reduced, the other stimuli on the hierarchy also tend to become less fear evoking. The patient gradually works up the hierarchy; once the fear of one stimuli is diminished, exposure to the next stimulus is attempted. This continues until all the stimuli on the hierarchy no longer evoke fear or distress.

Systematic Desensitization

Systematic desensitization consists of gradual, imaginal exposure to stimuli organized on a hierarchy constructed using SUDS ratings. Typically, systematic desensitization is combined with some form of relaxation training. The patient is asked to sit back in a comfortable chair and practice a relaxation exercise.

Once a state of deep relaxation is attained, the patient is asked to imagine the least upsetting stimulus on the hierarchy. Exposure duration might be only for a few minutes, alternating relaxation with imaginal exposure until the imagined stimulus no longer evokes fear or distress. The procedure is then repeated with the next stimulus on the hierarchy. The disadvantage of systematic desensitization is that it is slow, and that it is often necessary to eventually implement some form of real-life exposure in order to fully reduce the fears. The advantage is that it is easily tolerated and is therefore a good place to start when treating patients with extremely severe fears.

Implosion

Implosion involves exposing the person to intensely fear-evoking imagined stimuli, which lie at the top of the fear hierarchy. Often exposure is embellished by having the patient imagine extremely terrifying forms of the stimuli. Implosion is often used in the treatment of posttraumatic stress disorder, where the goal is to reduce the fear and associated distress associated with traumatic memories. More often, however, the feared stimuli are simply imagined in great detail, with the person imaging all the sensory aspects of the stimulus (e.g., sights, sounds, smells), along with any bodily sensations, emotions, and thoughts that might occur when the stimulus is encountered. A person who developed posttraumatic stress disorder as a result of being in a motor vehicle accident, for example, would be asked to

repeatedly imagine the traumatic experience, typically for 30 to 45 minutes per treatment session over several sessions. The narrative of the experience might be spoken into a tape recorder or written down, and the person would be encouraged to repeatedly go over the tape or transcript until the memory of the traumatic event no longer evokes distress. The advantage of implosion is that by reducing the distress associated with traumatic memories, the other symptoms of posttraumatic stress disorder also tend to abate. In other words, implosion can lead to reductions in reexperiencing symptoms (e.g., nightmares, flashbacks), hyperarousal symptoms (e.g., irritability, increased startle response), and avoidance and numbing symptoms (e.g., avoidance of reminders of the traumatic event). A further advantage is that implosion enables the patient to overcome fears for which live exposure is impossible or impractical. Fear of thunderstorms, for example, can be reduced by having the patient repeatedly imagine such events. The disadvantage of implosion is that it can be demanding on patients; they must be able to tolerate intense distress. A further problem is that implosion does not teach patients a skill that they can readily employ by themselves at home, simply because it is often too difficult because of the degree of emotion generated. Even if a patient is able to complete a course of implosion, it is often necessary to add some form of real-life exposure in order to completely reduce the fear. A person with a thunderstorm phobia might need to actually experience a thunderstorm in order to fully overcome his or her fear.

Covert Extinction

In this procedure, clients are instructed to imagine performing an undesirable behavior (the target behavior) and then not being reinforced for that behavior. For example, a client who talks too much will be instructed to imagine herself talking with no one listening.

Aversion therapies

Classical conditioning methods for reducing alcohol consumption include emetic treatments, where the person ingests a nausea-inducing drug and then is asked to sample his or her favorite alcoholic beverage. Over a series of trials, the person learns to associate drinking with immediate nausea. Thus, the former association between drinking and pleasure is replaced by an aversive association, thereby reducing alcohol consumption. Similar treatments have been devised for treating self-injurious behavior in children and paraphilias in adults. Aversion therapies are not commonly used today. This is because of ethical concerns about inflicting harm on patients, and concerns about patient welfare. The efficacy of aversion therapies is also debatable. Some critics argue that these treatments are short-lived in their effects, and that they only suppress maladaptive responses without teaching the patient more adaptive alternatives. Aversion therapies are widely regarded as interventions of last resort, to be used only when more benign methods have failed. If used, aversion therapies should be administered by a trained professional, with the approval of the institution in which the professional is working. Informed consent should be obtained and regional laws should be considered, because aversion therapies have been outlawed in some jurisdictions.

Skill training

In recent years, several expansions have been made that go beyond application of traditional behavioral principles to the practice of consultation. It has been recognized that changing the contingencies associated with various behaviors may not be sufficient to cause clients to improve their behavior. Clients may not possess the skills necessary to perform any positive, alternative behaviors to the problem behavior. In this case, even if clients are motivated to change, it may be impossible for them to alter their behavior because they do not know what else to do. The remedy in these situations is to

provide the client with skill training. Skill training serves to teach clients what positive, alternative behaviors they should perform, and how they should carry out those behaviors in practice. At times, skill training can be a lengthy process, especially for clients with significant cognitive or emotional challenges.

Assertive training

The aim of assertion training is to enhance interpersonal effectiveness in social situations. Positive consequences may be forgone because of anxiety in social situations. Assertion training emphasizes the extent to which we can influence our social environment by being active in its construction. In 1973 Joseph Wolpe defined assertive behavior as "The proper expression of any emotion other than anxiety toward another person." Lack of effective social skills may result in a variety of maladaptive behaviors. Assertion training often in combination with other methods, has been used to address a wide variety of presenting complaints including substance abuse, aggressive and explosive behaviors, and obsessive–compulsive behaviors. It has been used to help people make friends, arrange dates, and acquire needed help (e.g., on the part of individuals with learning disabilities). Essentially, assertive skills are effective social influence skills acquired through learning. Assertion training usually consists of a variety of components, including instruction, model presentation, behavior rehearsal, feedback, programming of change, and homework assignments. Other procedures that may also be used, depending on what is found during assessment, include self-instruction training, relaxation training, cognitive restructuring (e.g., decreasing unrealistic expectations or beliefs), and interpersonal problem-solving training (helping clients to effectively handle challenging situations that arise in social situations such as reactions of anger that get in the way of maintaining friendships).

Modeling

Instruction, model presentation, rehearsal, and coaching can be used when clients lack requisite behaviors in certain situations or when there is a need to refine behaviors. The need to use modeling will be influenced by the complexity of the skill to be acquired and nature of the entering repertoires (available skills) of clients. The greater the complexity of the skill and the more lacking the initial repertoire, the greater the value of model presentation is likely to be. An advantage of model presentation is that an entire chain of behavior can be illustrated and the client then requested to imitate it. Nonverbal as well as verbal behaviors can be demonstrated and the client's attention drawn to those that are especially important. Effective behaviors may be modeled by the counselor, or written scripts, audiotape, videotape, or film may be used. Essential elements of various responses can be highlighted and written models offered. The advantage of written material is that it can be referred to on an as-needed basis. In addition, the client may be asked to observe people with effective behavior who are in similar roles and to write down the situation, what was done, and what happened.

Behavior Rehearsal and Feedback

Following model presentation, the client is requested to practice (rehearse) the modeled behavior.

Corrective feedback is offered following each rehearsal. Specific positive aspects of the client's performance are first noted and praised. Praise is offered for effective behaviors or approximations to them, and coaching provided as needed. The focus is on improvements over baseline levels (what a client can do before intervention is initiated). Thus, approximations to hoped-for outcomes are reinforced. Critical comments such as "You can do better" or "That wasn't too good," are avoided. The client is encouraged to develop behaviors that are most likely to result in positive consequences. A hierarchy of scenes graduated in accord with the client's anxiety may be used for role-playing. Role-

playing starts with scenes that create low levels of discomfort. Clients who are reluctant to engage in role-playing can be requested to read from a prepared script. As comfort increases, role-playing can be introduced. If a client is too anxious to read from a script, relaxation training may be offered as a prelude to role-playing. When there are many skills to be learned, one behavior at a time may be focused on. Each role-play may be repeated until required levels of skill and comfort are demonstrated. Covert modeling or rehearsal in which clients imagine themselves acting competently in social situations may be as effective as actual rehearsal if clients possess needed social behaviors (but do not use them) and if social anxiety is low. Home sessions in which clients engage in covert rehearsal can be used to supplement rehearsal in office sessions. Not only does behavior rehearsal provide for learning new behaviors, it also allows their practice in a safe environment and so may reduce discomfort. Rehearsal involves exposure to feared situations. This exposure is considered to be a key factor in decreasing social anxiety, especially if people remain in the situation even when they are anxious and act effectively in spite of their discomfort.

Operant procedures

Token economy program (TEP) is a system whereby clients earn tokens in exchange for engaging in designated target behaviors. In some TEPs, clients will also lose tokens or be "fined" in response to engaging in inappropriate behaviors. Just as we use money to buy the things we want, clients in a TEP exchange their tokens for a variety of desirable backup reinforcers, including food, beverages, magazines, toiletries, CDs, potted plants, toys, crayons, school supplies, and other desirable goods. Clients in residential settings may be given the opportunity to purchase a private bedroom, room furnishings, or home passes.

B. F. Skinner's pioneering work in operant conditioning is generally credited with providing the theoretical foundation for the development of token economies. It is assumed that basic laws of learning account for the
occurrence of behaviors, whether adaptive or maladaptive, and these same laws can be extended to change
these behaviors through the thoughtful engineering of environmental events. Two major laws synthesize theories of learning and have been validated by a body of basic and applied research. The law of effect states that the frequency of a behavior is dependent on the resulting consequences, or effects. Thus, behavior is strengthened or weakened by what follows it in the environment. In fact, a consistent finding has been that the more immediately a consequence follows a behavior, the greater effect it will have. Reinforcement always has the effect of increasing the likelihood or recurrence of behavior. In positive reinforcement this is done by adding a positive or desirable event (e.g., tokens) to the environment when a behavior occurs.

Contingency management

Contingency management refers generally to the modification of behavior via the control or manipulation of consequences (contingencies) to the behavior. In essence, some control is sought over what follows a certain behavior to increase or decrease the frequency or other characteristics (e.g., intensity) of that behavior. Contingency management involves a collection of therapeutic techniques derived from Skinnerian principles of reinforcement. In addition, practitioners of contingency management often enlist those who surround a particular client to distribute consequences, but

contingency management may involve the client himself or herself as well. Consequences that follow a particular behavior can often be categorized as positive or negative. In addition, such consequences may be given or taken away. A positive consequence that is given refers to positive reinforcement and generally serves to increase the frequency of a behavior. A negative consequence that is given refers to punishment and generally serves to decrease the frequency of a behavior. Contingency management generally involves manipulating these consequences, both positive and negative, to modify a particular behavior. Specific types of consequences used in contingency management include tangible and intangible stimuli. Positive tangible consequences include such things as food, money, toys, and privileges, whereas negative tangible consequences include such things as pain or physical discomfort. Positive intangible consequences include such things as attention, sympathy, and social support, whereas negative intangible consequences include such things as emotional deprivation, lack of conversation, or failure to help. One of the biggest threats to the success of contingency management procedures is inconsistent application of the rewards, penalities, and punishments. Several scenarios for this apply to parents and children. First, it is often difficult for parents, who may have had a long history of reflexively attending to their child's misbehavior, to break free from that pattern and respond only to appropriate behavior. Second, parents who have little history of rewarding their children, who emphasize punishment instead, may require time to correctly implement a positive reinforcement system. Third, a mother and father may inconsistently apply contingencies or some third party (e.g., grandparent) may sabotage the process. Fourth, parents may implement contingency management procedures at first but then relapse to old patterns of behavior or for some reason delay the administration of consequences.

Contingency contracting involves the distribution of rewards, penalties, and punishments via a written contract constructed between two or more parties. Contracting is often used in family therapy as a vehicle for problem-solving, but is also an excellent way of providing rewards, penalties, and punishments in a structured and agreed-on format. After a specific behavior (e.g., curfew) is defined as the presenting problem, each party (e.g., parent and adolescent) develops parameters to indicate how they would like the behavior defined and what consequences may follow its presence or absence. Contracts are often used with parents and children but can also be used for other clinical populations.

Differential reinforcement of other behavior (DRO) refers to rewarding behavior other than a specified undesirable behavior. A child with autism, for example, may be given a reward during any 5-minute interval during which he or she plays with no aggressive behavior. In related fashion, an adult may be rewarded for interacting with others and not avoiding an anxiety-provoking situation.

Self-control procedures

Thought-Stopping

This procedure consists of three parts (hence the term "triad"). First, clients are instructed to shout "stop" to themselves (preferably not aloud, especially in public!) whenever they think of the undesirable behavior or thought. Second, they are instructed to take a deep breath, exhale, and relax. Third, they then imagine a pleasant scene as a reward for terminating the undesirable thought. Thus, negative reinforcement or escape conditioning is involved. Less obvious variations of this procedure have been used such as snapping a rubber band on one's wrist as an aid to stopping the thought or cognitive distraction from the intrusive thought to another image. The self-control triad can be useful for clients who are plagued by constant negative ruminations.

Paradoxical Intention

It is a technique used in logo therapy. Frankl defines paradoxical intention as follows: "The patient is encouraged to do, or to wish to happen, the very things he fears (the former applying to the phobic patient, the latter to the obsessive-compulsive)." This technique builds on the human capacity for self-detachment to break the vicious cycle, which traps people in psychogenic neuroses, such as phobia, anxiety, and obsessive—compulsive behaviour. Self-attachment enables to patient to adopt a new attitude, to stand back or laugh at the situation or self. In applying paradoxical intention, the therapist tries "to mobilize and utilize exclusive human capacity for humor." For the phobic patient, he has a "fearful expectation" that a particular symptom might occur, and his fear creates "anticipatory anxiety, which in turn brings about what the patient fears to happen. Thus "fear of fear" creates a "vicious cycle." The most common reaction to "fear of fear" is "flight from fear," and the phobic pattern is maintained by excessive avoidance. This vicious cycle is broken when "the pathogenic fear is replaced by a paradoxical wish." As a result, the patient no longer avoids situations that create anxiety. According to D. Guttmann, paradoxical intention has been used with increasing frequency with good results especially in treating clients who suffer from phobias and obsessive–compulsive disorder.

UNIT 6 : COGNITIVE THERAPIES

Cognitive therapies: Albert Ellis and Aaron Beck

In the mid-1950s there was an upsurge of interest in cognition within the field of experimental psychology, known as the 'cognitive revolution' (Murray, 1995). Among clinicians there was a growing interest in the role played by cognition in the formation and maintenance of human difficulties. In the 1950s both Albert Ellis and Aaron Beck, who were practising psychoanalysts, were beginning to explore this area. Ellis developed 'rational therapy' in 1955 (Dryden, 2000), publishing *Reason and Emotion in Psychotherapy* in 1962. Ellis developed his ABC model, which proposed the idea that **A**ctivating events in a person's life results in painful emotional **C**onsequences (depression, distress) because of the internal irrational **B**eliefs the person holds in relation to the event. Ellis intervened directly to challenge and alter these beliefs, resulting in the person feeling and functioning better. Early on Ellis' approach became known as rational-emotive therapy, and in 1993 it was renamed 'rational emotive behaviour therapy', to acknowledge its behavioural as well as its cognitive and emotional aspects. Aaron Beck first published his theories of cognitive therapy in the 1960s. Initially both behavioural and psychoanalytic psychotherapists roundly rejected them. However, with the publication of *Cognitive Therapy and the Emotional Disorders* (Beck, 1976/1991), cognitive psychotherapy began to gain recognition and respect. Beck proposed that our cognitions affect our feelings, behaviour and physical responses. Throughout life we develop cognitive 'schemas' or templates, based on past experience, which in turn lead to selective perception and evaluation of stimuli that reinforce the schema. These schemas may be helpful or dysfunctional. Because cognitions are conscious we can access them and change them, thus interrupting

the schema, and the thinking processes that maintain it, and this can then lead to changes in feelings and behaviours. For Beck, the role of the therapist is to form a collaborative, problemsolving alliance with the client, and treatment involves teaching theory and challenging cognitive 'errors' in order to develop more

helpful cognitions and schemas. Treatment also involves behavioural interventions, the purpose of these being to alter cognitions. As with behaviour therapy, the relationship itself is not considered a key factor in therapeutic change, although a good working alliance is considered essential.

Further developments in cognitive and behavioural therapy

Another key figure in the development of the cognitive therapies was Donald Meichenbaum, who, as well as making important contributions to theory, played an important role in promoting the professional acceptance of the cognitive therapies by, in 1975, establishing the 'Cognitive-Behavior Modification Newsletter' (Arnkoff and Glass, 1995). This was a significant factor in avoiding a theorydriven split, and cognitive approaches have been integrated by a majority of behaviour therapists, who now consider themselves 'cognitive behavioural' therapists. Many of the key developments in cognitive and cognitive-behavioural approaches in psychotherapy have taken place in the United States. These therapies, however, are well recognised and used worldwide. The British Association for Behavioural Psychotherapy founded in 1972 was the focus of developments in the UK, and in 1993 it changed its name to The British Association for Behavioural and Cognitive Psychotherapists to reflect the growing integration of cognitive and behavioural approaches (Lomas, 1985/2005). Partly because they are rooted in the empirical paradigm shared by natural science and medicine (Fishman and Franks, 1992) these approaches have evolved an extensive evidence base, which has facilitated their acceptance within mainstream mental health provision. A more recent advance in cognitive and behavioural psychotherapy takes account of the relative nature of experience, rather than assuming that there is an objective, rational 'truth' that a client can be taught to recognise (e.g. Guidano and Liotti, 1983; Neimeyer, 1986; Mahoney, 1990). These 'constructivist cognitive therapies' share many theoretical and technical elements with modern psychodynamic and humanistic approaches (Arnkoff and Glass, 1995: 669), and perhaps represent a move from the cognitive field towards psychotherapy integration.

Cognitive model

The cognitive model proposes that distorted or dysfunctional thinking influences a person's mood and behavior, and that such distortions are common to all psychological disturbances. Realistic evaluation and modification of thinking is used to produce rapid improvement in mood and behavior. Enduring improvement results from modification of the core beliefs underlying the dysfunctional thinking. In practice, cognitive therapy emphasizes the collaboration and active participation of both patient and therapist. It is goal-oriented, problem-focused, and time-limited, but ultimately aims to make the patient his or her own therapist. Highly structured sessions teach the patient to identify, evaluate, and respond to dysfunctional thoughts. By elucidating patterns in multiple circumstances, cognitive therapy offers the opportunity to change the underlying core beliefs and effect lasting change. Cognitive therapy is based on the cognitive model that describes the relationship between people's perceptions and interpretations of situations and their reactions (emotional, behavioral, and physiological). When people are in distress, their thinking is often characterized by faulty information processing; their perceptions are often invalid, or not completely valid. For example, a depressed woman makes only two mistakes when word processing a long document at work and thinks, "I can't do *anything* right." This thought is called an "automatic thought," because it seems to pop up spontaneously in her mind. Before therapy, she may have been only vaguely aware of these kinds of thoughts, if at all. She may have been much more cognizant of her reaction: her affect (sadness), her dysfunctional behavior (leaving work early), and/or her physiological response (heaviness in her body).

Techniques

Cognitive therapists use many different types of techniques to modify patients' thinking, mood and behavior, including

• *Cognitive:* identifying, evaluating, and modifying thoughts, images, and beliefs
• *Behavioral:* activity monitoring and scheduling, skills training, graded tasks, distraction, exposure, response prevention
• *Problem solving:* specifying problems, responding to automatic thoughts and beliefs that interfere with problem solving, brainstorming and choosing solutions, implementing solutions and evaluating outcomes
• *Emotional:* regulation of affect through engaging in self-soothing activities, relaxation, controlled breathing, distraction, seeking support, reading therapy notes
• *Physiological:* medication (if indicated), exercise, reducing caffeine and other drugs, focusing externally instead of internally
• *Interpersonal:* correcting faulty beliefs, learning communication, assertiveness and other social skills, solving interpersonal problems (bringing family members or significant others into therapy, if indicated)
• *Environmental:* weighing advantages and disadvantages of making changes in living or work environments (if indicated), responding to thoughts that interfere with making needed changes
• *Supportive:* demonstrating empathy, regard, caring, acceptance, and accurate understanding of patients' internal reality through verbal and non-verbal responses
• *Experiential:* roleplaying; using imaginal techniques to respond to automatic thoughts in the form of images; inducing images to heighten affect (to uncover key cognitions) or to reduce affect; restructuring the meaning of traumatic events through the re-experience of key memories in imaginal from in the presence of heightened affect, then using guided imagery and/or psychodrama techniques
• *Psychodynamic-like:* helping patients identify and evaluate automatic thoughts that arise and dysfunctional beliefs that become activated during the therapy session, particularly dysfunctional ideas about the therapist or therapy, then guiding them to generalize what they learned to relationships outside of therapy; drawing connections between beliefs (learned earlier in life and maintained throughout the patient's life) and his/her interpretations and reactions to current situations.

These techniques, whether they are specifically cognitive in nature or not, result in cognitive change. Much of the therapeutic work in cognitive therapy, however, *is* devoted toward directly identifying and modifying inaccurate or dysfunctional thoughts and assumptions. Therapists often use a worksheet, the *Dysfunctional Thought Record* (DTR) to help patients record and respond to their thoughts and assumptions in a structured way.

REBT

Rational emotive behavior therapy (REBT) was originated in January 1955 as a pioneering cognitive–experiential– behavioral system of psychotherapy. It is heavily cognitive and philosophical, and specifically uncovers clients' irrational or dysfunctional beliefs and actively and directively disputes them. But it also sees people's self-defeating cognitions, emotions, and behaviors as intrinsically and holistically connected, not disparate. It holds that they disturb themselves with disordered thoughts, feelings, and actions, all of which importantly interact with each other and with the difficulties they encounter in their environment. Therefore, with emotionally and behaviorally disturbed people, REBT employs a number of thinking, feel, and action techniques that are designed to help them change their self-defeating and socially sabotaging conduct to self-helping and socially effective ways. REBT theorizes that virtually all humans consciously and unconsciously train themselves to be to some degree emotionally disturbed. Therefore, with the help of an effective therapist and/or with self-help materials, they can teach themselves to lead more satisfying lives—if they choose to do so and work hard at modifying their thinking, feeling, and behaving.

THE ABCs OF RATIONAL EMOTIVE BEHAVIOR THERAPY

During the 1950s, Ellis put this constructivist theory into the now well-known ABCs of REBT. This theory states that almost all people try to remain alive and achieve basic Goals (G) of being reasonably content by themselves, with other people, productively working, and enjoying recreational pursuits. When their Goals are thwarted and they encounter Adversities (A) they are then able to construct Consequences (C)—mainly feelings and actions—that either help or hinder them satisfy these Goals. They largely (although not completely) do this by choosing to follow rational, useful Beliefs (B) or to follow irrational, dysfunctional Beliefs. Therefore, although the Adversities (A) they experience are important contributors to their emotional and behavioral Consequences (C), they do not directly or solely cause these Consequences. When at C, people feel and act dysfunctionally or self-defeatingly, their irrational Beliefs (B) and their experienced Adversities (A) bring on their disturbed reactions. So A does not by itself lead to C. A interacts with B to produce C; or A × B = C. However, people tend to be aware that C follows A, but not that B is also included in the process. They therefore think that adverse A's automatically lead to disturbed C's—that their internal reactions are controlled by external events.

REBT- CURRENT STATUS

When Ellis originated it in 1955, rational emotive therapy was unique. It was followed by somewhat similar forms of cognitive behavior therapy (CBT) in the 1960s and 1970s, particularly cognitive therapy of Aaron Beck, rational behavior therapy of Maxie Maultsby, Jr., cognitive behavior modification of Donald Meilchenbaum, and Multimodal Therapy of Arnold Lazarus. REBT was soon supported by about 300 published studies that showed its effectiveness with many different types of clients. As a result of its many successful outcome studies, REBT has become one of the most practiced psychotherapies. It is widely used with children, adolescents, couples, families, people with sex problems, and in other forms of counseling and psychotherapy. In addition, it is often used in business and industry, in education, in sports, in assertion training, in stress management, in parenting, and in many other fields. Finally, REBT has revolutionized the self-help industry and has been widely adapted in scores of best-selling books, workbooks, and audio-visual cassettes. The Albert Ellis Institute in New York, and in its many American and foreign branches, trains therapists and counselors in REBT and certifies them in its practice. To date, it and its affiliates have certified well over 5000 therapists.

Cognitive behavioural therapy (CBT)

Cognitive and behavioural therapy (CBT) is a collective description of a number of approaches. Central to this approach is the idea that identifying and changing unhelpful thoughts and behaviours results in the development of psychological well-being.

Key concepts

CBT's scientific, experimental background has and continues to have a major influence on the key concepts held. The key concepts are based on observation. Behaviour and thinking are given primary importance and, unlike many other psychotherapies, there is little conceptualisation of emotional states. The CBT therapist is not particularly interested *per se* in what the client is feeling. She is interested in what the client is thinking and doing. Classical conditioning, in which the capacity to associate a new stimuli with the effects of that stimuli and operant conditioning, contribute to the conceptual foundation. Operant conditioning conceptualises the process of change and development. The concept allows for an understanding of how people develop and claims that the consequences of behaviour havea primary influence on the behaviour being repeated. For example we will keep on doing something if we receive a positive response to what it is we do; a smile for doing something for someone results in the doer doing this more. It is now possible to develop ways to influence people through reinforcing 'good' behaviours and ignoring 'bad' ones. The problem with

the idea that positive reinforcement results in increased behaviour is that there are many examples of people doing things where the results are not particularly pleasurable. To explain this behaviourists developed the idea of habituation. We repeat behaviour when we experience some sort of gain from it. If an overly anxious person experiences a sense of security through realizing their front door is locked they might start to double-check that their front door is locked to encourage a sense of security. The behaviourists needed to incorporate the world of both thoughts and emotions into their conceptualisation for the theory to effectively explain dysfunctional emotional states. Beck's concept of schemata – cognitive structures in the mind that filter and organize incoming data – provided just the bridge behaviourists were looking for.

Therapeutic application

CLIENT GROUP :- CBT is primarily used in a one-to-one setting. The psychotherapy is symptom-based. It is practised with a wide client group demonstrating effectiveness with anxiety and mood disorders, personality disorders and psychotic states (National Institute for Clinical Excellence, 2005). The client needs to be motivated to work on themselves and to focus purposefully on symptom removal. The client and therapist define clearly the outcome desired from the therapy and thus measure its success in terms of symptom removal. A key ingredient in the effectiveness of CBT is the clients' willingness to engage in activities between sessions. This is called homework and clients will be set homework between sessions and discuss the application of the homework when in sessions.

Cognitive-behavioral therapies represent a class of pragmatic approaches to understanding and treating psychiatric disorders and problems. Although there is much diversity among these treatments, interventions are characteristically problem focused, goal directed, future oriented, time limited, and empirically based. Cognitive-behavioral theories assume that cognitive and emotional processes mediate the acquisition and maintenance of psychopathology. Accordingly, interventions effect change in symptoms, behavior, and functioning via changes in cognition (Dobson and Dozois, 2001). An impressive array of techniques has been developed to help patients learn enduring, portable skills that reduce current distress, improve current functioning, and prevent relapse. An equally impressive research literature supports the application of manual-based, cognitive-behavioral packages to a wide range of disorders.

Behavior therapies are the historical ancestors of cognitive-behavioral therapies. Theoretically allied to Charles Darwin and behaviorists such as Thorndike, Pavlov, Watson, and Skinner, behavior therapies were pioneered in the 1950s by Wolpe and Rachman, among others (Hawton et al., 1989; Craighead et al., 1995). Behavior therapies conceptualize psychopathology in terms of the elementary learning processes of classical and instrumental conditioning (Hawton et al., 1989; Mueser and Leiberman, 1995). Accordingly, the behavior therapist identifies objectively specifiable antecedents and consequences that maintain the maladaptive behavior. Therapy consists in altering environmental contingencies, which leads to change in behavior. Behavioral formulations and interventions are devoid of reference to mediational factors such as thought and cognition, which are inherently unobservable and unreliable (Skinner, 1953; Mueser and Leiberman, 1995).

By the 1970s behavioral therapies had become widely accepted efficacious treatments for a variety of psychological problems (Craighead et al., 1995). However, at this same time, several currents within the field emphasized the role of cognitive factors as mediators of behavioral outcomes: (1) covert behavior such as obsessional thought or observational learning could not be directly addressed by behavioral methods alone; (2) data emanating from the cognitive sciences posed challenges to strictly behavioral models; (3) theorist practitioners such as A. T. Beck, Ellis, and Meichenbaum began calling themselves cognitive-behavioral; and (4) research studies were published demonstrating cognitive-behavioral methods to be equivalent or better than behavioral methods for particular disorders or problems (Dobson and Dozois, 2001; Ingram and Siegle, 2001).

Cognitive-behavioral therapies can be thought to sit on a continuum in terms of how much cognition is included in the formulation: (1) on the one end are behavior therapies that focus upon behavior and environmental determinants in terms of elementary learning theory, and (2) at the other end of the continuum are therapies that formulate therapy purely in cognitive terms, allowing no behavioral intervention at all. Most cognitive-behavioral approaches fall somewhere in between, emphasizing the behavioral and cognitive interventions to differing extents.

Cognitive-behavioral techniques

Goal setting

Collaboratively setting concrete goals with the patient is an important early step that confers several advantages upon the therapeutic process of CBT (Kirk, 1989). First, goal setting helps to clarify the patient's expectations for therapy. Areas of miscommunication or misunderstanding between therapist and patient can be pinpointed and resolved at an early stage within the therapeutic interaction. Additionally, a discussion of goals may enable the patient to formulate a basis for deciding when to continue with and when to discontinue therapy. Goal setting, also, frames the patient's difficulties in terms of change and possibility, which is more hopeful than a framework that emphasizes symptoms, problems, and pain. The process of goal setting can, moreover, serve to reinforce the patient's active role within the therapeutic relationship. CBT is not a passive experience. If the client is going to benefit from treatment, full involvement in the process of therapy is required. Another advantage goal setting bestows upon the therapeutic process is structure. The patient's problems are addressed in a systematic way, and the risk that therapy will become a chaotic series of crisis interventions is reduced. Ultimately, goal setting prepares the patient for discharge, as it explicitly defines the end of therapy as the point when all of the goals are achieved. Therapy can also be terminated if little progress is made towards the goals within an agreed upon timeframe. Thus, goal setting provides a natural means to evaluate the outcome of therapy in terms of the patient's presenting problems.

Cognitive-behavioral assessment

Although most assessment takes place in the initial sessions, the process of assessment continues throughout treatment. Cognitive-behavioral assessment strategies take many forms across four domains: cognition, behavior, emotion, and physiology (Blankstein and Segal, 2001). Each assessment procedure yields specific information about a particular response system. Assessing a problem with multiple techniques produces a more comprehensive identification of the problem, and gives the therapist a better picture of how well the treatment addresses the problem (Kirk, 1989).

Cognitive-behavioral assessment often begins with an initial interview (J. S. Beck, 1995; Blankstein and Segal, 2001). During this interview, the therapist clarifies the patient's problems, formulating the difficulties in manageable units that will encourage the patient to believe that change is possible. Additionally, the assessment process helps the patient learn that variations in the intensity and distress of symptoms are predictable and potentially controllable. The assessment interview also highlights problems that should be prioritized, such as child abuse, suicidality, or problems with serious physical consequences.

The initial interview may be supplemented by a variety of other assessment techniques, including self-report questionnaires, direct observation of behavior, behavioral tests, physiological measures, and self-monitoring. Self-report questionnaires such as the Beck Depression Inventory (BDI-II; A. T. Beck et al., 1996) are easily administered and can be collected periodically throughout the therapy process. Moreover, normative data exist for many self-report questionnaires, which can help to contextualize a patient's score.

A particularly useful assessment technique involves the direct observation of behavior. This can be accomplished through frequency counts, duration of symptoms or behaviors, or observations made during role-plays with the patient. Direct observation of the problem behavior can be repeated during the course of treatment to assess change. Specific behavioral tests also provide direct observation of a wide range of problem behaviors.

Behavioral by-products (e.g., the number of cigarette butts in an ashtray, or the number of hairs pulled out by patients with trichotillomania) are indirect, objective measures that are relatively free from observer bias. While such by-products do not focus on the problem behavior itself, they do provide reliable physical evidence that the behavior has occurred. Patients are easily trained to monitor these by-products as an indication of positive or negative change. While there is accumulating support for the use of physiological measures (Kirk, 1989), they are not routinely used in clinical practice due to the prohibitive cost and availability of measuring equipment. However, less technical measurements can be used effectively, such as self-monitoring of headaches or gastric distress.

Self-monitoring

Self-monitoring is an important assessment tool. The therapist instructs the patient to observe and record her own behavioral and emotional reactions. As these reactions are distributed throughout the patient's daily life, self-monitoring tends to be employed as a homework assignment. The therapist and patient collaboratively select the target of monitoring (e.g., a symptom, behavior, or reaction) based upon the patient's goals and presenting problem list. Self-monitoring serves at least three purposes within a course of CBT: (1) it encourages and effectively trains the patient to observe her own reactions in a more scientific manner; (2) it renders a concrete record of the target symptoms and problems; and (3) new problems can become apparent and targeted for future intervention. Self-monitoring is especially useful in early sessions as a means of assessing the severity or frequency of a particular problem or symptom. However, self-monitoring is equally useful in later sessions as a means of tracking the patient's progress. Examples of self-monitoring include a record of daily activities and corresponding mood; a frequency count of the number of panic attacks per day; a record of the frequency and content of auditory hallucinations; and a food diary in which time, quantity, and type of food eaten are recorded (J. S. Beck, 1995).

Cognitive restructuring

Within the cognitive-behavioral framework, maladaptive thinking is both a symptom and a critical maintenance factor (Meichenbaum, 1995; J. S. Beck, 1995; DeRubeis et al., 2001). Negative automatic thoughts increase negative affect, which in turn increases the likelihood of further negative thought, producing a vicious cycle that tends to maintain dysphoria. It follows from this formulation that patients can overcome their problems by identifying and modifying their negative thoughts.

Within A. T. Beck's formulation (1967; A. T. Beck et al., 1979, 1985), cognitive change depends upon the patient noticing and remembering her own cognition as it occurs. Thus, the patient learns to attend to her own cognitive content as a vehicle for understanding the nature of an emotional episode or disturbance. The heuristic and therapeutic value of the cognitive model lies in its emphasis on the relatively easily accessed mental events that patient can be trained to report (DeRubeis et al., 2001). Once the patient has attended to the content of his or her cognitive reaction, she is then encouraged to view it as a hypothesis, rather than as a manifest fact. Through careful scrutiny and consideration of the belief-hypothesis, the patient gradually alters her perspective. By virtue of changing the relevant belief, change in the emotional reaction and behavior follows. The therapist will characteristically induce cognitive restructuring by asking leading questions that guide the patient to question and alter her faulty cognition (A. T. Beck et al., 1979; Overholser, 1993a,b; J. S.

Beck, 1996). This dialogue between patient and therapist is called â€˜guided discoveryâ€™ or â€˜Socratic questioningâ€™ (DeRubeis et al., 2001).

Over the course of therapy, the patient will become familiar with the process of evaluating her own thinking, applying it whenever she is confronted with new difficulties. Thus, the ultimate goal of cognitive restructuring is prophylactic: the patient acquires or refines a skill (e.g., to attend to and question her thinking), which she can apply in all domains of her life (Meichenbaum, 1995; J. S. Beck, 1995; DeRubeis et al., 2001). Cognitive restructuring is a central component of specific treatment programs for emotional disorders, personality disorders, eating disorders, and psychotic disorders.

Problem solving

Problem solving is a self-directed process by which a person attempts to identify or discover effective or adaptive solutions for specific problems encountered in everyday life. Initially, the therapist helps the patient identify and define the problems she faces. For each problem, therapist and patient brainstorm potential solutions, evaluate the quality of each solution, and test out the best ones. Problem solving also entails helping the patient identify and overcome difficulties (practical and cognitive) that she might encounter while carrying out the plan. Where testing and evaluation of possible solutions indicates that they are inappropriate, patient and therapist develop either modified or new solutions (D'Zurilla and Goldfried, 1971; D'Zurilla and Nezu, 1980; Hawton and Kirk, 1989).

Problem solving is easily learned and has been applied to a wide range of situations commonly encountered in psychiatric practice: example applications include difficulties associated with mood, anxiety, stress, substance abuse, psychotic symptoms, cancer, and other health problems (D'Zurilla and Nezu, 2001).

Behavioral activation/activity scheduling

The use of activity schedules serves to counteract the patient's loss of motivation, inactivity, and preoccupation with depressive ideas (Lewinsohn, 1974). As inactivity is associated with negative emotional states, the therapist may provide the patient with a schedule to plan activities in advance. By planning the day with the therapist, patients are often able to set meaningful goals. Comparison of the patient's record of the actual activities (compared with what was planned for the day) provides the therapist and patient with objective feedback about his achievements (A. T. Beck et al., 1979). Activities that are scheduled can come from several domains: those that were associated with mastery, pleasure, or good mood, as well as new activities that may be rewarding or informative.

Another tool that the therapist may introduce is â€˜chunking.â€™ As the patient is likely to perceive some tasks as insurmountably large, the therapist can help the patient to beak (i.e., â€˜chunkâ€™) these larger tasks into smaller, more manageable ones (DeRubeis et al., 2001). The use of â€˜graded tasksâ€™ is a related technique that the therapist may call upon in activity scheduling. Here, the patient first begins to schedule the easier or simpler aspects of larger tasks, before moving on to larger, more difficult tasks (A. T. Beck et al., 1979; J. S. Beck, 1995). Activity scheduling is used to overcome the lethargy and anhedonia of depressed patients, bipolar patients, schizophrenic patients, and eating-disordered patients.

Limitations and contraindications

It is safe to say that CBT has proved quite versatile, having been successfully applied to a wide spectrum of psychological difficulty. The limits of cognitive therapy have yet to be empirically established. However, several factors may make the cognitive-behavioral approach less effective in fact, these factors may interfere with the efficacy of any psychotherapeutic approach. Low patient motivation, unless appropriately addressed, can impede progress, especially among patients who hold beliefs that they will suffer significant adverse consequences if they comply with treatment. Patients who have positive beliefs about dysfunctional aspects of their disorder likewise need special intervention. Examples include the schizophrenic patient's grandiose

delusion (e.g., one who believes he is being persecuted because he is a great deity) and the anorexic patient's social beliefs (e.g., she is superior to others).

Even when motivation is present, the success of cognitive-behavioral methods can be hampered by mental facility. Severely retarded individuals, for example, might not be capable of the reasoning entailed in cognitive restructuring. Self-monitoring might also prove to be too demanding a task for a person with severe intellectual impairment. Behavioral methods may be more appropriate for these individuals than cognitive strategies. Psychopaths (Lykken, 1995) might also have difficulty with certain cognitive interventions; when performing a goal-directed task, they may be less able to attend to peripheral information or to self-regulate, especially under conditions of neutral motivation (Newman et al., 1997).

Finally, cultural differences may impact efficacy if therapists do not tailor the therapy appropriately. Therapists must understand, for example, how these differences may affect the building of a therapeutic alliance and how patients cultural beliefs affect their thinking and reactions. Different thinking styles and stylistic preferences must often be accommodated for patients to progress.

Dialectical Behavior Therapy

Dialectical Behavior Therapy (DBT), developed by Marsha Linehan, is a method developed specifically for the treatment of borderline personality disorder, particularly for those patients with chronic problems with suicide gestures and attempts. Combining techniques from cognitive, behavioral, and supportive approaches, the goals of the therapy are to reduce life-threatening behaviors, behaviors that interfere with the treatment process itself, and behaviors that significantly impair quality of life. DBT is based on a manual, and patients participate in both weekly individual and group therapies for 1 year. During this time, individual therapists are accessible by telephone between sessions, and the groups focus on skills training targeting interpersonal, distress tolerance, and emotional regulation issues.

Theoretical Bases

Within the DBT paradigm, the nature of borderline disturbance is thought to revolve around impaired regulation of emotions, stemming from biological sensitivity interacting with an early environment lacking in emotional validation. Linehan has defined the term dialectics as "the reconciliation of opposites in a continual process of synthesis." This applies most broadly to the notion that treatment must provide an environment of acceptance of the ways patients currently are while also trying to help them change. That is, because of the presumed roots of borderline issues, the patient's current experiences are validated by the therapist, while at the same time, problem-solving efforts are aimed at modifying maladaptive thinking patterns and information processing, and teaching new ways of coping. The therapy relationship is considered central as the laboratory for change, but also because, at times, it is the only thing that is keeping seriously suicidal patients alive.

DBT is rooted in standard cognitive and behavioral protocols, and it remains, first and foremost, a problem-solving, behavior therapy approach. Nonetheless, Linehan found that behavioral methods—though demonstrated to be effective with numerous other patient groups—required substantial modification when applied to a chronically suicidal population. First, strategies that more clearly reflected the acceptance and validation of clients' current capacities were added in order to balance the behavioral emphasis on client change. Second, the complexity and severity of the patient group suggested that a comprehensive, multicomponent treatment approach was needed. Thus, therapy was divided into different modes of service delivery, with each component emphasizing one or more of the following treatment functions: (1) enhance patient motivation; (2) enhance patient capabilities; (3) ensure that new capabilities generalize to the patient's natural environment; (4) enhance therapist capabilities and motivation to treat patients effectively; and (5) structure the environment in a manner that will promote and reinforce patient and therapist capabilities.

DESCRIPTION OF TREATMENT

Levels of Disorder and Stages of Treatment

Unlike many other empirically supported, manualized psychotherapy approaches, DBT does not provide a session-by-session protocol for how treatment should proceed. Rather, DBT is designed to be a highly flexible model that can be readily applied to patients with disorders of varying severity and complexity. Linchan conceptualized four levels of disorder, each level having a corresponding stage of treatment. Level 1 disorders are characterized by behavioral dyscontrol and typically include disorders that are both severe and pervasive. The overall goal of Stage 1 treatment for Level 1 disorders is to help these patients increase control over their behavior and their lives.

The second level of disorder is best described as "quiet desperation" (as opposed to the state of "loud desperation" in Level 1). The central problem at this level is avoidance of emotions and any environmental cues associated with them.

The goal of Stage 2 DBT is to increase the patient's ability to experience emotions without trauma. Level 3 disorders are best described as problematic patterns in living that interfere with goals. Thus, treatment at this stage focuses on achieving ordinary happiness and a stable sense of self-respect. Finally, the fourth level of disorder refers to those individuals who have a lingering sense of incompleteness despite the fact that they are otherwise satisfied with their lives. The goals of Stage 4 DBT include developing the capacity for sustained joy via psychological insight, spiritual practices, and an expanded awareness of oneself.

Treatment Strategies

DBT addresses all problematic client behaviors and therapy situations in a systematic, problem-solving manner. The behavioral analyses and change procedures involved in problem solving are surrounded by validation of the client's experiences, especially as they relate to the individual's vulnerabilities and sense of desperation. Furthermore, in contrast to many behavioral approaches, DBT places great emphasis on the therapeutic relationship as a mechanism of change. In times of crisis, when all else fails, DBT uses the relationship itself to keep the client alive. Treatment strategies in DBT are divided into four sets:

(1) Dialectical strategies, (2) core strategies (problem solving and validation), (3) communication strategies (irreverence and reciprocal communication), and (4) case management strategies (consultation to the patient, environmental Intervention, and supervision/consultation with therapists). In all treatment strategies, a DBT therapist must constantly strive for a balance of acceptance and change.

1. Dialectical Strategies

Dialectical strategies are woven throughout all treatment interactions. The most fundamental dialectical strategy is the balanced therapeutic stance—the constant attention to combining acceptance with change.

The goal is to bring out the opposites, both in therapy and the client's life, and to provide conditions for synthesis. Strategies include extensive use of stories, metaphor, and paradox; the therapeutic use of ambiguity; viewing therapy, and indeed all of reality, as constant change; cognitive challenging and restructuring techniques; and reinforcement for use of intuitive, nonrational knowledge bases.

2. Core Strategies

Core strategies consist of the balanced application of problem-solving and validation strategies. The problem-solving strategies are central to treatment and are drawn from behavioral psychology principles as described earlier. Included are a wide variety of behavioral assessment and behavioral therapy techniques that are used to directly target maladaptive behaviors. Problem solving is a two-stage process, involving, first, an analysis and acceptance of the current problem, and second, an attempt to generate, evaluate, and implement alternative solutions that might have been made or could be made in the future in similar problematic situations.

3. Communication Strategies

In DBT, the therapist balances two communication strategies, which represent rather different interpersonal styles. The modal style is the reciprocal strategy, which includes responsiveness to the client's agenda and wishes, warmth, and self-disclosure of personal information that might be useful to the client as-well as immediate reactions to the client's behavior.

4. Case Management Strategies

There are three strategies designed to guide each therapist during interactions with individuals outside the therapy dyad. First, the consultant-to-the-client strategy is the application of the principle that the DBT therapist teaches the client how to interact effectively with the client's environment rather than teaching the environment how to interact with the client. The third strategy, the consultation-to-the-therapist strategy, requires that each DBT therapist meet regularly with a supervisor or consultation team.

Schema-Focused Cognitive Therapy

Description

As an extension of Beck's cognitive therapy for personality disorders, Jeffrey Young and his colleagues formulated the concept of an "early maladaptive schema," defined as "a long-standing and pervasive theme that originates in childhood; defines the individual's behaviors, thoughts, feelings, and relationships with other people; and leads to maladaptive consequences." Applied specifically to characterological issues, the goal of schema-focused therapy is to help patients to identify cognitive distortions and challenge underlying beliefs that routinely result in impaired psychosocial functioning. For example, one might hold to a maladaptive schema centered on the fear of being abandoned, which results in excessive jealousy and clinging in relationships. The therapeutic approach would be to assist the patient in uncovering his or her assumption that significant others will inevitably leave and the connection of that assumption to particular behavioral and emotional responses. Over time, the patient is encouraged to develop other ways of relating to replace the counterproductive modes, accompanied by shifts in thinking and affective reactions.

2. Theoretical Bases

Maladaptive schemas arise as deeply entrenched patterns of response that developed as the child tried to organize personal experience of himself or herself and others in a world that may have been filled with abuse, instability, or neglect. Although they may have served as logical solutions in childhood, they remain in play in adulthood as ineffective means for meeting basic security and intimacy needs, and, thus, are associated with negative emotions and impaired functioning. David Bricker, Jeffery Young, and Catherine Flanagan have proposed three domains, or groupings, of schemas: instability and disconnection, impaired autonomy, and undesirability.

<u>Cognitive analytic therapy</u>

As the name implies, CAT evolved as an integrative therapy based on ideas from cognitive and analytic therapies. It was originated when Tony Ryle, a psychiatrist and analytically trained psychotherapist, was undertaking research into psychotherapy practice using repertory grids. The CAT practitioner aims to work with the patient to identify procedural sequences; chains of events, thoughts, emotions and motivations that explain how a target problem (for example self-harm) is established and maintained. In addition to the procedural sequence model, a second distinguishing feature of CAT is the use of reciprocal roles (RRs). These identify problems as occurring *between* people and not *within* the patient. RRs may be set up in early life and then be replayed in later life; for example someone who as a child felt neglected by parents perceived as abandoning might be vulnerable to feelings of abandonment in later life (or indeed neglect themselves). Subsequently CAT has been influenced by ideas from the work of Vygotsky and Bakhtin. From Vygotsky come concepts such as the Zone of Proximal Development (ZPD) and scaffolding.351 The ZPD implies that

new tasks set for the patient (for example, tolerating anxiety about social situations) should extend what they do beyond their current capabilities, but only by a small and achievable amount. Scaffolding involves the therapist providing support for the patient's efforts to change, but varying this level of support as the patient's needs change.

Bakhtin provided concepts such as dialogism from which come techniques such as Dialogical Sequence Analysis. This is a structured attempt to identify and visually display sequences of behaviour, thinking, and emotions so that the patient becomes more aware of these and can start to modify them.

The approach is always time-limited, typically taking place over 8-24 weekly sessions (the precise number being agreed at the start of therapy). Sixteen sessions is probably the most common length. In the first quarter of the therapy (the **Reformulation** phase) the therapist collects all the relevant information, asking the patient about present day problems and also earlier life experiences. At that point the therapist writes a reformulation letter to the client. This letter summarises the therapist's understanding of the client's problems. Particular attention is given to understanding the connection between childhood patterns of behaviour and their impact on adult life. The letter is agreed between patient and therapist and forms the basis for the rest of the work.

After the reformulation letter the patient may be asked to complete diaries or rating sheets to record the occurrence of problems and their context. During this period (known as the **Recognition** phase) patient and therapist construct a diagrammatic formulation to illustrate the unhelpful procedures which maintain problems for the patient. The aim of this phase is to enable the patient to recognise when and how problems occur.

In the second half of the therapy work moves into the **Revision** phase, where patient and therapist identify and practice "exits" from the procedural diagram established in the previous phase. For example a problematic procedure might move a patient from feeling angry to taking an overdose. An exit might involve expressing the anger in some way as an alternative to self-injuring behaviour.

At the end of the therapy, patient and therapist each write "goodbye letters" which they exchange, summarising what has been achieved in the therapy and what remains to be done. After the end of the agreed number of weekly sessions, planned follow-up sessions take place to monitor and support the changes that have been made. Typically, a 16-session CAT might be followed up by a single session one month after the end of therapy, and a final one three months later.

UNIT 7 – SYSTEMIC THERAPIES

Family therapy

Family therapy is any attempt to modify salient environmental features, most importantly interpersonal contacts or beliefs about those contacts, which alter interaction patterns, allowing the presenting problem to be unnecessary. Notice that family therapy does not necessarily exclude nonfamily members, nor does it necessitate that all family members be present in therapy. Problems are assumed to be embedded in behavioral patterns, and patterns are determined by context. Family therapy, then, exists to alter context. From this viewpoint, the job of the therapist is to alter the context driven behavior.

Like other forms of psychotherapy, during the first session the objective is to join with the client and assess the problem. However, unlike other forms of psychotherapy, family therapy also begins by attempting to alter the family’s perspective. Almost immediately an effort is made to dilute the idea that the presenting problem associated with an individual is encased within that person; instead an attempt is made to modify

beliefs so that the problem is perceived to be a byproduct of the situation that characterizes the system (i.e., environment) housing the individual. In effect, an initial objective is to have the problem viewed differently by the family. Family therapy then, especially initially, occurs through social negotiation. It involves determining what the family wants, how they see the problem, how they want it fixed relative to the assumptions and orientation of the family therapist, and how well the therapist can pull the problem away from an individual and distribute it to the system. Once this has commenced, the therapist then allows the presenting problem to be unnecessary by altering the family structure, which simultaneously alters interaction patterns, relationships, and beliefs about the family. Alternatively, the therapist modifies the beliefs about the relationship, which simultaneously alters patterns of interaction, and the family structure. Simply said, the therapist enters the family at the relationship level, not at the level of a particular individual, and alters interaction associated with the various relationships.

1. Initial Contact

Everyone relevant to the presenting problem is asked to attend the initial session. This includes all adults in the household, and all siblings. Each individual brings unique information about the family beliefs and the presenting problem. Subsequent sessions require various combinations of family members as dictated by the therapist needs.

2. First Session

In family therapy at least three features about the presenting problem are examined: (1) timing, (2) function, and (3) who. Each provides an important clue to discerning relevant contextual information associated with the problem.

Implementing Treatment: Dimensions of Family Therapy

It is assumed that the therapeutic process begins at first contact, and from that moment on, the distinction between assessment and prescribed change is blurred as these interdependent features continuously address the presenting problem. Instead of thinking of therapy changing over time (e.g., phases), it is better to think of therapy as containing three interdependent dimensions occurring simultaneously over the period of treatment. These dimensions are assessment, instilling doubt, and pattern change.

1. Assessment

Assessment includes determining relevant behavioral patterns and belief systems in the family. Assessment occurs throughout treatment; the therapist's concept of the problem is continuously updated using information gathered via questions and responses to tasks. Assessment is in two areas: interaction patterns and belief structures. Assessing behavioral interaction patterns include questions about who does what and when, and who responds to whom, when, and how?

2. Instilling Doubt

Simultaneous to assessment, the therapist begins the process of subtly casting doubt on the validity of the family's current belief system about the presenting problem. Seldom will the family acknowledge that marital or family interaction patterns produce the presenting problem, or even that the problem lies outside of the identified patient. Typically, the therapist addresses the interactional and contextual dynamics surrounding the presenting problem. This occurs in several ways; for example, perspective altering questions, reframes, and directives for in-session interaction.

3. Pattern Change

Implicit or explicit requests for change in the behavior patterns that are associated with the presenting problem characterize this dimension of therapy. Using the removal of the presenting problem as the goal, decisions are made about what needs to be changed, and how to shift the belief structure to allow the desired behavior to occur. Although the presenting problem is the defining reason for being in therapy, short-term

goals for smaller, less volatile behavior coax the individual or family toward more desirable behavioral patterns along with an altered family belief structure. This implies that as assessment is occurring and doubt is cast upon the existing beliefs, opportunities are offered for behavior change.

Applications and Exclusions

Because family therapy does not attempt to modify a person but rather the behaviors exhibited by a system, it is generally applicable to most of the problems seen by mental health practitioners. With children and adolescents, it has been applied to problems ranging from conduct disorder to anxiety and depression. In adults, it has been applied to relationship problems, as an adjunct to the treatment for schizophrenia, depression and anxiety, modifying family reactions to medical illness, and drug and alcohol abuse.

Group psychotherapy

Group psychotherapy is a widely used treatment modality. However, there are many types of groups for different conditions and often several models of groups for a particular condition. This makes a description of the field somewhat complex, and even the term "group therapy" relatively lacking in meaning except that several people are meeting together. Group psychotherapy is widely used as an intensive treatment modality to address psychological issues especially those involving unsatisfactory interpersonal patterns. Group therapy is also used in a more structured manner to address negative cognitions, to learn skills such as assertiveness, and for psychoeducational purposes. This wide range of group applications involves the use of groups that are conducted in quite different ways.

THEORETICAL BASES

There is a clearly articulated theoretical basis for group therapy based on general systems theory. The major difference between individual therapy and group therapy from a process viewpoint is the presence in groups of a range of relationships rather than the single relationship of individual therapy. This makes groups a more complex treatment system and has led to the idea of therapy through the group process as opposed to therapy of the individual in the group or therapy provided solely by the leader. The role of leader in a group is quite a different task than the role of the individual therapist. The leader must be monitoring each member as well as the general climate of the group.

APPLICATION

A. Group Development

There is substantial evidence from the group process literature for the phenomena of group development. Indeed, research findings indicate that those groups that follow a developmental sequence produce better outcome for the members. The most basic description of group development consists of a four-stage model of engagement, conflict, interpersonal work and termination. Additional stages have been described that would fit either as subunits under the broad heading of interpersonal work or as identifying issues regarding greater independence from the leader. The group development perspective is a powerful metaposition from which to understand group-level events. Assessment and preparation tasks are discussed first for, although they are not formally part of the group development sequence, they are part of the early tasks that the leader must address.

1. Assessment

The first set of tasks in assessment is to identify the predominant diagnostic difficulties. The purpose of this is to establish what treatment approach will be most likely to be effective in this specific clinical setting. A closely related task is to match to the extent possible what type of group (or other treatment) is likely to be most effective. The goal is to maximize the accuracy of prediction of who will benefit from what type of

treatment. The second set of assessment objectives is to develop a focus for treatment. This will begin with a review of the most troublesome symptoms.

2. Preparation

The empirical literature is clear that careful preparation leads to early group cohesion and therefore to fewer early dropouts and better eventual outcome. A systematic introduction to the nature of group therapy is indicated. This can be done verbally or preferably with the assistance of a brief handout. This might include mention of the research basis for group therapy and a brief description of how to get the most out of the experience. Some of the myths about groups may be discussed.

3. Developing a Focus for Treatment

The choice of issues to focus on is a central task in the pregroup interviews. The therapist can function as a helpful technical resource always trying to cite the patient's own words in describing issues. The use of descriptive language rather than interpretive comments makes it more palatable to discuss dysfunctional patterns. Being direct about this task brings relief at being understood and often clarifies the patient's own partial awareness of difficulties. If questionnaires are used it is helpful to give the patient copies of the results.

4. Engagement Stage

The initial task when starting a new group is to create a sense of membership in the group. The members of successful groups report an increasing feeling of belongingness over the first few sessions. The leader should take a modestly active stance particularly directed at stimulating inter-member communication patterns. This can be done in an unobtrusive manner by brief interventions such as "Have others had this sort of experience," "John, you look as if you understand what Mary is talking about," and so on. This simple strategy will facilitate interaction between members and away from the leader. This promotes the emergence in the group of important supportive therapeutic factors.

5. Conflict Stage

The positive collaboration of the engagement stage then shifts to an atmosphere of interpersonal tension that is characterized by a more confronting and challenging quality that may have an angry or resentful component. The essential task is to develop patterns for conflict resolution and tolerance of a more negative atmosphere. This stage is a necessary component for the developing group. The term "differentiation" has been used to emphasize the importance of self-assertion and self-definition that underlie the process. The interactional themes tend to shift from ones of commonalties to those of differences. This addresses the potential liability that the engagement stage group may have of avoiding difficult but important issues. The challenge within the group is accompanied by a parallel internal challenge regarding negative or shameful feelings about self. Generally, the confrontive style is also demonstrated toward the therapist as the collective group seeks to differentiate itself from the leader. This includes a reworking of group norms that now become the "property" of the group, not just the leader. This stage often has a somewhat adolescent quality of challenge for challenge's sake.

6. Interpersonal Work

The group is now equipped to address individual problematic matters in a more vigorous manner. The focus tends to shift to greater introspection and personal challenge. This promotes increased closeness among the members as they align in the exploration of more difficult common issues. This stage of the group is likely to stimulate themes central to interpersonal functioning such as tolerance of intimacy, management of control–dependency in relationships and fears of becoming over-involved in relationships and losing a sense of self.

7. Termination

This stage may occur at varying points in the development of a group depending on the circumstances promoting or impeding development. It will have greater salience in groups that have developed a strong interactive milieu, that have met at least for several months or in an intensive format, and that end together.

Endings are often a problematic area for people seeking psychotherapy. It is common for complicated grief issues to emerge; some members may experience acute anxiety around themes of abandonment or rejection, or of not getting enough. Termination confronts the members with the issue of assuming responsibility for managing one's self. These termination themes incorporate basic maturational tasks that are central to the human condition.

8. External Structural Factors

Clinical experience suggests that numerous external factors will have a significant impact on the process of group development. The number of sessions available will in part determine how far the group can progress. For example, a group restricted to six or eight sessions is unlikely to move beyond engagement tasks because it will shift directly from engagement to preparing for termination. Inpatient groups or crisis intervention groups that have changing membership almost every session will not be able to progress beyond the engagement stage. Highly structured groups that are designed for psychoeducational purposes, or for specific skill training such as assertiveness, may be limited in the amount of interaction that occurs and will therefore show weak developmental progress. The therapist for such brief groups may need to be relatively active to keep the group at the appropriate level. The leader needs to calibrate interventions therefore toward encouraging early-stage tasks that make full use of the supportive cluster of therapeutic factors that encourage group interaction and promote cohesion around addressing problems or topics that are external to the group itself. Trying to push such groups forward into more conflictual or introspective work will inhibit achievement of the goals of the group and may lead to dropouts.

Comparison of Time-Limited Group Models

Model features	*Support groups*	*Group cognitive-behavioral therapy (GCBT)*	*Interpersonal therapy for group (IPT-G)*	*Yalom model interpersonal psychotherapy*	*Psychodynamic group psychotherapy*
Time frame	Open ended	Time limited	Time limited	Time limited Longer term	Time limited Longer term
Group entry	Open	Closed	Closed	Open/closed	Open/closed
Composition criteria	Variable, often by condition or circumstance	Common diagnosis or situation (i.e., binge eating, depression)	Common diagnosis or situation (i.e., binge eating, depression)	Common interactional capacity	Common interactional capacity
Formal pregroup preparation	Low	Moderate/high (Incorporated into early sessions)	High (Incorporated into early sessions)	Low/moderate	Low/moderate
Therapist style	High activity	High activity	Moderate activity	Low/moderate activity	Low/moderate activity
Group structure	Moderate/high	High (Programmed sessions)	Moderate (Problem areas/goals established and actively kept in focus)	Low/moderate	Low/moderate
Extragroup socializing	Encouraged	Permitted, perhaps encouraged re assigned tasks	No	No	No
Process focus	Variable	Low group process focus; (managed to preserve a teaching environment)	Moderate group process focus (managed to preserve group integrity, not interpreted)	High "here-and-now" group process focus (interpreted re interpersonal conflict)	High "here-and-now" group process focus (interpreted re interpersonal and intrapsychic conflict)
Homework	Informal	Written homework and behavior change expected	Changing interpersonal/social patterns expected	Not formally prescribed	Not formally prescribed
Mediating strategies	Variable: Adapted to the common composition criteria	Identify and block negative cognitions	Identify and alter current interpersonal/social coping	"Here-and-now" interpersonal learning existential awareness	Identify and understand interpersonal and intrapsychic conflicts
Focus on affect	Variable	Low/moderate	Moderate (identification of excessive or blocked affect re current interpersonal tensions)	Moderate/high	Moderate/high

Interpersonal therapy

IPT is a time-limited, dynamically informed psychotherapy that aims to alleviate patients' suffering and improve their interpersonal functioning. IPT focuses specifically on interpersonal relationships as a means of bringing about change, with the goal of helping patients to either improve their interpersonal relationships or change their expectations about them. In addition, IPT also aims to assist patients to improve their social support network so that they can better manage their current interpersonal distress. IPT was originally developed in a research context as a treatment for major depression, and was codified in a manual developed by Klerman and colleagues in 1984. Since that time, a great deal of empirical evidence supporting its use has accumulated.

Characteristics of IPT

IPT is characterized by three primary elements: (1) IPT focuses specifically on interpersonal relationships; (2) IPT is time limited as an acute treatment; and (3) the interventions used in IPT do not directly address the transference relationship.

Treatment Process

IPT can be divided into assessment (described later), initial sessions, intermediate sessions, treatment conclusion, and maintenance treatment phases. During each, the clinician has a well-defined set of tasks to accomplish. Undergirding these therapeutic tasks and techniques is the stance taken by the therapist. Clinicians should be active and maintain the focus of therapy. The therapist should also be supportive, empathic, and strongly encouraging, and should make every effort to convey a sense of hope to the patient and to reinforce his or her gains.

1. Initial Sessions

During the initial sessions of IPT (usually the first one or two meetings following the general assessment) the therapist has three specific tasks. These are (1) to conduct an interpersonal inventory; (2) to work collaboratively with the patient to determine which problem areas will be the focus of treatment; and (3) to develop a treatment contract with the patient. The interpersonal inventory is a brief description of the important people in the patient's life, and for each includes the amount and quality of contact, problems in the relationship, and the expectations about the relationship.

2. Intermediate Sessions

During the intermediate sessions of IPT, the patient and therapist work together to address the interpersonal problems identified during the assessment. In general, work on these issues proceeds in the following order:

(1) identification of a specific interpersonal problem; (2) a detailed exploration of the patient's perception of the problem, including communication patterns and expectations about the relationship; (3) collaborative brainstorming to identify possible solutions to the problem; (4) implementation of the proposed solution; and (5) reviewing the patient's attempted solution, with positive reinforcement of the changes made and discussion of refinements to be carried out.

3. Completing Acute Treatment

Acute treatment with IPT comes to an end as specified by the therapeutic contract. There are both theoretical and practical reasons for keeping acute treatment with IPT time-limited. The time limit is very effective in generating change, as it often compels patients to work more rapidly on improving communication skills and building more effective social networks. In addition, the time limit influences both patient and therapist to focus on acute symptoms rather than on personality change. Moreover, to extend IPT beyond the acute time frame may lead to the development of problematic transference, as the relationship between patient and therapist assumes greater importance.

4. Maintenance Treatment

Rather than using the traditional psychoanalytic model in which "termination" is a complete severing of the therapeutic relationship, concluding acute treatment with IPT does not signify the end of the therapeutic relationship. In fact, in IPT it is usually agreed that there will be therapeutic contacts in the future, and provision is specifically made for these. Clinical experience, theory, and empirical evidence all make clear that IPT should be conceptualized as a two-phase treatment, in which a more intense acute phase of treatment focuses on resolution of immediate symptoms, and a subsequent maintenance phase follows with the intent of preventing relapse and maintaining productive interpersonal functioning. Therefore, the therapist should specifically discuss future treatment with the patient prior to concluding therapy. A specific contract should be established with the patient for the specific alternative for the provision of maintenance treatment that is chosen.

THEORETICAL BASIS

IPT is grounded in attachment theory, which as described by Bowlby among others, rests on the premise that people have an instinctual and biological drive to attach to one another. When crises occur, individuals seek reassurance and care from those important to them. Interpersonal communication is intrinsic to this process, and individuals who cannot effectively ask for care, and consequently cannot obtain the physical and psychological care they need, will suffer as a result. When interpersonal support is insufficient or lacking during times of stress, individuals are less able to deal with crises and are more prone to develop psychiatric symptoms. Bowlby described three different types of attachment styles that drive interpersonal behavior. Secure attachment describes individuals who are able to both give and receive care, and are relatively secure that care will be provided when it is needed.

Anxious ambivalent attachment, in contrast, is a style in which individuals behave as if they are never sure that their attachment needs will be met. Because of this, such individuals believe that care must be sought constantly. Such individuals often lack the capacity to care for others, since their concern about getting their own attachment needs met outweighs all other concerns. Individuals with anxious avoidant attachment typically behave as if care will not be provided by others in any circumstances. As a result, they avoid becoming close to others. The paucity of their social connections, along with their tendency to avoid asking for help during times of crises, leaves these individuals quite prone to difficulties.

APPLICATIONS AND EXCLUSIONS

The purpose of conducting an assessment is to determine when IPT should be used, and to whom it should be applied. The assessment may take several sessions to complete. It is only after the assessment, and the determination that the patient is suitable, that IPT should formally begin. During the assessment, the therapist should evaluate the patient's attachment style, communication patterns, motivation, and insight. Assessment of DSM-IV diagnoses should also occur. IPT should not, however, be restricted only to patients with DSM Axis I diagnoses; it is quite suitable for patients with a variety of interpersonal problems such as work conflicts or marital issues. In fact, because patients without major psychiatric illness often have more secure attachments and better social support networks, they are usually able to utilize IPT very effectively. Special attention should be paid to patients diagnosed with personality disorders. Those with cluster A disorders including paranoid, schizoid, and schizotypal personality disorders may be unable to form effective alliances with their therapists in short-term therapy, whereas those with severe cluster B disorders such as narcissistic, histrionic, borderline, and antisocial personality disorders may require more intensive therapy than can be provided in an IPT format. However, many patients with depression or anxiety superimposed on a personality disorder may benefit a great deal from short-term therapy with IPT if the focus is on the treatment of the depression or anxiety rather than on personality change.

Sex therapy

Although difficulties with sexual functioning have undoubtedly existed throughout time, early theories of the development and treatment of sexual dysfunctions first began to appear in the late 19th and early 20th centuries. In 1902, Richard von Krafft-Ebing published *Psychopathia Sexualis,* a book that addressed the existence of dysfunctions and deviations alike. Krafft- Ebing theorized that many sexual disorders resulted from the improper use of sexual energy, creating a state of moral degeneracy. For men, this loss was conceptualized as a waste of semen on nonreproductive activities such as childhood masturbation and excessively frequent sexual activity in adulthood. Mental health professionals at that time advocated preventive treatment through the use of restraining devices that inhibited children's masturbation, such as metal mittens, and through the maintenance of a bland diet in adulthood to avoid overstimulation of the senses.

Sigmund Freud presented a different view of sexual dysfunction in 1905 with the introduction of the Oedipal and Electra complexes. Sexual disorders were thought to be the result of failing to resolve these complexes and becoming fixated at an immature stage of psychosexual development. Treatment did not involve direct attention to sexual functioning, but instead revolved around indirect psychoanalytic approaches, such as insight attainment and transference techniques.

Behavioral Sex Therapy

Despite the failure of Freudian therapy in the successful treatment of sexual dysfunction, it was nearly 50 years before Freud's views were challenged by early behaviorists. These early behavioral psychologists posited that anxiety functioned to inhibit normal sexual arousal, resulting in dysfunction. Treatment consisted of techniques to reduce anxiety such as progressive relaxation and systematic desensitization.

In 1966 and 1970, William Masters and Virginia Johnson published their monumental works on the etiology and treatment of sexual dysfunctions. Masters and Johnson expanded on the theories of early behaviorists by stressing an informal social learning theory approach that emphasized the roles that negative messages about sexuality, lack of knowledge about sexuality, and traumatic first sexual experiences play in sexual functioning. In addition, they introduced the concept of sexual problems as self-maintaining cycles of dysfunctional sexual behaviors mediated by anxiety. They theorized that following a negative sexual experience, an individual might develop an anxious, self-evaluative spectator role that interferes with the normal sexual response cycle and results in the maintenance of sexual dysfunction. In addition to the anxiety reduction techniques utilized by early behaviorists, Masters and Johnson included the instruction of specific sexual stimulation techniques in their treatment protocol.

Later additions to the Masters and Johnson model for sex therapy have included elements of both cognitive and systemic theory. An emphasis was placed on a patient's thinking about sex, with various cognitive distortions becoming a major focus of treatment. Moreover, couple systemic considerations such as power struggles and difficulties with intimacy and trust have increasingly been included in both etiological considerations and treatment focus in a variety of sexual dysfunctions.

Post-modern Sex Therapy

Post-modern sex therapy is a therapeutic approach that uses behavioral, cognitive, and systemic methods to treat a variety of sexual disorders that affect both males and females. This approach is based on a theoretical foundation in which the etiology of sexual dysfunctions is considered to be multifaceted, with both psychological and physiological factors contributing to the onset of a particular disorder. As a result, both diagnosis and treatment of sexual dysfunctions must be considered in the context of a combination of behaviors, thoughts, and emotions.

BEHAVIORAL, COGNITIVE, AND SYSTEMIC TREATMENT FOR SEXUAL DYSFUNCTIONS

In 1997, LoPiccolo outlined nine general principles of sex therapy through which couple change takes place. First, the sexual dysfunction is conceptualized as a disorder of the couple, with both partners bearing a mutual responsibility for treatment. It is important to take the blame out of sexual disorders and facilitate a team approach to treatment.

Second, in many cases couples have limited knowledge of the basic anatomy and physiology of human sexual response. Consequently, it becomes necessary for the therapist to provide an informational and educational component to treatment. Clients often need to be educated about the process of sexual arousal in both males and females, as well as about human anatomy.

The third principle of sex therapy is to foster an attitude change in clients who have negative attitudes toward the expression of their sexuality. As noted earlier, negative attitudes can come from a variety of sources, including the family of origin.

A fourth mechanism of change in sex therapy is the elimination of performance anxiety. Many individuals who experience problems in sexual functioning possess negative self-fulfilling expectancies about their sexual abilities and how their sexual encounters will progress. There are many strategies used in sex therapy to help clients begin to enjoy the process of sex as opposed to focusing on sexual goals. Participants in therapy are taught that worrying about the outcome of sex guarantees that they will not attain their "goal," because it directly interferes with arousal. Instead, the emphasis is on enjoying the process of sex.

A fifth goal of sex therapy is to increase communication skills in the couple. Dysfunctional couples often have many communication deficits that directly affect their ability to have satisfying sexual relationships.

Changing destructive sex roles and lifestyles is the sixth mechanism of change in post-modern sex therapy. Many people have developed life habits that indirectly interfere with sexual functioning in a number of ways. Problems with extended family, children, and careers can all interfere with the sexual lives of adults. In addition, the sex roles that people acquire can interfere with sexual expression in a relationship.

Seventh, sex therapists must often help couples to change disruptive marital systems and enhance the marital relationship. Commonly, sexual dysfunctions are present in unhappy marital relationships. As such, it is unrealistic to believe that a couple can leave behind their disagreements about parenting, finances, or other issues while working solely on sexual issues.

Physical and medical interventions are sometimes needed to restore healthy sexual functioning. Many medical diseases can interfere with sexual functioning, including diabetes, heart disease, and neurological conditions such as spinal cord injury, multiple sclerosis, and pituitary/hypothalamic tumors. In addition, many prescribed medications can interfere with the arousal process. Antihypertensive medications and psychotropic medications such as antianxiety, antidepressant, and antipsychotic agents have been shown to interfere with arousal and orgasm for both men and women. Similarly, recreational drugs such as alcohol, marijuana, heroin, and cocaine can negatively impact sex drive, sexual arousal, and orgasm. Finally, hormone levels in the body can also have major effects on sexual functioning. Disruptions in the levels of testosterone, estrogen, and prolactin can suppress sex drive and negatively impact sexual functioning for men and women. As a result, team approaches to sex therapy are often warranted, with the therapist and a medical practitioner working together to improve sexual functioning.

Perhaps the most distinctive element of sex therapy is the behavioral component: changing sexual behavior and teaching effective sexual techniques. Stanley Althof outlined several goals of behavioral techniques in sex therapy in 1989. These goals include overcoming performance anxiety, altering the previously destructive sexual system, confronting resistances in each partner, alleviating the couples' anxiety about physical intimacy, dispelling myths and educating clients regarding sexual function and anatomy, counteracting negative concerns with body image, and heightening sensuality

After the completion of the assessment portion of therapy, some simple behavioral interventions are made that are common to all of the dysfunctions. First, the couple is asked to refrain from attempting sexual intercourse until it is prescribed by the therapist. Many sex therapists believe that this prohibition of coitus serves to alleviate performance anxiety, allowing the couple to rebuild their sexual relationship from the beginning. Next, the couple is typically asked to complete a series of sensate focus exercises. Sensate focus is a behavioral intervention that focuses the clients' attention on the sensuality of the body, without the pressure attendant upon sexual behaviors such as intercourse. These exercises include sensual touching of the clients' own bodies or their partners', and consist of caressing, hugging, kissing, and body massage. Participants in sex therapy are instructed to tune in to their sensual response to the touching. Sensate focus exercises serve to reduce anxiety in a number of ways, including providing a no-demand experience to the couple, eliminating the "spectator role," and increasing dyadic communication by giving feedback to the partner about what feels good. Breast and genital contact, intercourse, and orgasm are not allowed as part of these exercises.

Gender-Specific Sexual Dysfunctions

1. Male Sexual Dsyfunctions

a. Male Erectile Disorder

Male erectile disorder is characterized by an inability to attain or maintain an erection, resulting in an incapacity to complete sexual activities such as intercourse. They are taught the "tease technique" in which the couple is instructed to cease genital contact if the male should attain an erection. The couple can resume penile contact only after the erection is lost. This exercise teaches clients that erections occur naturally in response to stimulation, as long as the couple does not focus on performance. The male is paradoxically instructed throughout sensate focus that, "The purpose of this exercise is for you to learn to enjoy sensual pleasures, without focusing on sexual goals. Therefore, you should try to not get an erection." This demand to not get an erection frees the male to enjoy sensual situations without the accompanying anxieties that have worked against attaining an erection in the past. After the couple experiences this process several times, they move on to intercourse. Intercourse is also attempted in several steps.

b. Premature Ejaculation

Premature ejaculation is defined as the persistent onset of orgasm and ejaculation with minimal stimulation, before or shortly after intromission occurs. The standard treatment for premature ejaculation involves the "stop-start" or "pause" procedure, introduced by Semans in 1956. With this procedure, the penis is manually stimulated until the man is highly aroused and he feels that ejaculation is imminent. At this point, the couple stops stimulation until the arousal subsides, and they resume stimulation again when the male no longer feels that ejaculation is imminent. The "squeeze" technique can also be added to help the male delay ejaculation during manual stimulation. In this procedure, the female partner firmly squeezes the penis between her thumb and forefinger, at the place where the head of the penis joins the shaft. After a few weeks of this training when the necessity of pausing diminishes, the focus of behavioral exercises shifts to include intercourse. The couple continues to practice a modified stop-start technique in which the penis is placed in the vagina without any thrusting movements. The most effective intercourse position during this period of treatment is the woman on top position. The "quiet vagina" exercise is utilized, and the male partner is encouraged to make no movement but to feel free to engage in erotic touching of his partner. If this stimulation produces high levels of arousal and a feeling of ejaculatory inevitability, the penis is withdrawn and the couple waits for arousal to drop off. When good tolerance for inactive containment of the penis is achieved, the training procedure is repeated during active thrusting exercises with a variety of sexual positions. After 2 to

3 months of practice, males who undergo treatment for premature ejaculation are generally able to enjoy significantly prolonged intercourse without the need to use pause and squeeze techniques.

c. Male Orgasmic Disorder

Male orgasmic disorder is present when a man experiences a recurrent delay in, or absence of, orgasm following a normal phase of sexual excitement. Treatment of male orgasmic disorder is based on many of the standard strategies used with other sexual dysfunctions. Eliminating performance anxiety and ensuring adequate stimulation through paradoxical sensate focus exercises are the basis for treatment. The couple is instructed that during sex the penis is to be caressed manually and/or orally until the man is aroused, but that stimulation is to stop whenever he feels he might be close to orgasm. This procedure takes the focus of sex off of orgasm, and paradoxically, allows the man to fully enjoy the pleasurable sensations of stimulation. Additionally, in 1977 LoPiccolo reported that the use of electric vibrators, behavioral maneuvers called "orgasm triggers" (discussed in the section on female orgasmic disorder), and having the client role play an exaggerated orgasm all seem to have some success with the treatment of male orgasmic disorder.

2. Female Sexual Dysfunctions

a. Female Sexual Arousal Disorder and Female Orgasmic Disorder

A persistent inability to attain or maintain sexual excitement through the completion of sexual activity describes female sexual arousal disorder; female orgasmic disorder occurs when sexual excitement is normal, but orgasm does not occur. Directed masturbation, a treatment protocol for female arousal and orgasm disorders, has broad empirical support in individual, couple, and group modalities.

b. Vaginismus

Vaginismus is characterized by the involuntary contraction of the muscles surrounding the outer third of the vagina. Relaxation training, Kegel exercises, and use of progressive dilators inserted in the vagina are the procedures used to treat vaginismus. The woman is taught deep muscle relaxation and diaphramatic breathing techniques in order to decrease her overall feelings of anxiety and to help her gain volitional control of her vaginal muscles. Voluntary control of the vaginal muscles is acquired through Kegel exercises, in which the woman practices contracting and relaxing the pubococcygeal muscle. Next, the woman is helped to overcome her fear of penetration by using a set of gradually larger dilators that she inserts into her vagina at home and at her own pace. Once the woman has been able to comfortably insert the largest dilator, she can begin to guide her partner as her or she slowly and gently inserts the dilators. It should be stressed to women in treatment for vaginismus that they go slowly and become comfortable with each step and each size dilator before moving on to the next. If a woman or her partner pushes treatment forward at too quick of a pace, the result is often increased anxiety about penetration and a rapid return to experiencing the spastic contractions.

Hypoactive Sexual Desire Disorder and Sexual Aversion Disorder

Hypoactive sexual desire disorder, often called low sexual desire disorder, is characterized by the persistent absence of desire for sexual activity. In 1988, LoPiccolo and Friedman described a four element sequential model for hypoactive desire and aversion that has been widely adopted. The first component of the treatment program, called affectual awareness, focuses on helping the client to become more familiar with his or her negative attitudes, beliefs, and cognitions about sex.

The second phase of sex therapy for low sexual desire involves insight-oriented therapy. During the insight phase, clients are helped to understand the underlying causes for the negative emotions that they have identified. Family-of-origin experiences, religious teachings, depression, fear of having children, life stress, unresolved sexual trauma or abuse, masked sexual deviations, gender identity issues, and relationship problems are explored as possible initiating and maintaining causes of the low desire or aversion. In a sense, this and the previous step are preparatory. The more active treatment follows.

Stage 3 involves cognitive and systemic therapy. This phase of therapy serves to alter irrational beliefs that inhibit sexual desire and to identify and modify relationship problems that are suppressing sexual drive.

The final element of treatment for low desire and aversion consists of behavioral interventions. These include sensate focus, skills training and other general sex therapy procedures, as well as some interventions that are more specific to these disorders.

Pridal and LoPiccolo labeled this next set of behavioral procedures "drive induction." They posited that sometimes people do not become aware of their sexual drive until they expose themselves to external cues and stimuli that trigger it, and individuals with low drive have developed an extraordinary ability to avoid these sexually relevant cues. In therapy, then, low-drive clients are asked to begin attending to sexual cues. A number of interventions are used in drive induction. For instance, low-drive clients are asked to keep a "desire diary," in which they record all sexual stimuli, thoughts, and emotions. They are also instructed to watch films that have sexual content, read erotic books and magazines, read collections of sexual fantasies, note attractive people they see, and so forth. Finally, couples are also often asked to develop erotic fantasies, both alone and together. The low-drive partner is instructed to take several "fantasy breaks" during the day in which he or she spends some time fantasizing about the sexual scenes that have been developed.

APPLICATIONS AND EXCLUSIONS

Several special populations present unique challenges to the traditional methods of treating sexual dysfunction. Sex therapy with gay men, lesbians, and bisexuals must be inclusive of sexual identity issues, the many variations of homosexuality itself, the fear of AIDS, and the internalization of heterosexism. In addition, some modification to techniques will most likely be warranted. Treatment of clients who have experienced a sexual trauma such as rape or child sexual abuse can also be challenging. Judith Becker, in a chapter on this topic published in 1988, pointed out that some women who seek treatment for sexual dysfunction have had a history of sexual trauma. These women often present with symptoms of PTSD. Special attention must be paid to these symptoms, and the initial treatment goal should be to alleviate the impact of PTSD on sexual functioning. Finally, sex therapy with the chronically ill requires some adaptation.

UNIT 8 – PHYSIOLOGICAL THERAPIES

Eye Movement Desensitization and Reprocessing

Eye Movement Desensitization and Reprocessing (EMDR) is an eight-phase treatment approach that facilitates resolution of distressing historical events, desensitization of present triggering stimuli, and acquisition of desired behaviors. While originally designed to alleviate the distress associated with traumatic memories, EMDR is now implemented within comprehensive treatment plans to address a range of experientially based complaints. Eye Movement Desensitization and Reprocessing (EMDR) is an integrative psychotherapy treatment that synthesizes elements of many orientations, including psychodynamic, cognitive-behavioral, body based, person-centered, and interactional therapies, and that incorporates several unique elements, including eye movements or other bilateral stimulation. EMDR appears to expedite the accessing and processing of disturbing memories and an attendant learning process. It focuses directly on all the perceptual components of memory (cognitive, affective, somatic) and creates new links with the adaptive information contained in other memory networks. This reprocessing results in an adaptive resolution, as indicated by the desensitization of triggers, elimination of emotional distress, elicitation of insight, reformulation of associated beliefs, relief of accompanying physiological arousal, and enhancement of personal growth and functional behaviors.

Theoretical bases

A. The Adaptive Information Processing Model

The Adaptive (or Accelerated) Information Processing (AIP) model which guides EMDR practice assumes that humans possess a physiologically based information processing system, which, under normal circumstances, naturally responds to and resolves everyday minor disturbances. Information is normally processed to an adaptive state where connections to appropriate associations are made, emotional distress is relieved, experiences are used constructively, and learning takes place. Information is understood to be stored in a system of memory networks, which are neurobiological structures containing related memories, thoughts, images, emotions, and sensations. After a traumatic event, persons who were high functioning prior to the incident generally assimilate the perceptual experience and reestablish themselves. For others, an incident of sufficiently high negative valence can result in little integration with existing positive or more adaptive information stored in networks. The AIP model proposes that pathology results when experiences are not adequately processed and the memories are dysfunctionally stored with the perceived distressing affects and physical sensations. The traumatic memory network becomes effectively isolated, and no new learning takes place.

B. Reprocessing Information

The AIP model proposes that pathology arises when the information associated with seminal events is inadequately processed. Consequently, these disorders should respond to treatment that enhances reprocessing of the dysfunctionally stored information. The goal of EMDR therapy is to stimulate information processing by accessing the dysfunctionally held information and forging new connections with more adaptive information.

C. Mechanisms of Action

Although it is apparent that EMDR expedites information processing, the exact mechanisms by which this occurs are unknown. Speculation about the role of eye movements abounds. A wide variety of primary research studies dating from the 1960s through the present have indicated a correlation between eye movements and shifts in cognitive content and attribution. Several hypotheses attempt to explain how they may contribute to information processing within the EMDR procedures: (1) eye movements disrupt the function of the visuospatial sketchpad and interfere with working memory; (2) they elicit an orienting response that stimulates an instinctive interest–excitement affect creating new associative links with the dysfunctionally stored information; (3) they evoke a relaxation response, or a new set of physiological states and responses, creating new associative links with the dysfunctionally stored information; (4) they activate neurological processes that mimic REM sleep-type function and its information processing mechanisms; and (5) they act as a distractor that titrates the emotional overload and encourages client engagement.

The Eight Phases of Treatment

1.Phase One: Client History and Treatment Planning

During Phase One the therapist takes a full history, assesses the client's readiness for EMDR, and develops a treatment plan. The therapist evaluates aspects such as diagnosis, comorbidity, suicidality, dissociation, existing support system, life stability, presence of current stressors, physical health, secondary gain issues, and substance use.

2. Phase Two: Preparation

Phase Two involves establishing the therapeutic relationship, setting reasonable levels of expectation, educating the client about his or her symptoms and about EMDR, and ensuring that the client demonstrates adequate stabilization. Stabilization is a state of equilibrium achieved prior to the processing of distressing material and includes safety, affect management skills, and self-control.

3. Phase Three: Assessment

In the third phase, the client and therapist select a specific memory to address during the session and identify the associated mental image, beliefs, emotions, and physical sensation, taking baseline response measures. The assessment phase contains steps designed to fully activate the dysfunctional memory network.

4. Phase Four: Desensitization

During the desensitization phase of EMDR, adaptive processing (i.e., learning) takes place. The client is instructed to focus on the visual image, the identified negative belief, and body sensations, and then to "Let whatever happens, happen." He or she maintains this internal focus while simultaneously moving the eyes from side to side for 20 or more seconds (depending on nonverbal cues), following the therapist's fingers as

they move across the visual field. Other bilateral stimuli (e.g., hand-tapping, aural stimulation) can be used instead of eye movements. After the set of eye movements, the client is told "Blank out (or "Let go of") the material, and take a deep breath," and then is asked "What do you get now?" Depending on what emerges (image, thought, sensation, or emotion), the clinician then directs the client's attention to the appropriate target for the next set of eye movements. This cycle of alternating focused attention and client feedback is repeated many times according to specified procedural guidelines that address various aspects of the memory network and is typically accompanied by shifts in affect, physiological states, and cognitive insights.

5. Phase Five: Cognitive Installation

As negative imagery, beliefs, and emotions become diffuse and less valid, positive ones become stronger and more salient. This transformation appears to result from a shift in how the memory is stored with new associations to more adaptive information. The fifth phase occurs after the targeted issue is resolved and the accessed memory spontaneously arises without distress. This phase allows for the expression and consolidation of the client's cognitive insights.

6. Phase Six: Body Scan

In Phase Six, the clinician asks clients, while focusing on the image and positive cognition, to notice if there is any tension or unusual sensation in the body. Because dysfunctionally stored information is experienced physiologically, processing is not considered complete until the client can bring the previously disturbing memory into consciousness without feeling any significant body tension. Any residual negative sensations detected by the body scan are targeted with eye movements until the tension is relieved. Such body sensations can be linked to unprocessed aspects of the memory network.

7. Phase Seven: Closure

In this phase, the therapist determines whether the psychological material has been adequately processed and, if not, assists clients with the self-calming interventions developed in Phase Two. The client is told by the therapist that other material might emerge after the session and is asked to maintain a journal to record any disturbance that arises, such as nightmares or flashbacks, or related material such as insights, memories, emotions, and dreams.

8. Phase Eight: Reevaluation

Reevaluation takes place at the beginning of every EMDR session following the first. In this phase, the therapist determines whether the treatment gains from the previous session have been maintained by eliciting the previously processed targets and assessing the current emotional, cognitive, and physiological responses. The clinician reviews the client's journal to evaluate the degree to which treatment effects have generalized or need further attention, and to identify new issues that need to be addressed.

AUTOGENIC TRAINING

Autogenic training is a psychotherapeutic technique based on evidence from sleep and hypnosis studies and employing yogaprinciples. It is a relaxation technique originally proposed by the German psychiatrist Johannes Heinrich Schultz and first published in 1932.

The technique involves the daily practice of sessions that last around 15 minutes, usually in the morning, at lunch time, and in the evening. During each session, the practitioner will repeat a set of visualisations that induce a state of relaxation. Each session can be practiced in a position chosen amongst a set of recommended postures (for example, lying down, sitting meditation, sitting like a rag doll). The technique can be used to alleviate many stress-induced psychosomatic disorders.Schultz emphasized parallels to techniques in yoga and meditation. It is a method for influencing one's autonomic nervous system. Abbe Faria and Emile Coue are the forerunners of Schultz. There are many parallels to progressive relaxation.

In 1963 Luthe discovered the significance of "autogenic discharges", paroxistic phenomena of motor, sensorial, visual and emotional nature related to the traumatic history of the patient, and developed the method of "Autogenic Abreaction".

Effects of autogenic training

it is proposed that Autogenic Training restores the balance between the activity of the sympathetic (flight or fight) and the parasympathetic (rest and digest) branches of the autonomic nervous system. This has important health benefits, as the parasympathetic activity promotes digestion and bowel movements, lowers the blood pressure, slows the heart rate, and promotes the functions of the immune system.

Contraindications

Autogenic Training is contraindicated for people with heart conditions or psychotic disorders.

JACOBSON'S PROGRESSIVE MUSCLE RELAXATION

Progressive relaxation is a technique of stress management developed by the American physician Edmund Jacobson in the early 1920s. Jacobson argued that since muscular tension accompanies anxiety, one can reduce anxiety by learning how to relax the muscular tension.

Jacobson trained his patients to voluntarily relax certain muscles in their body in order to reduce anxiety symptoms. He also found that relaxation procedure effective with ulcers, insomnia, and hypertension. There are many parallels with Autogenic training, which was developed independently.

Jacobson's Progressive Relaxation is still popular with modern therapists.

Progressive Muscle Relaxation steps

Sit in a comfortable chair--reclining arm chairs are ideal. Bed is okay too. Get as comfortable as possible--no tight clothes, no shoes, don't cross your legs. Take a deep breath; let it out slowly. Again. What you'll be doing is alternately tensing and relaxing specific groups of muscles. After tension, a muscle will be more relaxed than prior to the tensing. Concentrate on the feel of the muscles, specifically the contrast between tension and relaxation. In time, you will recognize tension in any specific muscle and be able to reduce that tension.

Don't tense muscles other than the specific group at each step. Don't hold your breath, grit your teeth, or squint! Breath slowly and evenly and think only about the tension-relaxation contrast. Each tensing is for 10 seconds; each relaxing is for 10 or 15 seconds. Count "1,000 2,000..." until you have a feel for the time span. Note that each step is really two steps--one cycle of tension-relaxation for each set of opposing muscles.

Do the entire sequence once a day if you can, until you feel you are able to control your muscle tensions. Be careful: If you have problems with pulled muscles, broken bones, or any medical contraindication for physical activities, consult your doctor first.

1. Hands. The fists are tensed; relaxed. The fingers are extended; relaxed.
2. Biceps and triceps. The biceps are tensed (make a muscle--but shake your hands to make sure not tensing them into a fist); relaxed (drop your arm to the chair--really drop them). The triceps are tensed (try to bend your arms the wrong way); relaxed (drop them).
3. Shoulders. Pull them back (careful with this one); relax them. Push the shoulders forward (hunch); relax.
4. Neck (lateral). With the shoulders straight and relaxed, the head is turned slowly to the right, as far as you can; relax. Turn to the left; relax.
5. Neck (forward). Dig your chin into your chest; relax. (bringing the head back is not recommended--you could break your neck).
6. Mouth. The mouth is opened as far as possible; relaxed. The lips are brought together or pursed as tightly as possible; relaxed.
7. Tongue (extended and retracted). With mouth open, extend the tongue as far as possible; relax (let it sit in the bottom of your mouth). Bring it back in your throat as far as possible; relax.
8. Tongue (roof and floor). Dig your tongue into the roof of your mouth; relax. Dig it into the bottom of your mouth; relax.

9. Eyes. Open them as wide as possible (furrow your brow); relax. Close your eyes tightly (squint); relax. Make sure you completely relax the eyes, forehead, and nose after each of the tensings--this is actually a toughy.
10. Breathing. Take as deep a breath as possible--and then take a little more; let it out and breathe normally for 15 seconds. Let all the breath in your lungs out--and then a little more; inhale and breathe normally for 15 seconds.
11. Back. With shoulders resting on the back of the chair, push your body forward so that your back is arched; relax. Be very careful with this one, or don't do it at all.
12. Butt. Tense the butt tightly and raise pelvis slightly off chair; relax. Dig buttocks into chair; relax.
13. Thighs. Extend legs and raise them about 6" off the floor or the foot rest--but don't tense the stomach' relax. Dig your feet (heels) into the floor or foot rest; relax.
14. Stomach. Pull in the stomach as far as possible; relax completely. Push out the stomach or tense it as if you were preparing for a punch in the gut; relax.
15. Calves and feet. Point the toes (without raising the legs); relax. Point the feet up as far as possible (beware of cramps-if you get them or feel them coming on, shake them loose); relax.
16. Toes. With legs relaxed, dig your toes into the floor; relax. Bend the toes up as far as possible; relax.

Now just relax for a while. As your days of practice progress, you may wish to skip the steps that do not appear to be a problem for you. After you've become an expert on your tension areas (after a few weeks), you can concern yourself only with those. These exercises will not eliminate tension, but when it arises, you will know it immediately, and you will be able to "tense-relax" it away or even simply wish it away.

UNIT 9 – COUNSELING

Counselling is not just something that happens between two people. It is also a social institution that is embedded in the culture of modern industrialized societies. It is an occupation, discipline or profession of relatively recent origins. These are some definitions of ‘counselling’ formulated by professional bodies and leading figures in the field: The term ‘counselling’ includes work with individuals and with relationships which may be developmental, crisis support, psychotherapeutic, guiding or problem solving . . . The task of counselling is to give the ‘client’ an opportunity to explore, discover and clarify ways of living more satisfyingly and resourcefully (BAC 1984).

Counselling denotes a professional relationship between a trained counselor and a client. This relationship is usually person-to-person, although it may sometimes involve more than two people. It is designed to help clients to understand and clarify their views of their lifespace, and to learn to reach their self-determined goals through meaningful, well-informed choices and through resolution of problems of an emotional or interpersonal nature (Burks and Stefflre 1979: 14). a principled relationship characterised by the application of one or more psychological theories and a recognised set of communication skills, modified by experience, intuition and other interpersonal factors, to clients’ intimate concerns, problems or aspirations. Its predominant ethos is one of facilitation rather than of advice-giving or coercion. It may be of very brief or long duration, take place in an organisational or private practice setting and may or may not overlap with practical, medical and other matters of personal welfare. It is both a distinctive activity undertaken by people agreeing to occupy the roles of counsellor and client . . . and an emerging profession . . . It is a service sought by people in distress or in some degree of confusion who wish to discuss and resolve these in a relationship which is more disciplined and confidential than friendship, and perhaps less stigmatising than helping relationships offered in traditional medical or psychiatric settings (Feltham and Dryden 1993: 6).

It can be seen from these definitions that counselling can have different meanings. For example, Burks and Stefflre (1979) stress the idea of the 'professional' relationship, and the importance of 'self-determined' goals. The BAC definition places emphasis on exploration and understanding rather than action. Feltham and Dryden (1993) highlight the areas of overlap between counselling and other forms of helping, such as nursing, social work and even everyday friendship. Counselling is an activity that takes place when someone who is troubled invites and allows another person to enter into a particular kind of relationship with them.

• A person seeks such a relationship when they encounter a 'problem in living' that they have not been able to resolve through their everyday resources, and that has resulted in their exclusion from some aspect of full participation in social life.

• The person seeking counselling invites another person to provide him or her with time and space characterized by the presence of a number of features that are not readily available in everyday life: permission to speak, respect for difference, confidentiality and affirmation.

• ***Permission to speak***. This is a place where the person can tell their story, where they are given every encouragement to give voice to aspects of their experience that have previously been silenced, in their own time and their own way, including the expression of feeling and emotion.

• ***Respect for difference***. The counsellor sets aside, as far as they are able, their own position on the issues brought by the client, and his or her needs in the moment, in order to focus as completely as possible on helping the client to articulate and act on his or her personal values and desires.

• ***Confidentiality***. Whatever is discussed is confidential: the counsellor undertakes to refrain from passing on what they have learned from the person to any others in the person's life world.

• ***Affirmation***. The counsellor enacts a relationship that is an expression of a set of core values: honesty, integrity, care, belief in the worth and value of individual persons, commitment to dialogue and collaboration, reflexivity, the interdependence of persons, a sense of the common good.

These are simple principles, but taken together represent an arena for support, reflection and renewal that is distinctive within modern societies. Within this arena, the client and counsellor make use of whatever cultural resources come to hand (conversation, ideas, theories, rituals, altered states of consciousness, problem-solving algorithms, discourses, technologies) to achieve a satisfactory resolution of the initial problem in living that initiated the decision to engage in counselling.

Counseling and psychotherapy

There has been considerable debate over the difference between counseling and psychotherapy. Some would claim that a clear distinction can be made between the two, with psychotherapy representing a deeper, more fundamental or involved process of change with more disturbed clients. Others maintain that counsellors and psychotherapists are basically doing the same kind of work, using identical approaches and techniques, but are required to use different titles in response to the demands of the agencies that employ them. One significant difference between counselling and psychotherapy is that much counselling is conducted by nonprofessional volunteer workers, whereas psychotherapy is an exclusively professional occupation. However, both counselling and psychotherapy can be viewed as activities distinct from advice-giving, caring and teaching.

The aims of counselling

Underpinning the diversity of theoretical models and social purposes discussed above are a variety of ideas about the aims of counselling and therapy. Some of the different aims that are espoused either explicitly or implicitly by counselors are listed:

• ***Insight***. The acquisition of an understanding of the origins and development of emotional difficulties, leading to an increased capacity to take rational control over feelings and actions (Freud: 'where id was, shall ego be').
• ***Relating with others***. Becoming better able to form and maintain meaningful and satisfying relationships with other people: for example, within the family or workplace.
• ***Self-awareness***. Becoming more aware of thoughts and feelings that had been blocked off or denied, or developing a more accurate sense of how self is perceived by others.
• ***Self-acceptance***. The development of a positive attitude towards self, marked by an ability to acknowledge areas of experience that had been the subject of self-criticism and rejection.
• ***Self-actualization or individuation***. Moving in the direction of fulfilling potential or achieving an integration of previously conflicting parts of self.
• ***Enlightenment***. Assisting the client to arrive at a higher state of spiritual awakening.
• ***Problem-solving***. Finding a solution to a specific problem that the client had not been able to resolve alone. Acquiring a general competence in problem-solving.
• ***Psychological education***. Enabling the client to acquire ideas and techniques with which to understand and control behaviour.
• ***Acquisition of social skills*.** Learning and mastering social and interpersonal skills such as maintenance of eye contact, turn-taking in conversations, assertiveness or anger control.
• ***Cognitive change***. The modification or replacement of irrational beliefs or maladaptive thought patterns associated with self-destructive behaviour.
• ***Behaviour change***. The modification or replacement of maladaptive or selfdestructive patterns of behaviour.
• ***Systemic change***. Introducing change into the way in that social systems (e.g. families) operate.
• ***Empowerment.*** Working on skills, awareness and knowledge that will enable the client to take control of his or her own life.
• ***Restitution*.** Helping the client to make amends for previous destructive behaviour.
• ***Generativity and social action***. Inspiring in the person a desire and capacity to care for others and pass on knowledge (generativity) and to contribute to the collective good through political engagement and community work.

The diversity of theorizing in counselling

Why are there so many theories of counselling and psychotherapy? One widely publicized survey, by Karasu (1986), found more than 400 different named approaches to therapy. It is very difficult to accept that there are, in reality, so many unique ways to practise therapy. The proliferation of counselling and psychotherapy theories arises from a number of factors, which are discussed in this section.

1. Alternative 'images of the person'

One very fundamental source of theoretical complexity is that any psychological theory, or approach to counselling, ultimately relies on a root 'image of the person', a set of basic assumptions about the very nature of what it is to be human. Shotter (1975) has suggested that two of the dominant images of the person used in psychology have been the image of the machine ('the best way to understand people is to view them as mechanical objects') and the image of the organism ('the best way to understand people is to view them as animals, biological entities'). These two images can certainly be found running through different counselling approaches, from the mechanistic thinking of traditional behaviourism to the many references to bodily

functioning in classical psychoanalysis. The ideas of existential philosophy have, more recently, introduced the image of the person not as an organism or mechanism but as a social being. The influence of Eastern philosophies has introduced the image of the person as spiritual being.

It can be difficult to reconcile underlying images, and so at least some of the differences between counselling theories can be attributed to the different images of the person that underpin them. The situation is even further complicated by the fact that the same theory can encompass more than one root image. For example, psychoanalysis contains mechanistic ideas (e.g. defence mechanisms) and organismic/ biological ideas (e.g. libido, oral fixation). Person-centred theory contains organismic images (e.g. the organismic valuing process, the self-actualization motive) as well as existential ones (e.g. congruence, empathy). Any approach to therapy can be distilled into one or two core 'images'. Bioenergetics draws upon a strong image of the embodied, sexual person. Jungian and transpersonal approaches draw upon an image of the person as spiritual being. For systemic approaches, the person is primarily a social being. The medical model, used in psychiatry, views the person in terms of an 'illness' metaphor or image. There is a danger, in relation to considering theories in terms of underlying images or metaphors, of over-simplifying complex sets of ideas. On the other hand, in each case the root image or metaphor captures an essential truth about a theoretical approach. We live in a culture that supports a rich diversity of images of the person, and as a result generates a rich diversity of therapy theories.

2. The personal dimension of theory

In other disciplines, theories and ideas tend to be identified in terms of conceptual labels, rather than being known through the name of their founder. Even in mainstream psychology, theoretical terms such as behaviourism or cognitive dissonance are employed, rather than the names of their founders (J. B. Watson, Leon Festinger). In counselling and psychotherapy, by contrast, there is a tradition of identifying theories very much with their founders. Terms such as Freudian, Jungian, Adlerian, Rogerian or Lacanian are commonplace. There are probably many reasons for this. However, one factor is certainly the recognition that theories of therapy typically reflect, to a greater or lesser extent, the personality and individual world-view of the founder. Huge amounts have been written, for instance, about the links between Freud's own life and circumstances, and the ideas that came together in his psychoanalytic theory. It may be that theories of therapy are necessarily so personal, that it is impossible to write and formulate them without importing one's own personal experience and biases. One of the reasons for the multiplicity of therapy theories, therefore, is that many individual therapists and counsellors find that the personal tenor of established theories does not quite chime with their own experience, with the result that they are driven to write down their own, personal 'version' of the theory. The intimate connection between theorizing in therapy and the personality of the theorist is explored in a classic book by Atwood and Stolorow (1993).

3. The social context of theory construction

The idea of the 'reconstruction' of therapy in response to changing social and cultural forces has already been mentioned. Some of the important social factors responsible for the invention of psychotherapy in the nineteenth century were mentioned: the secularization of society, the movement away from authority-base relationships, the moves in the direction of greater individuality. All of these factors helped to determine the shape of psychoanalytic therapy. In more recent times, the increasing economic pressures facing health care systems and the political momentum of equal opportunities advocacy have required theories of therapy to accommodate new ideas around time limits and cultural identity. As the rate of social change continues to accelerate, it seems certain that theories of therapy will continue to proliferate, as theorists develop new ideas appropriate to the delivery of counselling in new situations.

Eclecticism and integrationism

An eclectic approach to counselling is one in which the counsellor chooses the best or most appropriate ideas and techniques from a range of theories or models, in order to meet the needs of the client. Integration, on the other hand, refers to a somewhat more ambitious enterprise in which the counsellor brings together elements from different theories and models into a new theory or model. To be an eclectic it is merely necessary to be able to recognize or identify what you like in the approaches on offer. To be an integrationist it is necessary not only to identify what is useful, but also to weld these pieces into a whole.

The use of 'transtheoretical' concepts in integrative approaches

In practice, the main strategy for achieving integration has been to find a central theoretical concept or framework within which some or all existing approaches can be subsumed. Barkham (1992) has suggested that integrationist counselors and therapists attempt to identify higher-order constructs that can account for change mechanisms beyond the level of any single model. The aim is to produce a cognitive 'map' that will enable the links and connections between ideas and techniques to be understood. There are several examples of approaches to counseling and therapy that employ such higher-order or transtheoretical constructs. One example of a transtheoretical approach to integration, which is widely used within counselling, is the 'skilled helper' model constructed by Egan (1990). The key integrating concept chosen by Egan is that of problem management. Egan suggests that clients who seek assistance from counsellors and other helpers are experiencing difficulties in coping with problems in their lives, and that the primary task of the helper is to enable the person to find and act on appropriate solutions to these problems.

The emphasis is therefore on a problem-solving process, which involves three stages. First, the client is helped to describe and explore the 'present scenario', the problem situation that he or she is faced with at present.The second stage is to articulate a 'preferred scenario', which includes future goals and objectives. The third stage is to develop and implement action strategies for moving from the current to the preferred scenario. Egan (1990) describes substages within each stage, and identifies the client tasks and helper skills necessary to facilitate this problem-solving process. The Egan model can usefully be viewed as a 'map' through which the usefulness of relevant elements of other approaches can be located and evaluated. skilled helper offers an intensely practical and pragmatic approach to working with people, and that it is applicable to a wide variety of situations, ranging from individual counselling to organizational consultation. As an integrationist approach, its limitation is that it is primarily based in a cognitive–behavioural perspective.

Another integrative approach that employs a central unifying concept as a device for combining different approaches is the self-confirmation model developed by Andrews (1991). The core idea in this model is that the individual acts in the world to reaffirm his or her self-concept. The process of self-confirmation involves a feedback loop consisting of a number of stages. The self-concept of the person represents the way he or she perceives his or her attitudes, feeling states, ways of acting in situations and all other dimensions of 'what is me'. This sense of self generates characteristic needs and expectations.

A third transtheoretical approach is cognitive-analytic therapy (CAT) (Ryle and Cowmeadow 1992; Ryle 1995; Ryle and Kerr 2002), originally developed by Ryle (1990). This model is based on some recent ideas from cognitive psychology, concerning the ways that people engage in intentional activity through sequences of mental and behavioural acts. In pursuing their life goals, people run into trouble when they encounter traps, dilemmas and snags. The psychoanalytic dimension of this model includes the Freudian idea of defence mechanisms as examples of cognitive 'editing', and takes account of the origins of traps, dilemmas and snags in early parent–child interactions.

An appraisal of the person-centred approach The early phase of the development of the person-centred approach, the 'school' years (Barrett-Lennard 1979), represents a unique achievement in the history of counselling and psychotherapy. Between 1940 and 1963, Rogers and others evolved a consistent, coherent body of theory and practice that was informed and shaped by ongoing research. Despite the later fragmentation of the approach, it remains a powerful strand of thought in the contemporary counselling world. However, recent developments resulted in the appearance of some fundamental tensions between alternative conceptualizations of the legacy of Rogers. One of these issues concerns the identity of the person-centred approach as a distinctive mode of counselling. Many counsellors and therapists regard themselves as 'client-centred' or 'person-centred' because their intention is to focus on the experience or needs of the client, rather than to impose their own definitions or structures, and because they find the ideas encompassed in the 'core conditions' model a useful framework for understanding the nature of the therapeutic alliance. However, these values are often combined with a view that, while the core conditions might be necessary, they are in themselves insufficient to bring about change (Bohart 1990; Tausch 1990). These counsellors use the core conditions as a basis for the employment of therapeutic techniques derived from other approaches.

This approach is characteristic of that of many counsellors who use the person centred perspective as a basis for integration (see Chapter 3). The end product is a way of working that is *a* person-centred approach rather than *the* person centred approach. There is support for this kind of endeavour in Rogers' writings. He suggested in relation to the core conditions that empathy, congruence and unconditional positive regard could be communicated to the client in many different ways, for example through psychoanalytic interpretation (Rogers 1957). On the other hand, there is the danger that counsellors working in this fashion may merely use person-centred ideas as a gloss beneath which they are operating in a quite different fashion. For example, a rigorous interpretation of person-centred principles involves a reliance on the actualizing tendency of the client, and continual use of self in the relationship. the theoretical framework of the person-centred approach places great emphasis on the quality of the relationship between counsellor and client: for example, in terms of a gradual growth in trust and safety. It also states that this mode of working relies on the presence of a set of attitudes and beliefs in the counsellor, and an ability to convey these qualities in terms of a powerful, authentic presence. Clearly, brief skills training and self-help programmes can do little to address relationship and attitude issues in any serious or systematic manner. There are times when this split between technique and underlying philosophy produces uncomfortable results.

Behaviorism, Behavior Modifi cation, and Behavior Therapy

John B. Watson coined the word *behaviorism* while Arnold Lazarus created the term *behavior therapy*. Counselors who use these approaches assume that behavior is based on learning rather than insight into the unconscious mind. The criticisms of behavior therapy are that it does not yield insight, it treats symptoms not the root cause, and it can be manipulative and often changes behavior but not underlying feelings.

B.F. Skinner's radical behaviorism purports that behavior is molded solely by its consequences. This paradigm is known as operant or instrumental conditioning. **A positive reinforcer** is a stimulus that raises the probability that a behavior will be repeated. The reinforcer must come after the behavior (or operant). **Negative reinforcers** also raise behavior. For example, a recruit in the military makes a bunk bed to avoid being yelled at by a drill instructor. All reinforcers, whether positive or negative raise behavior. All reinforcers are said to follow or come *after* a behavior (e.g., a youngster gets a prize after she completes a math problem). **Albert Bandura** speaks of **social learning theory**. Here the person's own behavior increases when he or she sees somebody else getting reinforced for it; also referred to as vicarious learning or modeling. **Punishment** is intended to lower behavior. Behaviorists champion role playing (e.g., assertive behavior). **Extinction** (such

as time-out) will lower behavior after an initial extinction burst or response burst. **Ratio schedules of reinforcement** rely on work output whereas interval schedules rely on time.

Continuous reinforcement occurs when each behavior is reinforced. **Intermittent reinforcement** occurs when some, but not all of he desired behaviors are reinforced (e.g., a child gets a treat for every third math problem he completes). **Shaping with successive approximations** is reinforcing small chunks of behavior that lead to the desired behavior.

Differential reinforcement of other behaviors (DRO)/ differential reinforcement of alternative behavior (DRA) takes place when the helper reinforces behaviors other than the dysfunctional behavior to reduce the dysfunctional **target behavior** (e.g., you want a child to quit talking in class so you give him a gold star only when he is doing his work and not talking). In Skinnerian **operant conditioning**, the behavior is affected by the consequences that come after the behavior. **Ivan Pavlov** popularized what is now known as **classical conditioning**. Behavior modification is generally based on Skinner, while behavior therapy usually has its roots in Pavlov. **Joseph Wolpe's systematic desensitization** can be conducted individually or in a group to curb fears and abate anxiety. His technique of counter conditioning is based on Pavlov and relies on relaxation and imagining feared stimuli. Behaviorists may also use implosive therapy where the client imagines scary or feared stimuli in the safety of the counselor's office. Biofeedback devices are used to enhance the client's self control (such as muscle tension) and are a form of operant conditioning.

Criticisms of behavior therapy: It does not yield insight; it is mechanistic; it treats symptoms and not the cause; it can be manipulative; generally ignores developmental stages; and it often changes behavior but not underlying feelings. This approach deals with behaviors rather than the whole person. **Hint:** A great way to determine whether a question is referring to reflexive classical conditioning or Skinner's operant conditioning is to ask yourself a simple question: Would the conditioning work with every member of the species who is not disabled? If the answer is *yes*, then it is most likely Pavlovian conditioning (e.g., all dogs salivate when they eat meat, so it's Pavlov).

Carl R. Rogers's Person-Centered Humanistic Therapy

This approach has also been called client-centered, Rogerian, nondirective, or self-theory. The basic notion (a very positive one) is that human beings can self-actualize and reach their full potential in a therapeutic setting that fosters growth; classifi ed as an optimistic form of therapy.

Three conditions for effective helping: The therapist must show empathy; be genuine/congruent; and display unconditional positive regard (UPR).

Empathy is not sympathy! It is the ability to subjectively understand the client's world in the here-and-now—to walk in his or her shoes—and convey this to the client. Robert Carkhuff created a fi ve-point empathy scale with a level five response as the best response. The counselor must be genuine/congruent. The counselor cannot be a phony. Words must match (i.e., be congruent) with actions. The counselor accepts the client regardless of his or her behavior. This does not imply that you necessarily agree with the client. This is called **unconditional positive regard**. The counselor will often use refl ection or open-ended questions. Counselor strives to improve congruence so that the person is more like his or her ideal self. Rogerians are not big fans of traditional diagnosis and testing. Moreover, they do not believe in the unconscious. Humans can control their own behavior. Critics are concerned that this approach is too optimistic and may not be the treatment of choice for severely disturbed individuals or very young children.

<u>Process of counseling</u>

The concept of ‘process’ is defined and understood in several different ways in the literature, which can lead to confusion. Four main meanings of ‘process’ can be identified. First, there is a very broad sense in which any activity involvingchange can be described as being a ‘process’. This meaning of the term merely refers to the idea that what happens in therapy is not static, and that there is some sort of sequence of events that takes place. A second meaning of ‘process’ has been employed mainly in the research literature, to refer to a very wide set of factors that may promote or inhibit therapeutic effects in clients. The use of the term contrasts ‘process’ with ‘outcome’: therapeutic ‘processes’ are the ingredients that contribute to outcomes. A third meaning of ‘process’ is found mainly within humanistic perspectives on therapy. This definition characterizes process as an essential human quality of being and becoming. A fourth sense of ‘process’ that is sometimes used by counsellors and psychotherapists describes the way that clients in therapy attempt to comprehend or assimilate difficult experiences in their lives.

It is clear that counselling process represents a huge topic. Any of the processes that have been mentioned can be explored from the point of view of the client or the therapist, or through the eyes of an external detached observer. Moreover, what takes place can occur simultaneously at different levels of awareness and visibility: there is always a hidden, covert process unfolding inside the consciousness of each participant. Elliott (1991) suggests that it can be helpful to break down the ongoing flow of the process occurring in counselling into different types of units demarcated by their time boundaries:

• The speaking turn (interaction unit), encompassing the response of one speaker surrounded by the utterances of the other speaker. This can be regarded as a *microprocess* that lasts for perhaps no more than one or two minutes.

• The episode, comprising a series of speaking turns organized around a common task or topic. This process unit is sometimes described as a therapeutic *event*, and can last for several minutes.

• The session (occasion unit).

• The treatment (relationship unit): the entire course of a treatment relationship. Each of these units can be regarded as representing a different way of ‘seeing’ what takes place in counselling. Analysing microprocesses is like looking at counseling through a microscope; examining the process of a whole treatment is like constructing a map by using a telescope to view the furthest horizons.

Many writers on counselling have tended to divide up the process of treatment into three broad phases. For example, Mearns and Thorne (1988) talk about ‘beginnings’, ‘middles’ and ‘ends’. Egan’s (1994) ‘problem management’ approach is structured around three main stages: helping clients to identify and clarify problem situations; developing programmes for constructive change; and implementing goals. The opening and concluding phases of counselling can be split into further sets of discrete component elements or tasks. For example, the beginning phase may include negotiating expectations, assessment of suitability for counselling, the formation of a therapeutic alliance, agreeing a contract, helping the client to tell their story and so on. The final phase may entail negotiating the ending, referral, dealing with issues of loss, ensuring transfer of learning into real-life situations, anticipating and preventing relapse and planning follow-up meetings. Each of these aspects of the counselling process raises key issues for theory and practice.

Although there exist different meanings of process, they all refer to a sense of change, movement and activity within the counselling session. It is useful to break down the overall process into three broad phases: beginning , middle and end.

Some of the key factors in the opening phase of counselling are the negotiation of expectations, assessment of the client and the formation of a therapeutic relationship.

The key processes at the ending phase of counselling are dealing with loss, maintenance of learning and referral.

The middle phase of counselling is the stage where most learning and change occurs. Two useful models of the change process are the *assimilation* model and the *multiple processing* theory. It can often be valuable to analyse what happens during significant helpful or hindering *events* as windows into the process of counselling.

Assimilation of problematic experiences

The first model of change being considered here is the *assimilation model*, devised by Stiles and his associates (Stiles *et al*. 1990, 1992; Barkham *et al*. 1996; Honos- Webb *et al*. 1998, 1999; Stiles 1991, 2001, 2002). The basic idea behind this model is that the individual possesses a model of the world, or a set of cognitive schemas that guides that person's behaviour. New experiences need to be assimilated into that model if they are to be understood and to make sense. Experiences that do not fit into the schema or model can lead to a process of change, or accommodation, in the model itself. This theory is basically adopted from Piagetian developmental psychology, but is consistent with most models of therapy. It therefore represents a transtheoretical or integrative model.

Some researchers have attempted to understand the process of change in counseling in terms of the set of behaviours or intentions employed by the counsellor. This approach is particularly useful in training counsellors. Much of the process of what happens in and between client and counsellor is covert and hidden; it can often be useful to inquire into what is *not said* in a counselling session. Practitioners or students/trainees interested in exploring process in the context of their own work may find that the comprehensive process analysis method provides a useful set of guidelines. The goal of studying and understanding process is to help counsellors to become more actively *intentional*, to be aware of what they do and to be in a position to follow the line that is most helpful for their client.

UNIT 10 - THERAPY IN SPECIAL CONDITIONS

Psychotherapy in personality disorders

Cluster A comprises paranoid personality disorder (PPD), schizoid personality disorder (SPD), and schizotypal personality disorder (StPD). These disorders affect 2%, slightly less than 1%, and 4% of the Western population, respectively, and can be highly disabling. Their incidence is higher in men than in women and the conditions are characterized by odd, eccentric, or â€˜coldâ€™ behavior (particularly SDP and StPD). It is thought that a biological relationship may exist between the disorders and the schizophrenias, although of the three, StPD is more demonstrably linked to schizophrenia phenomenologically and genetically (McGlashan, 1983). SPD and StPD are sometimes grouped as part of a continuum, given the similarity of certain symptoms. No distinctive set of psychoanalytic, cognitive-behavioral therapy (CBT) or group theories is applicable to these conditions. More research is needed before specific psychological theories can be established. Conceptualization of Cluster A disorders tends to utilize theories developed from the study of psychosis.

Key practice principles

Psychodynamic and cognitive-behavioral practice principles will be addressed first, followed by group and TC concepts and practices.

Psychodynamic approaches pay special attention to transference counter transference phenomena in order to grasp what is taking place in the therapeutic relationship. Without careful attention to the therapeutic alliance and interpretation of the transference particularly the negative transference treatment can founder, above all with PPD where levels of suspicion are high. In practice, this means that the therapist must try to understand

how he or she is being experienced by the patient, not least in object relations terms (Who am I currently representing for the patient, and in what way? and to how the patient is making the therapist feel [e.g. I am now experiencing strong feelings (these may be boredom, sexual, aggressive feelings, etc.): to what extent do these feelings originate in me or is the patient inducing me to feel these?). The effects of splitting of the patient's ego and of the object (cf. the paranoid-schizoid position, Klein, 1946) underlie these transference/countertransference issues and can make treatment confusing and erratic. At one moment the therapist may be experienced positively, even as an idealized figure; this can change dramatically into the therapist being seen as a persecuting critic or tyrant. This can take place without the therapist saying anything controversial and signifies a radical disjuncture in the patient's affective experience of others.

The more a patient can express true feelings in the transference, the more therapeutic the treatment is likely to be. Avoiding malignant regression is important. Regression is a defensive reversion, under stress, to earlier forms of thinking and object-relating and is often inevitable in therapy. Benign regression signifies a healthy satisfying of certain infantile needs by working these through collaboratively in the therapy. Malignant regression denotes a situation is which the patient tries but fails to have these needs met and the situation yields a vicious cycle of demanding, addiction-like states. The analyst's technique, counter transference responses and capacity for maintaining boundaries are important in avoiding malignant regression (Balint, 1968). To achieve the trust of a Cluster A patient the therapist must tolerate difficult, even extreme counter transference feelings. These feelings are commonplace because the patient will try to rid him or herself of unacceptable feelings by projecting them on to the therapist. This activity needs to be attended to for its communications value and for its potential to derail an understanding of the patient's emotional state if the therapist reacts in an overemotional way (Heimann, 1950; Carpy, 1989; Gabbard and Wilkinson, 1994).

Negative therapeutic reactions (stubborn resistances to improvement usually following some improvement; cf. Freud, 1923; Riviere, 1936; Steiner, 1994) are to be expected and the separation anxieties, narcissistic rages, and envious impulses associated with these require interpretation. For SPD patients transference interpretation of claustro-agoraphobic anxieties is necessary (Rey, 1994). Actings-out by patients and crises over money, timings, holidays, etc. can arise and these may need to be responded to by reality-based, supportive interventions, together with interpretation of the anxieties being defended against. Therapeutic goals require realistic assessment and regular monitoring: progress may be slow and erratic with setbacks and perhaps limited eventual gains. Interpretation and explication together may be required to support movement from disorganized thinking towards integration of severe anxieties, especially in StPD where fragmentation of the ego may prevail.

Cognitive approaches tend be technically similar for all Cluster A disorders in that they target the pathological belief or system (schema) and this helps particularly in the amelioration of maladaptive habits and in limit setting. Associated problems of depression and self-image are tackled as secondary phenomena. Establishing initial trust in the therapeutic relationship requires a flexible approach that is sensitive to changes in the patient's mental state. A neutral standpoint is maintained in relation to the patient's perspective of their problems alongside validation of the affective experience. The therapist teases out particular life circumstances and events that provide a context for the formation and maintenance of the patient's beliefs. One important difference between a psychodynamic and CBT perspective is that paranoid responses (these are common in Cluster A disorders) are not necessarily regarded as transference issues but instead as reactions to perceived threat. In CBT the patient is invited to test out their beliefs and to review evidence and alternative hypotheses. A new, more realistic model of events is constructed with the patient. The therapist and patient collaborate to examine and assess evidence for and against certain beliefs using behavioral experiments.

Negative self-evaluations may be isolated and reviewed according to a more realistic appraisal of the person's circumstances.

Group approaches are based upon the conscious and unconscious network of relationships within groups, sometimes referred to as the matrix (Blackwell, 1998). The emphasis lies on social functioning rather than individual unconscious drives (the whole is more elementary than the parts). The matrix" or the way the group functions as a social unit"is a powerful agency. It is an object of attachment and a source of safety and containment, and these harbor therapeutic potential. Group analytic theory was developed by Foulkes, a psychoanalyst, who paved the way for understanding group relations processes (Foulkes, 1964, 1986). He identified processes in groups such as resonance, condenser phenomenon, and mirroring through which unconscious activity can be described: (1) resonance involves shared experience of supportive identifications between group members; (2) condenser phenomena describe articulation of unconscious feelings through shared forms of symbolization; and (3) mirroring is where group members can observe and integrate split-off parts of themselves by seeing them in others and coming to understand them through engagement with the group. Nonverbal therapeutic techniques such as acceptance of silence without striving to explore or interpret (particularly for schizoid members), tolerance of oddness (for the schizotypal) and use of sympathetic eye contact can be therapeutically effective in addition to verbal interventions. Foulkes ideas have been applied to work in TCs (therapeutic community) where the activities of daily living offer a benefit to Cluster A patients as they can be engaged in therapeutic relationship building without needing to talk. TC approaches embody two main precepts: the community as the agent of change and the TC culture of self-help. Typically, TCs are residential facilities and the resident is expected to adhere to certain behavioral norms. The resident may progress through a hierarchy of increasingly more important roles, with greater privileges and responsibilities. Individual and group therapy, group sessions with peers, community-based learning, confrontation, games, and role-playing may all be utilized as part of an extensive therapeutic experience. TC treatment varies but can be broadly divided into three major stages: (1) induction during the first month or so in order to assimilate the individual into policies and procedures; (2) systematic involvement at multiple levels of engagement individual, group and social using the methods described above (a typical day might start at 7 a.m. and end at 11 p.m. and comprise morning and evening community groups/meetings, groups, seminars, work tasks, individual therapy, and recreation); and (3) a phased transition into the outside world in which the values and practices of TC are carried into normal living.

Challenges

Cluster A individuals are the least likely of the personality disorder groups to undertake psychotherapy of any kind due to their reduced capacity to engage in relationships. However, it does not follow that psychotherapy is automatically contraindicated: many patients do benefit from therapy. All forms of therapy face significant challenges with Cluster A patients, although their strategies for dealing with these differ. PPD patients threaten the therapeutic relationship through their suspiciousness and distorted conviction that hostility and danger are omnipresent. Alertness to signs of mounting suspiciousness is therefore essential: this needs to be responded to by transference interpretation (psychodynamic therapists), empathic discussion, and review of evidence for beliefs (CBT) and open acknowledgement within the matrix (group therapists). Directly confronting PPD beliefs by argument can have the effect of reinforcing the belief as PPD patients use projective identification extensively to externalize hostile impulses for fear of destroying the therapeutic relationship. The following is an extract from a psychoanalytic psychotherapy session with a 40-year-old patient with PPD symptoms. She suffered an internal, superego voice that advised her against relationships.

Individual and group psychotherapy of antisocial personality disorder

For Winnicott, the first necessity in the treatment of the delinquent was for clinical management; and the same applies for the treatment of ASPD. Only when sufficient control has been achieved, can further therapeutic intervention proceed. Thus, treatment may frequently require initial residential, possibly locked, provision, and only later may outpatient, ambulant, therapy be possible; different examples are described later. Sometimes ambulant therapy may be possible from the outset, sometimes with more or less support between therapeutic sessions, e.g., via partial hospitalization (day hospital). For milder psychopathology outpatient individual or group psychotherapy will be indicated. As all forms of â€˜acting outâ€™ are to be expected, there may commonly be sabotage of the therapeutic setting in a variety of ways if management is insufficient, e.g., there may be gross misuse of drugs or alcohol. Even in the best set-up this may happen from time to time and needs to be understood as part of the problem, rather than evoking a response of immediate rejection.

The mode of psychotherapy offered will depend on many factors: the degree of psychological mindedness; motivation; the extent and nature of â€˜denialâ€™; the degree of subjective distressâ€”or ulterior reasons, such as the recognition of the self-destructive consequences of the antisocial acts.

As a whole spectrum of different types of people and psychopathology are encompassed within the term ASPD, only a few general statements or guidelines, largely of a practical nature, will be offered.

Cognitive therapy for antisocial personality disorder

The cognitive therapy model is based on the notion that attributional bias is the main problem accounting for behavioral and affective dysfunction. The way in which information is perceived, interpreted and acted upon is problematic. Schemas are a central concept in the model and can be conceptualized as cognitive structures that organize experience and behavior. Schemas are thought of as a guiding behavior in a consistent rule bound manner. Cognitive therapy focuses on the product of schemas, the patient's behavioral and interpersonal problems, and the core beliefs that underlie them.

In Beck and Freeman's (1990) cognitive model of personality disorder, some individuals may show more extreme forms of the personality types or patterns that were once adaptive but are now maladaptive in the contemporary world. For example, in ASPD, combative and explorative behavioral patterns are overdeveloped and other behavioral patterns, such as sharing, group identification, and intimacy are underdeveloped. These latter patterns also have evolutionary survival value, particularly in maintaining relationships where consideration for others, kindness, and intimacy are valued, but in ASPD these patterns are underrepresented.

Overdeveloped and underdeveloped behavioral strategies

In personality disorder, certain types of behavioral patterns are overdeveloped and others are underdeveloped. These patterns are related to each other in that the behavioral patterns that are overdeveloped appear to be the reciprocal of the underdeveloped behavioral patterns. It is not that the various overdeveloped behavioral strategies seen in personality disorder are, in themselves, without their usefulness. The problem arises when they are applied in a pervasive, inflexible, and exaggerated manner that is inappropriate to the situation.

Developing a coherent cognitive formulation of an individual's problems is therefore central to progress in therapy. As the patient's past history plays a role in the development of problems, there is a greater degree of historical information sought in cognitive therapy for personality disorder and in arriving at the formulation compared with therapy of an Axis I disorder. In an Axis I disorder, the patient's past history is usually helpful in highlighting potential vulnerability factors rather than being central to pathology.

The function of the formulation is to engage the patient in therapy by making explicit the relationship between beliefs about self and others and long-standing behavioral patterns, which in the case of those with ASPD, particularly those that are self-destructive or have a destructive effect on others. The cognitive formulation

takes into account that patient's early as well as recent experience. Reaching a formulation is a time consuming process for the therapist but one that is essential if therapy is to remain structured and focused. It will point to the underdeveloped behavioral patterns that require to be strengthened and uncover the content of core beliefs about self and others that need to be replaced by more adaptive beliefs. Arriving at a formulation is not an intellectual exercise for the therapist as it must make sense to the patient and be readily understood in terms of his past experience. It is also the process through which the patient engages with therapy and through which the therapist demonstrates that he has been understood.

As with all therapies with individuals with ASPD, there should be a transparent overall structure to therapy. Generally, ASPD patients are offered up to 10 sessions at the beginning of therapy to assess problems and agree a formulation. If it has been possible to agree the formulation and identify underdeveloped behavioral strategies and beliefs that are unhelpful and counterproductive to change, then the therapist negotiates the next stage of therapy focusing on change.

Cognitive therapy for those with ASPD will usually focus on developing and strengthening more adaptive behavioral strategies aimed at improving interpersonal relationships and managing conflict by learning to see the perspective of the other. Low self-esteem, a frequent problem, is often associated with negative thoughts towards others. Beliefs such as I must not show that I am weak and I have to get the better of everyone or they will get me need to be loosened or weakened by strengthening new more adaptive beliefs such as it is okay to have both strengths and weaknesses and if I behave in a reasonable way with others, they may treat me better.

The treatment of antisocial personality disorder in prison therapeutic communities

In various jurisdictions, therapeutic work is carried out with ASPD clients in prisons. The advantages to the provision of such treatment in a prison setting include the presence of a captive audience ASPD patients during prison sojourns are predictable in their location, unlike much of the rest of their lifestyle; the hotel residential costs of the program (often the most expensive aspect of residential ASPD treatments) are assumed as the clients are incarcerated anyway, and because the outcome of the treatment process (greater insight and understanding of the context of the offending cycle) can be fed into the parole and pre-release risk assessment process.

Broadly there are two TC cultures in prisons. On the one hand, there are concept or hierarchical communities, based on a charted progression in treatment, for example away from drug use. Many prison drug TCs have this culture, with which parallels can be drawn with the self-help and 12-step movements originally pioneered for alcoholism.

The TC and group psychotherapeutic technique enables the power, authority, and developing (hard won) insight of peers to be utilized in the treatment of newer patients, and in the formulation and delivery of insights that would be unheard if formulated by staff. The maintenance of a culture of enquiry about all aspects of living together promotes deep exploration of current behaviors as they reenact the index offending pattern and often aspects of developmental experience. Gradually, the ASPD patient can begin to integrate aspects of their personality, the often extreme levels of rage and destructiveness sparked or potentiated by developmental traumas, and the shame and loathing that can be both the precipitant and the result of the offending.

Narcissistic personality disorder

Kohut's is a developmental theory of self and not a personality characterization. Nevertheless, it leads to a clinical picture that is at variance with those of Freud, Kernberg, and the DSM-IV. The features that emerge from Kohut's descriptions have been summarized by Forman (1975). Listed among the more prominent are: (1) low self-esteem; (2) tendencies toward periodic hypochondriasis; and (3) feelings of emptiness or deadness. To illustrate their contrasting views, for example, the episodic depression that Kohut finds so

characteristic of narcissistically injured persons is not seen by Kernberg to be a true depression at all. Rather, Kernberg contends that when â€˜narcissistsâ€™ feel seriously disappointed or abandoned they may appear depressed on superficial examination, but they are, in fact, smoldering with constrained anger and revengeful resentment.

Kohut's model encourages the therapist to assume a sympathetic and accepting stance, while addressing the objective need for the patient to accept personal limitations. Short-term methods may be especially useful for crisis intervention and to establish a bridge to more long-term treatment procedures. Binder (1979) reports on the use of a brief treatment method for increasing self-esteem, also in preparation for a longer-term program. The hope here is to increase the patient's awareness of his/her vulnerability to shame and disappointment, as well as to increase the capacity to moderate intense affects, such as irritability and rage.

The cognitive-behavioral approach

Differing significantly from the traditional views of psychoanalytically oriented concepts, the cognitive school has followed its model for treating the dysfunctional thoughts frequently seen in cases featuring clinical depression, and recognized that these cognitions regarding the self, the world, and the future also extend to the complex personality matrix beyond immediate clinical symptoms. The narcissistic personality, according to Beck and his colleagues, can be conceptualized as stemming from a combination of dysfunctional schemas. The early foundation of these schemas is developed by direct and indirect messages from parents, siblings, and significant others, and by experiences that mold beliefs about personal uniqueness and self-importance. Narcissists regard themselves as special, exceptional, and justified in focusing exclusively on personal gratification; they expect admiration, deference, and compliance from others, and their expectations of the future focus on the realization of grandiose fantasies. At the same time, beliefs about the importance of other people's feelings are conspicuously lacking. Behavior is affected by deficits in cooperation and reciprocal social interaction, as well as by excesses in demanding, self-indulgent, and sometimes aggressive behaviors.

The various techniques of dysfunctional thought modification, cognitive reframing, and similar techniques espoused by Beck (and many other cognitively oriented thinkers, e.g., Sperry, 1999) are well-known and highly validated components of contemporary clinical practice. More recently, Young (1999) has provided insightful cognitive schema-focused inroads to the challenges of personality pathology.

The interpersonal communications approach

Timothy Leary (1957), a disciple of Horney and others of the social and interpersonal school of thought, pioneered what may be termed the interpersonal approach to the problem of personality pathology, and extended their notions to what he terms adjustment through competition.

Benjamin's (1993) interpersonal approach suggests that achieving the first crucial therapeutic objective, the patient's recognition of problematic interpersonal patterns, is particularly challenging with narcissistic patients. While the therapist's empathic understanding is necessary in facilitating this process, the form of therapist statements needs to be carefully considered to prevent encouraging narcissistic tendencies inadvertently.

Key practice principles

The following represents the senior author's (T.M.) synergistic therapeutic approach for the narcissistic personality patterns that fully integrates the traditional psychotherapy schools reviewed in earlier sections.

Identifying dysfunctional domains

The most salient narcissistic dysfunctions are manifest in the self-image and interpersonal conduct domains, and are expressed in the form of an admirable self-concept and unempathic, even exploitive treatment of others. At best, the narcissist confidently displays achievements and behaves in an entitled, and occasionally grating manner. Facts are twisted and the line between fantasy and reality becomes blurred as narcissists boast of unsupportable personal successes and talent; at the same time, interpersonal behavior moves toward the inconsiderate, arrogant, and exploitive.

Establishing psychic balances

Characteristic narcissistic confidence, arrogance, and exploitive egocentricity is based on a deeply ingrained, if sometimes fragile, self-image of superior self-worth. Achievements and manifest talents are often not proportional to the narcissist's presumptions of specialness. The alternative to maintaining unsustainable beliefs of personal infallibility, that is, recognition of imperfections, limitations, and flaws, however, is tantamount to reconciliation with failure and utter worthlessness.

Countering perpetuations

Narcissists characteristic difficulties almost all stem from their lack of solid contact with reality. The same disdain for objectivity prevents effective coping with subsequent troubles. The problem-perpetuating cycle begins with early experiences that provide noncontingent praise that teach narcissists to value themselves regardless of accomplishments. Their inflated sense of self-worth causes them to conclude that there is little reason to apply any systematic effort toward acquiring skills and competencies when it is so clear that they already possess such obvious and valuable talents and aptitudes. Therapeutic intervention offers an inroad into the pathological cycle through the modification of the overblown self-image. As the self is appraised more realistically, perfection is seen as unattainable, and the need to employ self-discipline to achieve goals is understood, the narcissistic patient may come to recognize and accept his or her similarity to others.

Challenges

Narcissists are not inclined to seek therapy. Their pride disposes them to reject the imperfection-confirming weak role of patient. Most are convinced they can get along quite well on their own. Often if a narcissist does accept voluntary treatment, he or she will try to enlist the therapist to support the opinion that the patient's problems are largely the result of the imperfections and weaknesses of others. Alternatively, the narcissist may adopt a stance of superiority and discredit the therapist, or terminate treatment prematurely. In sum, narcissists will not accede to therapy willingly. Moreover, once involved, they will maintain a well-measured distance from the therapist, resist the searching probes of personal exploration, become indignant over implications of deficiencies on their part, and seek to shift responsibility for these lacks to others.

Borderline personality disorder

Psychodynamic therapy

In keeping with all other therapies used in the treatment of BPD there is general agreement that dynamic therapy should be adapted from its pure, traditional form if it is to effect personality change. It is more structured, requires an active, participating therapist (rather than the archetypal passive, unresponsive therapist), may combine individual and group therapy, and commonly agrees treatment priorities with the patient. This is illustrated by TFP. TFP relies on the techniques of clarification, confrontation, and transference interpretation within the evolving relationship between patient and therapist. The primary focus is on the dominant affect-laden themes that emerge in the therapeutic relationship in the here-and-now of the transference. At the beginning of treatment a hierarchy of issues is established: the containment of suicidal and self-destructive behaviors, the various ways of destroying the treatment, and the

identification and recapitulation of dominant object relational patterns as they are experienced and expressed in the here-and-now of the transference relationship.

The initial aims in dynamic therapies are to engage the patient in treatment and to develop a secure enough relationship to allow the patients inner representational world to become manifest in the relationship with the therapist. There is a continuum of intervention that moves through affirmation, advice and praise, empathic validation, encouragement to elaborate, and clarification to confrontation and interpretation (Gabbard, 1999). Only interpretation is specific to the psychodynamic approach. Distortion of representations is challenged or confronted and particular emphasis is placed on defense mechanisms as they operate within the therapeutic relationship. Misrepresentation of self and other representations in BPD arises because of splitting and projective identification in which the therapist himself may be changed emotionally. Containment and confrontation of anger and self-destructiveness is also a core aspect of treatment as aggression and its unmodulated expression is thought to be important as a cause of many of the borderline symptoms. The therapist should not act in the heat of the moment but analyze the underlying causes of outbursts once a crisis is over, linking feelings with actions. The aim is to make unbearable feelings bearable, the bewildering occurrence into an understandable experience, and the undigested trauma of the past into an assimilated event of the present.

In psychodynamic treatments suicide is considered from a relational and intrapsychic perspective by understanding the interpersonal context in which suicidal acts occur and identifying indicators within the transference relationship that may predict suicide attempts. The individual therapist needs to build up a picture of suicide episodes of not only by itemizing the antecedents and outcome of the episodes but also by identifying the concurrent mental experience at the time and the exact context. Clinicians are familiar with the enormous fear of physical abandonment in borderline patients. This, perhaps more than any other aspect, alerts clinicians to increased risk especially if the other, perhaps the therapist, is needed for self-coherence. Patients must be prepared for therapist absences and clear contingencies made if suicide risk increases.

All clinicians recognize that affective instability causes marked problems in treatment. Rapidly shifting emotional states, often triggered by apparently minor incidents, lead to sudden rejection of therapy, increasingly unreasonable demands, refusal to engage in dialog, clinging desperation, and impulsive actions. Within dynamic therapy affective instability is viewed, in part, as secondary to instability of the self so that whenever the self is threatened emotional storms persist until stability of the self is regained. Intervention therefore focuses on helping patients understand their intense emotional reactions in the context of the treatment setting and identifying aspects of interpersonal interaction that have stimulated the feelings. In transference focused work it is necessary to identify the predominant object relations active at the point of rage and to identify aggression that is seen as a problem through its effect on internal representations, which become unstable because of the borderline individual's difficulty in integrating positive and negative representations.

Dialectical behavior therapy

The core strategies of DBT include: behavioral analysis, solution analysis and solution strategies, skills training (acquisition and strengthening of new skills), insight strategies, contingency management, exposure, cognitive modification, didactical interventions, orienting strategies, and the acquisition and strengthening of commitment. Because it is important that patients show effective behavior outside the therapy, generalization of new, adaptive behaviors needs to occur. Generalization is aimed at during role-play in the group session, in homework assignments of the skills training and through the phone consultation. DBT differs from pure behavior therapy to the extent that it integrates acceptance-based approaches with cognitive-behavioral change-based procedures. Validation, mindfulness practices, reciprocity, and a focus on the patient therapist

relationship are integrated with basic behavioral procedures of skills training, exposure-based procedures, cognitive modification, contingency management, and problem solving.
The goals of treatment for the first stage of DBT treatment are behavioral control, stability, and connection with treatment and care provider. Comprehensive DBT treatment for individuals in stage 1 includes five important functions necessary to decrease dysfunctional behaviors, to increase functionality, and to enhance quality of life. These functions include: (1) enhancing behavioral capabilities; (2) improving motivation to change; (3) assuring new capabilities generalize to the natural environment; (4) structuring the environment in the ways essential to support client and therapist capabilities; and (5) enhancing therapist capabilities and motivation to treat patients effectively.

These five functions are addressed within four different standard treatment modes of DBT lasting for 1 year: (1) weekly individual cognitive-behavioral psychotherapy sessions with the primary therapist; (2) weekly skills training groups lasting 2.5 hours per session; (3) weekly supervision and consultation meetings for the therapists; and (4) phone consultation, where patients are encouraged to get coaching in the appliance of new effective skills by phoning their primary therapists either during or outside office hours, for in vivo skills coaching to avert crises, facilitate skills generalization, and to repair between-session conflicts or misunderstandings between therapist and client. Individual therapy focuses primarily on motivational issues, including the motivation to stay alive and to stay in treatment.

Suicide and self-harm

In the pretreatment phase of DBT a global analysis is made of suicidal behaviors. This is done from a behavioral perspective by assessing the level of seriousness, in what way the suicide attempts take place, are predictive

factors distinguishable, and what are the reinforcement contingencies. To some extent DBT elides suicide and self-harm into parasuicide so the treatment strategies are similar.
A suicide protocol is commonly made (what to do when your therapist is not available, who can be reached, what arrangements need to be made), based on an assumption that, although the goal is to prevent hospitalization, it can be needed to cope with a crisis situation. A contract may be set that outlines clearly the role of the patient and the therapist when suicide threats and suicide attempts occur. In DBT the primary objective is to teach patients to manage their own lives via the consultation-to-the-patient principle. DBT teaches patients how to deal with parasuicidal behavior. Patients are coached in vivo in managing parasuicidal behavior and crisis situations through the phone consultation with the individual therapist. Suicide contracts have been recommended in the treatment of BPD and some clinicians seek the patient's agreement of no self-harm as part of the treatment contract, although it is of dubious value if used as an aid to reduce serious suicidal behavior (Kroll, 2000).

Affective instability and crises

DBT follows the similar principles to those used in suicidal behavior during acute crises. The therapist coaches the patient in answering the question what to do. Because borderline patients often have learned to ask for help in a manipulative way one of the goals in DBT is to teach the patient appropriate help-seeking skills.

Cognitive-behavioral therapy

CBT for BPDs differs from standard CBT (Davidson, 2000). Treatment is longer, there is greater emphasis on the therapeutic relationship, a focus on affects in conjunction with core beliefs, encouragement to develop

new ways of behaving and thinking, and past history is explored to understand the context in which the core beliefs have developed.

Core beliefs are identified with the patient and are commonly at an extreme: I am no good, I am very clever, I am special, I am worthless, Other people cannot be trusted, People will abandon me. Work on the core beliefs is combined with identification of more intricate relationships between affects, cognitions, motivation, and actions. These schemas are elaborated though direct questioning of the patient. A functional analysis is performed considering the affective element, the cognition, and their effect on self-regulation, motivation, and action.

Suicide and self-harm

Few specific strategies for treatment of suicide attempts and self-harm are described in CBT and the techniques are those used to tackle any problem behavior. Decreasing self-destructive behavior and behaviors that cause harm to others is an initial target of treatment and the main strategy is to understand self-harm or suicide attempts though a formulation of problems identifying the relationship between core beliefs and self-harm behaviors.

Affective instability and crises

Once again affective outbursts are treated within a cognitive frame of identifying core beliefs, linking them to motivations and finding alternative pathways of expression. The patient's ways of coping are reviewed and any important events since the previous session are explored.

Integrative approaches: cognitive analytic therapy

CAT is offered for longer when used to treat patients with BPD than is traditional in the treatment of neurotic disorders. Twenty-four rather than 12 sessions are offered and the primary aim is to gain a developmental and social understanding of the patients problem and to share this in a clear, user-friendly way. Understanding is operationalized as defining reciprocal emotional roles that are exacerbated or perpetuated by redundant coping procedures. Many of these may have been effective solutions to childhood distress but are now outdated and inappropriate. These reciprocal roles are viewed as being enacted within the relationship to the therapist and working them through in treatment is at the heart of the therapy. The work is active and shared. Diagrams and outlines of problems are written down and emotional roles are drawn out to become tools for use within and without therapy the practical manifestation of the PSORM. The axiom is reformulation of problems, recognition, and revision.

The initial sessions involve clarification of difficulties and completion of a psychotherapy file, which asks about typical common problems known as traps, dilemmas, and snags. The problems are explored and at around the fourth session the therapist presents the patient with a reformulation. This includes a description of the patients life, their difficulties, and a formulation of their problems as target problem procedures that become the focus of therapy. This may also be presented as a sequential diagrammatic reformulation (SDR) and the patient is asked to reflect on it and to make modifications until a shared understanding is arrived at. The SDR is a jointly constructed diagram of a patient's interpersonal function that maps the movement of their feelings and resulting behaviors.

Histrionic personality disorder

Cognitive therapy treatment

The structure of cognitive psychotherapy for personality disorder patients is much more complex than the treatment for patients with Axis I disorders alone. Special care must be taken to evaluate and understand the

underlying schematic structures as multidimensional forces pressing on the person's cognitive, behavioral, and affective interpretation of any and all stimuli. Suggested modifications of treatment include increased focus on the therapeutic relationship, increased emphasis on developmental events, individualized variations in session structure, and utilization of specialized strategies to alter dysfunctional beliefs and compensatory behavioral strategies (J. Beck, 1998).

The full range of cognitive and behavioral techniques, as outlined by J. Beck (1995) are applicable to HPD patients. In fact, using a variety of techniques will ensure that therapy remains interesting, and therefore important to the person with HPD. As with most of their patients, cognitive therapists help HPD patients collaboratively set incremental, short-term goals, which are meaningful to the patients. Encouraging the patient to write each goal to increase commitment, reduce premature termination, and produce stronger shifts in cognition (Cialdini, 2001). Generated goals are specific, measurable behavioral tasks that serve to challenge maladaptive cognitions while progressively moving the patient closer to their long-term goals (Bordin, 1979; A. T. Beck et al., 2003). Therapists also formulate in their own minds several important goals for them to work toward with their HPD patients: learning to slow down, interrupt their impulsive behavior, and modify their global emotional reasoning style. At each session, patients and therapists collaboratively set an agenda and orient the session toward helping patients solve their problems. Behavioral skills training and cognitive restructuring are important components of the problem-solving process. A particularly useful technique is behavioral experimentation outside of the therapy office. Using a collaborative style, the therapist and the patient design experiments to test a new behavior or cognitive response.

Psychodynamic components of treatment

Psychodynamic treatment goals include the gradual uncovering of the HPD patientsâ€™ underlying conflicts and the development of insight into the highly exaggerated behavior. The primary focus of therapy is to â€˜address the resistance before attempting to interpret the underlying contentâ€™ (Gabbard, 2000, p. 529). Issues of interpersonal style, family relationships, behavioral repertoire, and schema are brought out by examining current relationship situations and exploring where the patient first learned the behavior described. It has been hypothesized that HPD families of origin are high in control, highly intellectual-cultural, and low in cohesion (Baker et al., 1996). The parents in these families were most likely self-absorbed with difficulty expressing sincere, deep, and genuine emotion. If this hypothesis holds true, uncovering the schema related to the use of superficiality, every man for himself and I must be first, needed, etc would assist the patient in normalizing their development and choosing a healthier alternative.

The most useful technique for uncovering these schema is the use of Socratic questioning. Using this technique early in therapy acclimates the patient to the style and encourages self-exploration. As the therapy moves forward it is much more powerful for the patient to uncover latent or inactive schema rather than the therapist providing expert intellectual interpretation that actually reinforces the family of origin dynamic. As the patient uncovers their own dynamics the experience increases the feeling of independence, reduces dependent behavior, increases the use of problem-solving skills, reduces impulsive conclusion formation, and reinforces more adaptive thinking.

Psychotherapy for avoidant personality disorder

In evaluating the effectiveness or efficacy of treatments for AvPD, multiple outcome criteria can be used such as: (1) degree of social interaction; (2) levels of anxiety and dysphoria; (3) changes (improvements) in beliefs specific to AvPD; and (4) degree of AvPD symptomatology as per the DSM-IV.

Psychodynamic-based models

The SE approach to AvPD puts great emphasis on giving the patients empathy for their humiliation, embarrassment, and shame surrounding interpersonal situations, and on pointing out how anxieties occurring in the transference with the therapist may provide useful information about similar anxieties in other relationships (Gabbard, 2000). Therapists help their patients explore important etiological and developmental factors pertinent to their expectations for failure, rejection, and loss of nurturance. While some focus is placed on the patient's actual past interactions with primary caregivers, additional attention is paid to the patient's unconscious impulses and fears that have led to avoidant behavior. For example, a patient may harbor an unacknowledged expectation that he will lose control in social situations, revealing primitive feelings that will lead to shame and rejection (e.g., expressing sexual interest in an inappropriate love object), and/or threat of retaliation and harm (e.g., expressing heretofore hidden anger toward a parental figure, such as an employer, mentor, or therapist).

Interestingly, psychodynamic approaches dovetail with cognitive-behavioral methods in emphasizing the importance of patientsâ€™ gaining exposure to feared situations (Gabbard, 2000). This â€˜exposureâ€™ includes the in-vivo work involving the therapeutic relationship, where the AvPD patients may otherwise be hesitant to discuss highly emotional material, to discuss serious topics, to think about planning for the future and making personal changes, and to settle into a comfortable, open, trusting relationship with the therapist.

Cognitive-behavioral models

Targets for intervention typically include the patients inhibited social performance, their aversion to growth-related discomfort, their tendency to engage in marked procrastination, and their expectations for interpersonal censure and rejection, to name a few. Additionally, cognitive-behavioral therapists will assess, highlight, and try to facilitate the modification of the AvPD patients specific beliefs and related schemas that maladaptively shape their perceptions of themselves and their interactions with others. Interestingly, cognitive therapy also focuses on the patients avoided emotions (Newman, 1991), and thus bears some similarity to alternative models such as focused-expressive psychotherapy (Daldrup et al., 1988). Imagery techniques are commonly used to heighten affect in the hope that hot cognitions (see Greenberg and Safran, 1984) will be accessed that are most relevant to the patient's problems emotion-laden thoughts that the patients usually do not notice, acknowledge, or reveal easily.

A cognitive conceptualization of a typical AvPD case often reveals that the patients demonstrate strong approach-avoidance conflicts about closeness with other people, leading to high anxiety, shying away, loneliness, and reduced opportunities to learn social skills. They have a low sense of self-efficacy reinforced in part by their lack of practice in volunteering for challenging tasks and interpersonal engagement along with a strong belief that others will punish them for their failures. They compensate for these problems by reducing stimulation and risk (as they see it) as much as possible. They minimize their exposure to social situations, academic/ vocational tasks, interesting and novel life experiences, and even their own most important thoughts, emotions, and memories. In short, these patients fail to show up for their own lives (Newman, 1999), and become very dissatisfied with the relatively empty results.

Graded tasks

In order to manage the anxiety that accompanies therapeutic changes, cognitive-behavioral therapists assist their patients in structuring tasks that are graded from easiest to most challenging. In other words, patients learn that therapy generally does not involve dramatic breakthroughs as much as gradual improvements. Behavioral changes and cognitive changes reinforce each other in a virtuous cycle, and the patient's emotional and interpersonal life improves in the process.

Role-playing

Another important technique is role-playing, most often involving social situations requiring assertiveness, congeniality, or general conversational skills. Patients with AvPD often need considerable practice in acquiring or reinforcing such skills, and the therapist's office provides a unique opportunity to rehearse these behaviors without the threat of social faux pas. Unfortunately, AvPD patients often fear making mistakes in front of their therapists, and may even be sufficiently ashamed that they do not want to do role-plays even if nobody but themselves is around to judge. Thus, AvPD patients frequently decline to take part in role-playing, which becomes a therapeutic issue.

Group therapy

Group models of psychotherapy (e.g., Yalom, 1980, 1995) tend to focus more on the process of treatment in the here-and-now of the group session than on the individual etiologies of the group participants. Depending on the theoretical orientation and training of the group leader, the explanations for each group members AvPD characteristics may focus on such factors as early-life rejection, humiliation, and shaming, schemas of incompetence and social undesirability, magnified sense of risk coupled with minimal confidence in personal resources, and the negative reinforcing value (and life inhibiting results) of avoidance. However, the purpose and function of group therapy is to use the group dynamic to test participants expectations for negative judgment and rejection, to connect with others in a meaningful way, and to learn interactional skills that can improve interpersonal performance in everyday life. In the meantime, group members learn to tolerate the experience and expression of affect, both within themselves and in front of others.

Dependent personality disorder

Individual dynamic psychotherapy

In the dynamic psychotherapy literature there is apparent consensus about two central aspects in the therapy of DPD. The first is that the emergence of a dependent transference toward the therapist should be addressed in a way to promote emotional growth. The second is that therapist expectations and direct support should be used to promote self-expression, assertiveness, decision making, and independence. If both aspects are not addressed, treatment may be incomplete (Hill, 1970; Saul and Warner, 1975; Malinow, 1981; A. T. Beck et al., 1990).

At the outset of therapy, it is important to aid the development of a trusting relationship and allow the patient to begin to transfer dependent wishes on to the therapist. Hill (1970) suggested telling the patient that extra sessions may be allowed early on in therapy, especially around the patient's episodes of panic or distress. This assurance of readily available support helps the patient develop trust, and aids alliance formation. As therapy progresses, the therapist may help the patient find substitute ways of dealing with such feelings and limit extra sessions.

The hardest work of therapy occurs when a patient experiences increased dependency on the therapist and simultaneously has setbacks or losses in his or her outside life. Offering sympathy for the patient's distress is not helpful alone (Hill, 1970). The therapist should also encourage the patient to express real feelings and wishes and to bear the anxiety of making decisions, accepting pleasurable experiences, and dealing with episodes of anxiety. During the final stage of therapy, the therapist gradually increases the level of expectations for autonomous decision making and action and for socially effective responses (Leeman and Mulvey, 1975). This includes reinforcing the individual's increasing ability to handle crises without extra sessions, to manage anxiety/panic episodes by self-soothing rather than by seeking reassurance from others (Hill, 1970). The therapist must help the patient to resolve transference wishes to be dependent and fears of aloneness, powerlessness, and others intolerance for self-initiated expression and action, while accepting instead a more self-reliant position in relationships.

Cognitive-behavior therapy

Turkat and Carlson (1984) reported two successive behavioral treatments of a patient with DPD. The patient had initially been treated with behavioral techniques for anxiety-related complaints but had relapsed immediately after termination. The authors then reformulated the case, focusing on the dependency constructs of excessive reliance on others and deficient autonomous behavior, which they posited resulted from long-standing anxiety over independent decision making. The therapist and patient constructed a hierarchy of situations with which the patient had little experience but about which the patient was required to make independent decisions. The therapist emphasized previously taught anxiety management skills. Treatment proceeded every other week for 2 months. As therapy progressed, the patient showed decreasing levels of self-rated anxiety, and less avoidance of situations requiring independent decisions. The gains were maintained at 1-year follow-up.

A. T. Beck et al. (1990) and J. S. Beck (1997) have described cognitive-behavioral treatment for DPD. As in dynamic therapy, they view the patient therapist relationship as a microcosm of the patient's dependent beliefs and behaviors. The therapist must foster the therapeutic alliance early and adjust the therapeutic approach somewhat to maintain it. For instance, some patients need to begin a session telling the therapist whatever is on their mind, in order to cooperate subsequently with more directed or structured tasks. The therapist formulates the case and then chooses each technique to foster accurate self-appraisal and independent decision making and behavior. The patient's dependent behavior is initially accepted, but the therapist encourages self-reflection and agenda setting for sessions.

Independence is first encouraged by helping the patient set goals for treatment. Using a Socratic method avoids directing the patient's agenda. The therapist continually challenges the patient's dichotomous thinking (e.g., If I am not fully successful, then I'm inadequate) to improve self-evaluation. Successful graded exposure to anxiety-provoking situations in real life challenges the patient's belief about being incompetent. Patient diaries can be used to monitor the patient's automatic thoughts, especially of inadequacy, highlighting their negative consequences. The therapist can challenge the patient to select healthier responses that aid the development of positive schemas. Relaxation training may aid in the reduction of anxiety surrounding independent reflection and decision making. Assertiveness training and role playing may help counter submissive behavior whenever real skill deficits exist.

Group psychotherapy

Several reports suggest that group psychotherapy can be successful for the treatment of DPD. Montgomery (1971) used group therapy for dependent patients who used medications for chronic complaints such as insomnia and nervousness. Sadoff and Collins (1968) employed weekly group psychotherapy for 22 patients who stuttered, most of whom had passive-dependent traits. Although the dropout rate was high, the authors found that the interpretation of passive-dependent behavior and attitudes (e.g., asking for help, believing that others are responsible for helping them) as a defense against recognizing and expressing anger proved helpful. Both stuttering and passive dependency improved in two patients who became angry and were able to confront their anger. Attrition may be higher in group than individual therapy for PDs (Perry and Bond, 2000), although may be less of a problem for individuals with DPD. Budman et al. (1996) demonstrated moderate improvements after an 18-month group for PDs (10% with DPD), although some changes were not evident until 6 months. These reports suggest the usefulness of group psychotherapy in the treatment of DPD. Most clinicians employ weekly sessions of 1/2 hours duration. Sessions may be more frequent when group therapy is used as a major treatment modality in a day or residential treatment setting (Piper et al., 1993). Outpatient group therapy generally lasts 6 months to several years.

Psychotherapy of obsessive-compulsive personality disorder

Key practice principles

Psychodynamic

A general practice principle in treating patients with OCPD is that the intrapsychic defense mechanisms will be transformed into resistances as a psychotherapy process begins. If a patient characteristically intellectualizes, for example, as a way of fending off affect, that same pattern of intellectualization will occur when the therapist attempts to explore the patient's feelings. Patients may identify facts and gather data as a way of not dealing with feelings either directly toward the therapist or outside the therapeutic situation. Because lack of control and emotional spontaneity are among the most dreaded possibilities with someone with OCPD, patients with this disorder will often attempt to maintain firm control over what transpires in the session. A typical pattern of resistance to the free flow of associations and the exploration of feelings as they spontaneously occur is to structure the session by bringing in an outline of topics the patient wishes to cover. A useful strategy to deal with this defensive style is for the therapist to make active efforts to help the patient identify feelings. When the patient provides a long factual account of events, it is helpful for the therapist to ask, But what did you feel in reaction to those events The therapist can also be active in making observations about feelings that slip through the defensive barrier. Another key practice principle is modification of the patient's harsh superego. The patient's punitive self-critical tendencies repeatedly get in the way of open and free exploration in the treatment process. Therapists must constantly look for ways to help patients accept their humanness. Reassuring the patient of his or her essential goodness is usually not effective. A nonjudgmental stance by the therapist is essential, and from this nonjudgmental perspective, the therapist can interpret conflicts around aggression, sexuality, and dependency. While OCPD patients will repeatedly attribute punitive attitudes toward the therapist, a consistent, nonjudgmental, accepting attitude over time will help patients begin to see that they are projecting their own self-critical nature on to the therapist. There is a cumulative effect of repeated interactions in which the therapist does not behave as the patient expects, leading to a gradual internalization of the therapist associated with a corresponding modification of the patient's superego (Gabbard, 2000). The therapist also looks for opportunities to point out to OCPD patients how their defensive style interferes with pleasure outside of therapy. The tendency to overwork and to ignore intimate relationships can be an active focus of the treatment. Pointing out the patient's difficulty in prioritizing and delegating may be useful. In interpersonal relationships, the patient will describe interactions that reveal the discrepancy between how he views himself and how others view him. The therapist should systematically address the patient's behaviors, both in the transference and outside the transference, that produce certain reactions in others.

Cognitive/behavioral

In cognitive therapy, patients learn to assess their own thoughts and beliefs, and to make modifications based on the evidence of their life experiences, and on the basis of an objective evaluation about what would serve to improve the quality of their lives. Some OCPD patients are adept at recognizing the demanding and punitive nature of their thoughts, while others need the therapist to offer hypotheses for their consideration. Whatever the route to better understanding, it is important for patients to come to appreciate the impact that their subjective construal of themselves and their world play on their emotions and actions. A typical intervention, therefore, is to list and examine some of the typical beliefs that OCPD patients maintain, such as I am a failure if I make a mistake, and I must stay in complete control or else I will fall apart. Open-ended questioning also known as guided discovery and the Socratic method are very useful in testing such rigid, problematic beliefs. Rather than simply telling OCPD patients that their beliefs are maladaptive and instructing them on what they should believe instead, cognitive therapists ask patients questions such as, How else could you maintain high

standards, and yet not be so punitive toward yourself or others? or Under what conditions could a person such as yourself show a lot of emotions sadness, exuberance, grief, love, and so on and yet still feel reasonably secure, safe, and even proud of yourself? These are important thought exercises that are intended to stretch the OCPD's conceptual comfort zone, while reducing the risk of incurring a power struggle in the therapeutic alliance.

An important goal in treatment is for the OCPD patient to learn to be more tolerant of mistakes. Patients often misconstrue the intent of this goal, believing that the therapist is asking them to lower their standards. Quite to the contrary, the therapist is trying to help patients to raise their standards in terms of risk-tolerance, willingness to do difficult tasks with uncertain outcomes, composure under duress, and benevolence toward the demonstration of human flaws in oneself and in others. The only variable the therapist is trying to attenuate is the degree of punishment that the patients heap on themselves as a result of their perceived miscues and shortcomings.

GRIEF THERAPY

Most people experience the loss of a loved one as a painful, confusing, and disruptive event. Several approaches to therapy with grieving persons have been widely applied. The two most well-known models of grief therapy are J. William Worden's "tasks of mourning" and Robert Neimeyer's "meaning reconstruction." Both of these approaches offer valuable specific therapeutic interventions. When applied well, they allow for the expression of feelings and provide bereaved persons with important opportunities to make sense of their grief experiences.

Four tasks of mourning

There are four tasks in the mourning process, according to William Worden. The first task is to accept the reality of the loss (death). The second task of mourning is to acknowledge and work through the pain of grief. This is a particularly difficult task because no one wishes to feel such pain. First, grieving persons may actively seek to avoid painful feelings and thoughts. They may, for example, use thought stopping, they may idealize the deceased to allow themselves only fond memories, they may use alcohol or other drugs, or they may even try a "geographic cure" (e.g., selling the house and moving away). Second, friends may prefer to avoid the pain of the grieving person's experiences. This leaves the griever isolated. Finally, therapists themselves may avoid the griever's pain in therapy. The third task of mourning is to adapt to an environment in which the deceased is missing. This model has several strengths. It places a strong explicit emphasis on the feeling experiences of grieving persons. It is relatively easily understood and applied without necessarily relying on its psychodynamic underpinnings. Perhaps most significant, by presenting "tasks" it provides a structure for the grieving person as well as for the therapist. In the face of the intrinsically confusing experiences of grief, a set of tasks can provide grieving persons with a sense of order, self-control, and self-determination in their lives. The main criticism of this approach is its implicit stage like quality. It is very tempting to fit all grieving individuals into a mold of "working through the stages" (tasks) of grieving. Nothing could be less therapeutic.

Meaning reconstruction with grieving persons

Another widely accepted approach to grief therapy is a reconstruction of meaning model. Robert Neimeyer, the editor of the journal *Death Studies*, has most effectively articulated this highly idiographic model. This approach emphasizes the uniqueness of individuals' experiences of grieving. The basic premise is that the revision of one's life story in response to a loss is the central process in grieving. Grief therapy is, therefore, primarily an opportunity for grieving persons to tell their stories in ways that will help them to make sense of loss and of life. This approach views life itself as a story-telling process. People "write" and rewrite their stories and thus develop a sense of meaning, purpose, and identity. Losses, particularly through death, disrupt

our stories. They may invalidate or threaten beliefs and assumptions that have lent meaning and order to our lives. Significant losses often trigger a search for meaning. Consequently, grief therapy is focused on a retelling or reconstruction of the grieving person's narrative. This work is done in a way that will allow the survivor to once again find meaning in life in the context of a new reality.

Theoretical bases

Grief therapy, whether utilizing the tasks of mourning or focusing on the reconstruction of one's meanings, can be understood as an opportunity for heightened self-perception. Recent studies have indicated that at least 70% to 85% of bereaved persons typically engage in a search for meaning in their lives following a death. This means that for most people bereavement holds the potential to be a time when they are looking at such existential questions as "What is the purpose of my life?" and the corollary "Who am I, after all?" While completing the many therapeutic experiences described earlier, grieving persons observe their own responses to the urgent experience of death and loss. With the therapist's aid, they can examine their own answers to existential questions and learn who they are. If therapists explicitly foster such self-perception, grieving persons have unique opportunities to redefine themselves.

TRAUMA MANAGEMENT THERAPY

Trauma Management Therapy (TMT) is a multi-component behavioral treatment program for chronic PTSD in veterans. It is a comprehensive treatment designed specifically to target various aspects of the clinical syndrome associated with chronic PTSD, particularly reducing emotional and physiological reactivity to traumatic cues, reducing intrusive symptoms and avoidance behavior, improving interpersonal skills and emotion modulation (e.g., anger control), and increasing the range of enjoyable social activities. The program is designed to incorporate exposure therapy, the PTSD psychosocial treatment approach with the most empirical support, with a social skills training component designed specifically for veterans with PTSD. It is a comprehensive treatment designed to address all aspects of the primary clinical syndrome seen in veterans, via a combination of patient education, exposure therapy, social skills training, and relevant homework assignments. It is important to note that this treatment is not merely a combination of exposure and traditional social skills training procedures. Rather, it includes strategies designed to remedy specific difficulties seen in veterans with chronic PTSD, and the particular sequencing and timing of the individual components are thought to contribute to its overall effectiveness. The major components of TMT are described next.

A. Education

All patients are provided with a general overview of chronic PTSD, including common patterns of expression, issues of diagnosis, comorbidity of other anxiety and Axis I disorders, etiological pathways, and a review of current treatment strategies. This phase is important for ensuring that veterans not only develop a realistic understanding about treatment prognoses, but also an overall positive expectancy regarding the efficacy of behavioral treatment. Finally, this phase is used to educate veterans about the treatment they will be receiving and what will be expected from them regarding their participation in TMT.

B. Exposure Therapy

Individually administered intensive exposure therapy is included as the first active component of TMT, because it has been shown to effectively address the unique features of each patient's fear structure, allowing for a reduction in general anxiety, physiological reactivity, and intrusive symptoms. Patients are exposed imaginally to feared or anxiety-producing stimuli in a prolonged fashion until there is a decrease in fear and anxiety (i.e., until habituation is obtained) within session. Repeated contact with the feared stimulus hastens the habituation process and, with sufficient pairings, the stimulus loses its ability to elicit the fear response.

Typically, most veterans with PTSD escape or avoid feared stimuli, which functions to increase the intensity of the fear response. The goal of exposure therapy is to provide prolonged contact with the feared stimuli of sufficient duration that within session habituation occurs.

C. Programmed Practice

The programmed practice component of TMT is implemented in the final seven individual exposure sessions and is a form of exposure that does not require therapist accompaniment (i.e., it is "homework"), but requires careful planning on the part of the therapist and patient together.

D. Social and Emotional Rehabilitation (SER)

A highly structured group (3–5 people) social skills training component (SER) was developed to target PTSD features that are not improved by exposure therapy only. In other words, interpersonal difficulties, commonly associated with chronic PTSD, such as social anxiety, social withdrawal, excessive anger and hostility, explosive episodes, marital and family conflict are targeted via a number of specific interventions. SER includes instruction, modeling, behavioral rehearsal, feedback, and reinforcement.

E. Treatment Implementation

TMT consists of 29 treatment sessions ideally administered over a period of about 17 weeks. Sessions initially occur three times a week through the Exposure phase, then twice a week at the start of SER, and then once a week for the final 10 weeks of the program.

SUBSTANCE DEPENDENCE: PSYCHOTHERAPY

Some form of psychotherapy or behavioral therapy is usually considered as a treatment option for virtually all patients seeking treatment for substance use disorders. Treatment seekers typically represent the more severe end of the spectrum of community members who meet criteria for current substance use disorders. Most of those who seek treatment do so only after numerous unsuccessful attempts to stop or reduce substance use on their own. The alternatives to psychotherapy are either pharmacological or structural limitations from access to drugs and alcohol such as in residential setting. Both these alternatives have limited effectiveness if not combined with psychotherapy or counseling. Removal from the substance-using setting is a useful and, sometimes, necessary part of substance treatment but is seldom sufficient, as is shown by the high relapse rates typically seen from residential detoxification programs or incarceration during the year following the patient's return to the community.

The major strategy that is now common to all currently practiced psychotherapies for individuals who abuse substances is to place primary emphasis on controlling or reducing substance use, while pursuing other goals only after substance use has been at least partly controlled. This means that either (a) the therapist employs techniques designed to help the patient stop substance use as a central part of the treatment, or (b) the therapy is practiced in the context of a comprehensive treatment program in which other aspects of the treatment curtail the patient's use of substances (e.g., methadone maintenance, disulfiram for alcoholics, residential treatment). Because people who abuse substances frequently react to increased anxiety or other dysphoric affects by resuming substance use, anxiety-arousing aspects of treatment are typically introduced only after a strong therapeutic alliance has been developed or within the context of other supportive structures (e.g., inpatient unit, strong social support network, methadone maintenance) that guard against relapse to substance use when the patient experiences heightened anxiety and dysphoria in the context of therapeutic exploration.

Beyond setting differences, psychotherapy for substance use disorder may be delivered in a range of different modalities, such as group or individual. Major advantages to group substance treatment are: (a) cost savings, (b) given the social stigma attached to having lost control of substance use, having group members who acknowledge similar problems can provide comfort, (c) group members who have longer periods of

abstinence can model that attempting to stop using is not a futile effort, (d) group members can act as social supports, and (e) the public nature of the group can provide powerful incentive to avoid relapse. Principal advantages to individual substance treatment are: (a) privacy for members whose careers or reputations may be damaged from more widespread knowledge of their substance use, (b) increased flexibility to address problems that are uniquely relevant to that patient, (c) easier resolution of logistics surrounding therapist caseloads where often there are not enough substance abuse clients to form a group or determent from individuals having to wait to engage in treatment until the group is formed, and (d) therapists may use the one-to-one relationship to explore relational elements not possible in group treatment.

Common Goals and Strategies for Substance Abuse Psychotherapy

This section reviews issues presented by persons who abuse substances that should be addressed, if not emphasized, by any type of individual or group psychotherapy that is likely to be effective. As noted in reviewing the difficulties encountered by early psychodynamic practitioners, the central modification that is required of psychotherapists is always to be aware that the patient being treated is a substance abuser. Hence, even when attempting to explore other issues in depth, the therapist should devote at least a small part of every session to monitoring the patient's most recent successes and failures at controlling or curtailing substance use and being willing to interrupt other work to address slips and relapses when and if they occur.

1. Enhancing Motivation to Stop Substance Use

Even at the time of treatment seeking, which usually occurs only after substance-related problems have become severe, patients usually can identify many ways in which they want or feel the need for drugs or alcohol and have difficulty developing a clear picture of what life without substances might be like. To be able to achieve and maintain abstinence or controlled use, individuals who abuse substances need a clear conception of their treatment goals. Several investigators have postulated stages in the development of ones' thinking about stopping use, beginning with pre-contemplation, moving through contemplation, and culminating with determination as the ideal cognitive set with which to get the most out of treatment.

2. Teaching Coping Skills

One enduring challenge of treating substance abuse is to help the patient avoid relapse after achieving an initial period of abstinence. A general tactic for avoiding relapse is to identify sets of circumstances that increase an individual's likelihood of resuming substance use and to help the patient anticipate and practice strategies (e.g., refusal skills, recognizing and avoiding cues for craving) for coping with these high-risk situations.

3. Changing Reinforcement Contingencies

As substance abuse worsens, it can take precedence over concerns about work, family, friends, possessions, and health. As compulsive substance use becomes a part of every day, previously valued relationships or activities may be given up so that the rewards available in daily life are narrowed progressively to those derived from substance use. When substance use is brought to a halt, its absence may leave the patient with the need to fill the time that had been spent using drugs or alcohol and to find rewards that can substitute for those derived from use.

4. Fostering Management of Painful Affects

Dysphoric affects are the most commonly cited precipitant for relapse, and many psychodynamic clinicians have suggested that failure of affect regulation is a central dynamic underlying the development of compulsive substance use. To foster the development of mastery over dysphoric affects, most psychotherapies include techniques for eliciting strong affects within a protected therapeutic setting and then enhancing the patient's ability to identify, tolerate, and respond appropriately to them.

5. Improving Interpersonal Functioning and Enhancing Social Supports

A consistent finding in the literature on relapse to substance abuse is the protective influence of an adequate network of social supports. Gratifying friendships and intimate relationships provide a powerful source of rewards to replace those obtained by drug and alcohol use, and the threat of losing these relationships can furnish a strong incentive to maintain abstinence. Typical issues
presented by individuals who abuse substances are: (a) loss of or damage to valued relationships occurring when using substances was the principal priority, (b) failure to have achieved satisfactory relationships even prior to having initiated substance abuse, and (c) inability to identify friends or intimates who are not, themselves, abusing substances. For some types of psychotherapy, working on relationship issues is the central focus of the work (e.g., interpersonal therapy, supportive-expressive treatment), whereas for others, this aspect is implied as a part of other therapeutic activities such as identifying risky and protective situations.

Contingency Management Approaches

Perhaps the most exciting findings pertaining to the effectiveness of behavioral treatments for cocaine dependence have been the recent reports by Higgins and colleagues discussed briefly earlier in the article. In this approach, urine specimens are required three times weekly to systematically detect all episodes of drug use. Abstinence, verified through drug-free urine screens, is reinforced through a voucher system in which patients receive points redeemable for items consistent with a drug-free lifestyle that are intended to help the patient develop alternate reinforcers to drug use (e.g., movie tickets, sporting goods). Patients never receive money directly. To encourage longer periods of consecutive abstinence, the value of the points earned by the patients increases with each successive clean urine specimen, and the value of the points is reset when the patient produces a drug-positive urine screen.

Cognitive Behavioral/Relapse Prevention Therapy

Another behavioral approach that has been shown to be effective is cognitive-behavioral treatment (CBT). This approach is based on social learning theories on the acquisition and maintenance of substance use disorders. Its goal is to foster abstinence through helping the patient master an individualized set of coping strategies as effective alternatives to substance use. Typical skills taught include: (a) fostering resolution to stop drug use through exploring positive and negative consequences of continued use, (b) functional analysis of substance use, that is, understanding substance use in relationship to its antecedents and consequences, (c) development of strategies for coping with craving, (d) identification of seemingly irrelevant decisions that could culminate in high-risk situations, (e) preparation for emergencies and coping with a relapse to substance use, and (f) identifying and confronting thoughts about substance use.

Motivational Approaches

For individuals with severe dependence who deny the seriousness of their involvement, a course of individual therapy in which the patient is guided to a clear recognition of the problem may be an essential first step toward more intensive approaches. Motivation enhancement treatment (MET) sets out to accomplish this in a brief therapy approach (i.e., 2–4 sessions). Included in these sessions are typically emphasis on personal responsibility for change with advice and change options, objective feedback of impairment, therapist empathy, and facilitation of patient self-efficacy.

Family Therapy

Early intervention with individuals who abuse alcohol has historically been approached in some settings by addressing past crisis caused by the alcohol abuse into one dramatic confrontation by family and friends. This therapeutic approach is designed to combat denial by having family and individual close to the person present the negative effects of the individual's use in attempts to move the individual to agree to get treatment. Moving beyond initial confrontation, others have included family in ongoing aspects of treatment. Edward Kaufman has identified three basic phases of family involvement in treatment: (a) developing a system for establishing and maintaining a drug-free state, (b) establishing a workable method of family therapy, and (c) dealing with

the family's readjustment after the cessation of drug abuse. Where these three stages may vary is based on the substances abused, stage of the addiction, family reactivity, and gender of the individual.

Combining Psychotherapy with Pharmacotherapies

At times psychotherapy may be combined with pharmacotherapies to enhance adherence to the pharmacotherapy or synergistically enhance the effects of both treatments. Even when medications have been proven to be effective, dropout from treatment and compliance have still been a problem. Moreover, there has been no study that has demonstrated that the addition of psychotherapy did not help the medication effect. The most powerful and commonly used pharmacologic approaches to substance abuse are maintenance on an agonist that has an action similar to that of the abused substance (e.g., methadone for opioid addicts, nicotine gum for cigarette smokers), use of an antagonist that blocks the effect of the abused substance (e.g., naltrexone for opioid addicts), the use of an aversive agent that provides a powerful negative reinforcement if the substance is used (e.g., disulfiram for alcoholics) and use of agents that reduce the desire to use the abused substance (e.g. naltrexone and acamprosate for alcoholics).

THERAPY FOR HIV/AIDS PATIENTS

The human immunodeficiency virus (HIV) is a retrovirus of the human T-cell leukemia/lymphoma line and is the causative agent of the acquired immunodeficiency syndrome (AIDS). (Although AIDS is believed to be caused by the human immunodeficiency virus-type 1 (HIV-1), we have for simplicity used the term HIV throughout this chapter.) A unique feature of HIV infection is that it can have a long asymptomatic phase lasting up to 10±15 years (Munoz et al., 1988) followed by the appearance of constitutional signs and symptoms of disease progression. A diagnosis of AIDS is made when a person shows laboratory-based signs of severe decline in the number of T-helper±inducer (CD4) cells (5 200 cellmm73), or clinical signs consistent with the emergence of opportunistic infections and neoplasias. Some of these include: candidiasis of the esophagus, trachea, bronchi, or lungs; cryptoccosis; cryptosporidio- sis with diarrhea persisting longer than one month; cytomegalovirus disease of an organ other than the liver, spleen, or lymph nodes; herpes simplex virus infection causing a muco- cutaneous ulcer for longer than one month; Kaposi's sarcoma; primary lymphoma of the brain; pneumocystis carinii pneumonia; pro- gressive multifocal leukoencephelopathy; tox- oplasmosis of the brain; and a variety of other opportunistic infections and neoplasias.

Behavioral and Psychosocial Factors and Disease Progression

1. ***Behavioral factors***

Behavioral factors such as the use of alcohol, tobacco, caffeine, recreational drugs (cocaine, nitrites) during sex, unprotected anal inter- course, and poor nutrition have also been suggested as possible cofactors of accelerated HIV disease progression.

2. ***Depressive symptoms***

There is considerable evidence that depressive symptoms are correlated with immunosuppression in otherwise healthy individuals (Herbert& Cohen, 1993; Irwin, Caldwell et al., 1990; Irwin, Patterson et al., 1990), though available evidence suggests that depression and affective disturbances may be associated with physical disorders in a bidirectional fashion.

3. ***Stressful life events***

There is now ample literature (Herbert & Cohen, 1993) showing that stressors are associated with decrements in immune function. Some of these stressors may be particularly prevalent among HIV-infected people attempting to cope with their illness and include: bereavement; the stress of being a caregiver for a loved one with a terminal disease; divorce (break-up), and marital distress , Chronic uncontrollable stress has been

operationalized in one form as people dealing with catastrophic environmental changes. These stressors are also associated with immune alteration for instance, both at the Three Mile Island nuclear plant disaster and in Miami after Hurricane Andrew.

4. Coping strategies

There is some evidence that the use of certain coping strategies for dealing with HIV infection may be associated with the rate of disease progression. Ironson et al. (1994) found that the use of denial and behavioral disengagement to cope with an HIV-positive diagnosis, was predictive of lower CD4 counts at 1 year follow-up (controlling for CD4 counts at entry to the study) and greater likelihood of progression to HIV-related symptoms and AIDS at 2-year follow-up. Consistent with these findings, Goodkin et al. (1993) found that use of passive coping strategies (including denial and disengagement) was inversely related to long-term CD4 cell count in HIV-positive gay men. Solano et al. (1993) also found that denial/repression was associated prospectively with the emergence of symptoms in an HIV-positive sample. Mulder, Antoni, Duivenvoorden et al. (1995) found that active confrontational coping with HIV infection was predictive of decreased clinical progression over a 1-year period after taking into account baseline biomedical and behavioral variables. Finally, Blomkvist et al. (1994) followed HIV- infected hemophiliacs longitudinally and found that self-oriented active-optimistic coping behavior was related to prolonged survival after controlling for age and CD4 counts at study entry. Thus, there is some evidence that coping strategies may be reliably associated with certain immunologic and health measures in HIV- infected persons from a variety of risk groups.

5. Social support

A person's social support network may also influence their ability to manage, cope with, and recover from serious illness. For instance, it appears that being involved in a committed relationship (i.e., marriage) is associated with greater survival time among patients who have experienced a myocardial infarction (Wiklund et al., 1988; Williams et al., 1992) or a diagnosis of breast cancer (Neale, Tilley, &Vernon, 1986). However, these associations have not been replicated in other studies. Although a growing body of evidence supports the potential benefits of social ties in healthy people and several chronic disease populations (e.g., cancer patients), scientists are just beginning to examine the health correlates of social support in HIV-infected individuals. A variety of models have been developed to explain the ways in which social support might provide direct influences on psychological and physical well-being (Cohen & Wills, 1985). Some of these direct psychological benefits may stem from the possibilities that social support helps people define certain stressors as being less overwhelming, provides opportunities for people to openly express fears, frustrations, and other emotions, thereby decreasing ruminations and obsessive thinking about stressors. Social support also provides key information and informational support provided by knowledgeable others, which may help individuals to assume a more realistic attitude toward life goals. In addition, social support may help people feel connected, which may increase their sense of self-worth, belongingness, meaning, purpose, and well-being. Finally, social ties may reduce initial feelings of burden caused by the onset of a stressor (e.g., a medical diagnosis), allowing the person to employ active coping strategies to deal with the stressor.

Health Psychosocial Interventions with HIV-infected People

Since 1990 a small collection of studies have examined the effects of psychosocial interventions in HIV-infected populations. These studies used both psychological and immune measures as dependent variables, although none of them have completed long enough follow-up periods to clearly establish a health/disease progression effect. Most of these interventions employ cognitive behavioral techniques and operate in a group-based format.

Common Features of Psychosocial Interventions in HIV Infection

Relaxation aspects

It is noteworthy that many of the psychosocial intervention studies with HIV-infected persons have utilized some form of relaxation- based technique as part of the intervention protocol. This is relevant given the great deal of distress and anxiety that these people experience. With regard to HIV infection, one of these studies found that frequency of home relaxation practice was related to better immunological functioning and slower disease progression in HIV-infected men (Ironson et al., 1994).

Another study examined alternate ways of inducing relaxation in HIV-infected individuals (Ironson et al., 1996) massage therapy because of its expected relaxing effects, and because it is easy to monitor objectively how much massage someone is getting. HIV-infected gay men showed a significant increase in NK cell number, NKCC, the soluble CD8 receptor, and the cytotoxic subset of CD8 cells during the weeks of daily massage intervention compared to the same men's values during a no-massage period.

The cognitive-behavioral therapy (CBT) intervention included training in cognitive restructuring, relaxation exercises, behavior change, assertiveness skills, coping fit for controllable vs. uncontrollable stressors, and information on stress responses.

The experiential therapy (ET) condition was less structured and focused on increasing awareness of the here and now, of their inner experiential process, and restoring congruence between emotional, cognitive, and behavioral schemata. The intervention also attempted to help participants deal with a shortened life perspective and anxiety about future illness and death.

Stress Management Intervention Effects at Different Critical Points in HIV Infection

Some of the most well-documented behavioral interventions applied to HIV-infected individuals have attempted to use procedures that reduce distress, promote self-efficacy, and encourage the use of active coping strategies. Operating from a chronic disease-PNI rationale we have hypothesized that if behavioral interventions can retard or normalize declines in immune status by bringing about these psycho- social and behavioral changes, then they may be able to forestall the onset of disease complications, such as opportunistic infections and neoplasias. For some HIV- infected people these sequelae include being faced with a wide variety of chronic, uncontrollable, and unpredictable stressors (e.g., such as changes in health, job status, health insurance, and medical costs) and a loss of social support resources which is triggered by their own withdrawal, the death of close friends, or the responses of friends and family. These are likely to persist across and grow in magnitude over the course of the infection (see Hoffman, 1991).

This model reasoned that the combination of multiple uncontrollable burdens and declining coping resources may overwhelm HIV-infected persons previously adequate direct coping strategies such as active coping, positive appraisals of burdens, problem-solving, and seeking social support, resulting in the adoption of more indirect strategies such as avoidance and denial, on the one hand, and substance use and possibly risky sexual behaviors on the other. One self-defeating loop that this model highlights follows: the infected person's awareness of the ineffectiveness of their coping modes and strategies may increase their sense of low self-efficacy, helplessness, and hopelessness, which in turn perpetuates social isolation, depressive symptoms, continued substance use, and other negative health behaviors.

UNIT 11 – THERAPY WITH CHILDREN

Play Therapy

Play Therapy is defined by the Association for Play Therapy as “the systematic use of a theoretical model to establish an interpersonal process wherein trained play therapists use the therapeutic powers of play to help clients prevent or resolve psychosocial difficulties and achieve optimal growth and development.” Play relieves feelings of stress and boredom, connects us to people in a positive way, stimulates creative thinking and exploration, regulates our emotions, and boosts our ego. Through play, children can learn about their own capabilities and limitations, social rules, the difference between fantasy and reality, rehearse new skills and come to master challenging and difficult situations.

John A. B. Allan (1997) noted, the difference between play and therapy is the therapist’s ability to think analytically about everything that is going on in the session verbally, nonverbally, and symbolically in the child’s play and artwork.Although everyone benefits, play therapy is especially appropriate for children ages 3 through 12 years old (Carmichael, 2006; Gil, 1991; Landreth, 2002; Schaefer, 1993). Teenagers and adults have also benefited from play techniques and recreational processes.

Use of play therapy with adults within mental health, agency, and other healthcare contexts is increasing (Pedro-Carroll & Reddy, 2005; Schaefer, 2003).The curative powers inherent in play are used in many ways.Therapists strategically utilise play therapy to help children express what is troubling them when they do not have the verbal language to express their thoughts and feelings.In play therapy, toys are like the child's words and play is the child's language. Through play, therapists may help children learn more adaptive behaviours when there are emotional or social skills deficits.

Major techniques to play therapy, which developed following Anna Freud and Melanie Klein’s incorporation of play into their psychoanalytic sessions with children in the 1930’s include psychoanalytic play therapy , cognitive behavioural play therapy, structural play therapy, along with non- directive or child centred play therapy.Jesse Taft (1933) and Frederick Allen (1934) developed several therapy modalities grouped as “relationship therapies” in which the primary emphasis is placed on the power of the emotional relationship between the therapist and the child.

David Levy developed a structured play approach referred to as ‘release therapy’ to help children recreate, through play, their experience of a specific traumatic event. According to him ‘a child that has security, support, and the right play materials is able to play out a previous traumatic event, over and over, until he or she is able to abreact or let go of the fears, anxieties, and responses related to the trauma’.

Solomon developed ‘active play therapy’ which was a method to be used with acting-out and impulsive children. It is based upon the rationale that abreaction can occur through the safety of play and a supportive relationship. A child can play out difficult feelings and experiences, and thereby avoid acting them out through socially inappropriate behaviours.

Axline’s 8 guidelines

1. Therapists must develop a warm, friendly relationship with the child in which good rapport is established as soon as possible.
2. Therapists accepts the child exactly as he or she is.
3. Therapists establishes a feeling of permissiveness in a relationship so that the child feels free to express feelings completely.

4. Therapists is alert to recognize the feelings the child is expressing and reflects those feelings back in such a manner that the child gains insight into his or her behavior.
5. The therapists maintains a deep respect for the child's ability to solve problems if given the opportunity. The responsibility to make choices and institute change is the child's.
6. Therapists does not attempt to direct the child's actions or conversation in any manner. The child leads the way; the therapists follows.
7. The therapists does not attempt to hurry the therapy along. It is a gradual process recognized by the therapists.
8. Therapists establish only those limitations necessary to anchor the therapy to the world of reality and to make the child aware of his or her responsibility in the relationship.

Filial therapy, developed by Bernard and Louise Guerney, was a new innovation in play therapy during the 1960s. The filial approach emphasizes a structured training program for parents in which they learn how to employ child-centered play sessions in the home.During the 1960's, the establishment of counselling and guidance programs in elementary schools contributed significantly to the development of play therapy.Use of play therapy in schools was encouraged in order to meet many developmental, social, preventive, and emotional needs of all children, as well as helping children who were identified as maladjusted.

Process of play

Play therapists use their understanding of play to increase children's coping skills and to translate their understanding of the play to parents, caregivers and educators involved in their lives.In the playroom, under the guidance of a trained play therapist, children can safely confront their problems in a protected play environment. They learn to confine, define and master their problems.Research indicates that an average of 20 sessions is needed to resolve the problems of a typical child coming to therapy.

Things to consider

The arrangement of the toys and the atmosphere in the play therapy setting must provide comfort and consistency so that children will feel safe in acting out problem situations and relationships.All play things should be simple in construction and easy to handle so that the child will not be frustrated by equipment which she cannot manipulate.Moreover, they should be durably constructed, designed to withstand strenuous handling in the playroom.

Some therapists have very few toys and limited visual stimulation in their play rooms because they work with children who are easily distracted. Other therapists have many costumes and a stage in their play rooms because they, personally, are dramatic and like to use drama and enactments with their clients. Other therapists who have a need for order and structure, or who have limited space, have everything built in-shelves, kitchen appliances and a sand box.

Importance of play

Play is the natural world of the child. Children learn about themselves, others, and their world through play. In 1989, the Office of the United Nations High Commissioner for Human Rights identified play as a right for all children everywhere to achieve optimum development.Research in neurobiological development and psychological trauma, as discussed by Phyllis T. Stien, Joshua C. Kendall in 2004 and by Bruce D. Perry and Maia Szalavitz in 2006, has demonstrated how play stimulates the neural structures in the brain and is critical for normal development.

According to Bessel A. Van der Kolk, Trauma often remains stuck in the nonverbal parts of the brain—amygdala, thalamus, hippocampus, or brain stem but the ability to think through life events and the ability to process these events takes place in the frontal lobes of the brain. Play provides physical activity, so that "playing out" the event assists the brain in moving the memory from the nonverbal parts of the brain to the frontal lobes.

Psychoanalytic play therapy

Anna Freud and Klein working in Vienna and London in the 1920's and 1930's, made a lasting contribution to the development of play therapy. The former advocated using play mainly to build a strong positive relationship between child and therapist, the latter proposed using it as a direct substitute for verbalisations. The primary goal of their approach was 'to help children work through difficulties or trauma by helping them gain insight'. They proposed that play uncovered the child's unconscious conflicts and desires and that play was the child's way of free-associating. The prime task of child analysts is to understand and interpret the symbolic content of children's play.

Release therapy/world technique/ structured play therapy

A form of play therapy directed towards helping a child who has experienced a particular painful or traumatic event to work through and gain mastery over the feelings engendered by it. It is based on psychoanalytic idea of repetition compulsion, whereby through re-enacting and re- experiencing a particular event, pent-up or blocked- off feelings are released and eventually extinguished. The therapist provides security, support and the materials needed to recreate a traumatic event so that the child may assimilate the negative thoughts and feelings associate with the trauma. The primary role is to set the stage for emotional catharsis by preparing and directing structured play scenes. Thus, anxiety-provoking situations are introduced into the play setting by the therapist. The therapist intentionally plans play activities that will encourage a strong emotional reaction from the child and refrains from interpretations. The concept of interpretation is to enable the child to connect his/her therapy with the outside world.

Family play therapy

Family Play therapy or Filial therapy was developed by Bernard and Louise Guerney in the early 1960s and is an eclectic technique, combining elements from play and family therapies.This technique includes parent(s), child(ren), and a therapist together in a pre-planned play situation. An approach wherein parents can learn more effective parenting skills and styles of interaction in an environment that provides a medium that is comfortable for their children. This technique allows the therapist to be a role model for parents who previously had been exposed to deficient role models. It also provides a controlled and non-threatening environment in which parents can comfortably experiment with change. It is a highly practical technique—short-term, flexible, and designed to remove families from their recurring cycles of failure and fear of change concerning change itself.

Cognitive behavioral play therapy

Knell introduced the concept of cognitive-behavioural play therapy (CBPT) in which cognitive and behavioural interventions are incorporated within a play therapy paradigm. Knell identified six properties of CBPT:

1.The child is involved in the treatment through play

2. CBPT focuses on the child's thoughts, fantasies, and environment

3. CBPT provides strategies for developing more adaptive thoughts and behaviours. The child is taught coping strategies for feelings and for situations

4. CBPT is structured, directive, and goal oriented, rather than open-ended

5. CBPT incorporates empirically demonstrated techniques, such as modelling

6. CBPT allows for an empirical examination of treatment

Sand tray therapy

Sand tray or Sand box Therapy is a form of experiential workshop which allows greater exploration of deep emotional issues. Sand play therapy is suitable for children and adults and allows them to reach a deeper insight into and resolution of a range of issues in their lives such as deep anger, depression, abuse or grief. This method works especially well with those individuals who are young or have trouble comprehending and talking about difficult issues, such as domestic or child abuse, incest, or the death of a family member.

Helpful for children who :

- Are dealing with **parental conflict, separation or divorce**
- have been **traumatized** (sexual, physical or emotional abuse)
- have been **adopted** or are in **foster care**
- are dealing with issues of **loss**, such as **illness or death** of a loved one
- have been **hospitalized**
- have **witnessed domestic violence**
- Are diagnosed with **Attention Deficit Disorder (ADD/ADHD)**
- have experienced serious **accidents or disasters**

<u>Issues in play therapy</u>

1. The critical need for an increase in the number of mental health professionals trained in play therapy.
2. Play therapists must be responsive to their clients' culture, and as the field grows globally, the application of Western play therapy theories and practices in other cultures is another area of concern.

While some cultural adjustments—like types of toys and materials—are easily accomplished, others are difficult to identify.

3. Play therapy supervision is another critical component in developing well trained, competent play therapists. Bratton, Landreth, and Homeyer first wrote about the importance of play therapy supervision and discussed an intensive supervision model in 1993.

4. Another issue is play therapists' personal problems, which may cause difficulty in therapeutic relationships. Quality play therapy supervision includes working with therapists who have personal issues and difficulties and must prevent those from affecting their counselling.

Therapy with adolescents

Puberty, for example, is accompanied by a range of changes, both hormonal and physical (Richards and Petersen, 1987; Richards et al., 1993). The physical transformations that accompany puberty can be confusing, exciting, and challenging. The effects of physical maturation on adjustment during adolescence, however, are complex. Significant individual differences exist in the age of onset of puberty and in the rate at which physical maturation occurs. Moreover, there can be asynchronies in development across physical, social, and emotional domains. The effects of physical maturation on psychosocial development and adaptation appear to be mediated by a number of factors including gender, age of onset of puberty, the relative maturity of peers, and cultural, familial, and community beliefs about maturation. That said, hormonal changes accompanying puberty appear to have broad effects on adolescent development. They have been associated with changes in expression of anger, oppositionality toward parents and other adults, sexual behavior, aggression, mood, self-confidence, and level of psychopathology. It is not clear, however, that relations between physical maturation and adjustment are direct. Rather, the effects of hormonal changes accompanying puberty appear to be mediated and moderated by psychological, familial, and social variables (Richards et al., 1993).

The effects of puberty on adjustment are clinically important for a number of reasons. Physical maturation during adolescence has significant effects on the social status of the individual, how they view themselves, how their peers see them, and how they are viewed by their family and the larger community. Others expectations for them will change as they mature. Teenagers who appear mature may not, however, be socially, emotionally, and cognitively mature, leading to confusion and conflict. Moreover, teenagers naturally experience a range of thoughts and feelings about their physical and sexual maturation. Their thoughts, fantasies, and expectations about these changes, and their effects on their life and relationships, are worthy of discussion during psychotherapy. This is particularly important when the teen is dissatisfied with the changes in their appearance or the ways that these changes have affected their relationships with others. Physical maturation, and the social changes that accompany it have important effects on adolescent adjustment and can, as a consequence, complicate the practice of psychotherapy with adolescents.

Epidemiology

Epidemiological and clinic-based studies indicate that a substantial percentage of adolescents manifest significant behavioral and emotional difficulties, and that these problems can have adverse effects on their development and adaptation. Early studies of psychiatric illness among adolescents in community samples indicated that between 10% and 20% of adolescents experience some form of psychiatric illness (Langer et al., 1974; Leslie, 1974; Gould et al., 1981; Offer et al., 1987).

Depression

Studies suggest that a several forms of psychotherapy can be helpful in treating clinical depression among adolescents (for reviews see Lewinsohn and Clarke, 1999; Moore and Carr, 2000a; Curry, 2001; Fonagy et al., 2002). Two approaches have received the largest amount of empirical interest and enjoy the strongest support: **CBT and interpersonal psychotherapy** for adolescents (also referred to as IPT-A). A substantial body of research indicates that CBT can be efficacious for treating depression among adolescents (Birmaher et al., 1996; Harrington et al., 1998; Lewinsohn and Clarke, 1999; Curry, 2001). Controlled outcome studies suggest that both individual and group CBT can be useful in alleviating dysphoria, and that gains may be maintained over time (Lewinsohn et al., 1990; Wood et al., 1996; Brent et al., 1997; Birmaher et al., 2000). A recent meta-analysis indicated that the effect sizes for CBT for depression among adolescents were moderate to large, and that gains were maintained for up to 2 years (Reinecke et al., 1998). The efficacy of CBT for depression during adolescence appears, then, to be well-established (Curry, 2001).

IPT, a form of psychotherapy developed by Gerald Klerman et al. (1984) for treating depressed adults, has been adapted for use with adolescents (Mufson et al., 1993). The approach focuses on addressing common interpersonal difficulties experienced by adolescents, including challenges associated with autonomy from parents, relationships with peers, and managing the loss of significant relationships. Explicit attempts are made to identify interpersonal factors that are associated with the etiology and maintenance of the depressive episode. Information is gathered about the nature and quality of the adolescent's relationships, their expectations for the relationships, whether these expectations are being met, goals for their relationships, and how they have attempted to accomplish these goals. Particular attention is given to separations and losses, conflict, changes in roles, interpersonal deficits (including social withdrawal or isolation, social skills deficits, and social anxiety), and difficulties encountered in single family homes. Active attempts are then made to address difficulties identified in these domains.

Anxiety

Several protocols have been developed for treating child and adolescent anxiety disorders. Controlled outcome studies completed over the past 15 years indicate that **behavioral psychotherapy** and **CBT** can be useful in treating generalized anxiety, school anxiety, specific phobias, panic, and obsessive-compulsive disorder among youth (for reviews see Ollendick and King, 1998; Moore and Carr, 2000b; Barrett, 2001; Piacentini et al., 2003).

Based upon cognitive and behavioral models, these approaches endeavor to alleviate anxiety by teaching children and adolescents to monitor their moods, anticipate situations in which they are likely to become anxious, identify specific distressing thoughts, and respond to these cues by actively using cognitive and behavioral coping strategies. Exposure and desensitization, relaxation training, guided imagery, rehearsal of adaptive self-statements and encouragement of adaptive coping attempts are frequently used. Parent and family sessions are typically included in these treatment programs, both to address parental behaviors that may be maintaining the child's anxiety and to provide them with strategies for managing their child's anxiety at home. Cognitive strategies (which focus upon reducing cognitive distortions, developing coping skills, and enhancing perceptions of control or efficacy) and behavioral approaches (which emphasize desensitization to anxiety-provoking stimuli and operant reinforcement of adaptive coping) are typically used together (Kendall et al., 1992).

Conduct disorder

Conduct problems, including aggressive behavior, disobedience and defiance at home and at school, and major rule violations, are among the most persistent and difficult to treat clinical problems in adolescence

(Eyberg et al., 1998). They are among the most common reasons for clinical referral, reflecting their high prevalence rates (Hinshaw and Anderson, 1996) and the fact that they can be quite distressing to parents and school officials. Traditionally, serious conduct problems have been treated with long-term, dynamically informed psychotherapy aimed at offsetting major ego deficits in the form of low frustration tolerance, limited self-awareness, impaired empathy, compromised interpersonal relations, or a fragmented, noncohesive sense of self. Often psychotherapy is only one component of a broader milieu treatment, either in a residential setting, group home, or therapeutic school. Such intensive treatment is viewed as necessary because of the breadth of impairment found among conduct disordered youth. Recent evidence, however, has suggested that aggregating conduct disordered youth in residential or group treatments can have unintended, deleterious effects (Dishion et al., 1999).

Three treatments have been developed for and evaluated with conduct disordered adolescents. The first is **anger control training** with stress inoculation (Feindler, 1991). At the core of this intervention is the view that youth with delinquent and aggressive problems have serious difficulties with the expression and regulation of anger. The treatment, then, principally aims at teaching youth a variety of coping strategies for reducing angry arousal. Therapy focuses on helping youth to identify anger provocation cues, to suppress immediate anger responses with self-instructions, to modulate arousal with relaxation or self-instructional techniques, and to consider consequences of aggressive behavior or explosive anger. In addition, a portion of the treatment is directed toward training individuals to behave in an assertive rather than an aggressive manner.

Among the most promising treatments for adolescent conduct disorder are **family-based therapies**. A growing body of research suggests that disrupted family relations, poor parental monitoring, inconsistent discipline, and cross-generational continuities may contribute to aggressive and disruptive behavior among youth (Hinshaw and Anderson, 1996). Based on these findings, family processes have been targeted for intervention. **Functional family therapy** draws heavily on social learning formulations of noncompliance and aggressive behavior. At the core of this intervention is the view that aggressive and disruptive behaviors are maintained through patterns of family interaction that unintentionally reinforce problem behaviors while failing to reward prosocial behaviors. Functional family therapy (Alexander and Parsons, 1982) attempts to modify such dysfunctional family patterns by altering parental monitoring and disciplinary strategies.

Because conduct disorder is multidetermined, emerging treatments increasingly emphasize comprehensive, multicomponent interventions that address multiple pathogenic processes at multiple levels of context. **Multisystemic therapy** (MST; Hengeler et al., 1998) is an integrative and comprehensive approach to treating youth conduct problems and antisocial behavior. Unlike traditional, comprehensive treatments that remove the adolescent from his or her social environment through placement in residential treatment settings, MST aims at restructuring multiple levels of the youth's environment in order to promote pro-social functioning. Drawing on Bronfenbrenner's (1979) ecological model of development, individual behavior is viewed within the context of multiple, nested contexts. Relevant context is not limited to the family, as in functional family therapy, but extended to the school, neighborhood, peer group, and broader community, as well as to linkages among these systems.

MST begins with the assumption that the purpose of assessment is to understand the fit between identified problems and the functioning of multiple systems. Psychiatric diagnosis is not the primary aim, instead MST therapists attempt to identify processes at multiple levels that support or impede adaptive functioning. In turn, therapeutic interventions attempt to use systemic strengths, for example, a committed extended family, as levers for change. All interventions are present focused and action oriented. Typically, many interventions focus on specific contingencies that sustain problematic behaviors. Therapist and family

agree upon specific, well-defined goals, and progress is closely monitoring, including family feedback on treatment fidelity.

Behavioral, emotional, and social difficulties experienced by adolescents can have pernicious effects that persist into adulthood. It is important, then, to include long-term assessments as a part of both clinical practice and research. We should, at the same time, attempt to insure that our interventions have broad, positive effects on adolescents development.

UNIT 12 – PSYCHOEDUCATION

Psychoeducation is a professionally delivered treatment modality that integrates and synergizes psychotherapeutic and educational interventions. Many forms of psychosocial intervention are based on traditional medical models designed to treat pathology, illness, liability, and dysfunction. In contrast, psychoeducation reflects a paradigm shift to a more holistic and competence-based approach, stressing health, collaboration, coping, and empowerment
(Dixon, 1999; Marsh, 1992). It is based on strengths and focused on the present. The patient/client and/or family are considered partners with the provider in treatment, on the premise that the more knowledgeable the care recipients and informal caregivers are, the more positive health-related outcomes will be for all. To prepare participants for this partnership, psychoeducational techniques are used to help remove barriers to comprehending and digesting complex and emotionally loaded information and to develop strategies to use the information in a proactive fashion.

Psychoeducation embraces several complementary theories and models of clinical practice. These include ecological systems theory, cognitive-behavioral theory, learning theory, group practice models, stress and coping models, social support models, and narrative approaches (Anderson, Reiss, & Hogarty, 1986; Lukens, Thorning,& Herman, 1999; McFarlane, Dixon, Lukens, & Lucksted, 2003).

Ecological systems theory provides the framework for assessing and helping people understand their illness or experience in relation to other systems in their lives (i.e., partners, family, school, health care provider, and policymakers). Under this umbrella, psychoeducation can be adapted for individuals, families, groups, or multiple family groups. Although psychoeducation can be practiced one-on-one, group practice models set the stage for within-group dialogue, social learning, expansion of support and cooperation, the potential for group reinforcement of positive change, and network building (Penninx et al., 1999). They reduce isolation and serve as a forum for both recognizing and normalizing experience and response patterns among participants, as well as holding professionals accountable for high standards of service.

Cognitive-behavioral techniques such as problem solving and role-play enhance the presentation of didactic material by allowing people to rehearse and review new information and skills in a safe setting. These can be amplified through specific attention to the development of stress management and other coping techniques (Anderson et al., 1986; McFarlane, 2002).

Narrative models, in which people are encouraged to recount their stories as related to the circumstances at hand, are used to help them recognize personal strengths and resources and generate possibilities for action and growth (White, 1989).

Recent mandates at both the federal and international levels have pushed to include psychoeducation as a focal point in treatment for schizophrenia and other mental illnesses, and are backed by national policymakers (President's New Freedom Commission on Mental Health, 2003) as well as influential family self-help groups such as the National Alliance for the Mentally Ill (NAMI).

In schizophrenia, any form of intervention is complicated by the symptoms of the illness, which include psychosis as well as functional and cognitive deficit or distortion, alogia, inertia, denial, and/or lack of awareness of illness (American Psychiatric Association,1994). Patients, formal care providers, and informal caregivers are confronted not only by the severe burden of the illness, but by the distorted sense of reality by which it is characterized. To address this multifaceted set of challenges, the various psychoeducational models for schizophrenia build on a series of principles that exemplify the paradigm shift to a strengths-based approach to intervention. Key aspects of these approaches include service coordination (i.e., easy access and clarity of expectation regarding service, medication management and adherence, and crisis planning), provision of relevant up-to-date information in a timely and flexible manner, attention to family conflict, communication, loss, problem solving, and attention to social as well as clinical needs for the person with illness, along with expanded social support for the family, through multiple family psychoeducation and family support groups (e.g., NAMI).

Psychoeducational approaches also are well established as adjunctive treatment for cancer, where patients and families are struggling with different forms of challenge. Although persons with cancer typically fall into the normal range in terms of psychological processes, they inevitably struggle with the anxiety and depression following the extraordinary stress associated with the diagnosis and treatment of the cancer(Cunningham,Wolbert,&Brockmeier,2000).

FAMILY PSYCHOEDUCATION

Family Psychoeducation (FPE) is an approach for partnering with consumers and families to treat serious mental illnesses. The term *psychoeducation* can be misleading. While FPE includes many working elements, it is not family therapy. Instead, it is nearly the opposite. In family therapy, the family itself is the object of treatment. But in the FPE approach, the illness is the object of treatment, not the family. The goal is that practitioners, consumers, and families work together to support recovery.

Serious mental illnesses such as schizophrenia, bipolar disorder, and major depression are widely accepted in the medical field as illnesses with well-established symptoms and treatment. As with other disorders such as diabetes or hypertension, it is both honest and useful to give people practical information about their mental illnesses, how common they are, and how they can manage them.

Research shows that the critical ingredients of effective FPE include the following (Dixon et al., 2001):

- Education about serious mental illnesses;
- Information resources, especially during periods of crises;
- Skills training and ongoing guidance about managing mental illnesses;
- Problem solving; and
- Social and emotional support.

The phases of Family Psychoeducation

FPE services are provided in three phases:

- Joining sessions;
- An educational workshop; and
- Ongoing FPE sessions.

Joining sessions

Initially, FPE practitioners meet with consumers and their respective family members in introductory meetings called joining sessions. The purpose of these sessions is to learn about their experiences with mental illnesses, their strengths and resources, and their goals for treatment.

Educational workshop

In the second phase of the FPE program, FPE practitioners offer a 1-day educational workshop. The workshop is based on a standardized educational curriculum to meet the distinct educational needs of family members.

Ongoing Family Psychoeducation sessions

After completing the joining sessions and 1-day workshop, FPE practitioners ask consumers and families to attend ongoing FPE sessions. When possible, practitioners offer ongoing FPE sessions in a multifamily group format. Consumers and families who attend multifamily groups benefit by connecting with others who have similar experiences.

FPE is based on research that shows that consumers and families who participated in the components of the evidence-based model had 20 to 50 percent fewer relapses and rehospitalizations than those who received standard individual services over 2 years (Penn & Mueser, 1996; Dixon & Lehman, 1995; Lam, Knipers, & Leff, 1993; Falloon et al., 1999). Those at the higher end of this range participated for more than 3 months.Studies also show that FPE improved family well-being (Dixon et al., 2001). Families reported a greater knowledge of serious mental illnesses; a decrease in feeling confused, stressed, and isolated; and reduced medical illnesses and use of medical care (Dyck, Hendryx, Short, Voss, & McFarlane, 2002). The greatest amount of research has shown benefits for consumers with schizophrenic disorders and their families (Dixon et al., 2001). Studies also show promising results for the following illnesses:

- **Bipolar disorder**—(Clarkin, Carpenter, Hull, Wilner, & Glick, 1998; Miklowitz & Goldstein, 1997; Moltz, 1993; Parikh et al., 1997; Miklowitz et al., 2000; Simoneau, Miklowitz, Richards, Saleem, & George, 1999);
- **Major depression**—(Simoneau et al., 1999; Emanuels-Zuurveen & Emmelkamp, 1997; Leff et al., 2000);
- **Obsessive-compulsive disorder**—(Van Noppen, 1999); and
- **Borderline personality disorder**—(Gunderson, Berkowitz, & Ruizsancho,, 1997).

Studies show that the effectiveness of FPE does not differ depending on consumers' age, gender, education-level, or severity of illness.

UNIT 14 – PSYCHOTHERAPY IN THE INDIAN CONTEXT

AYURVEDA

Ayurveda (Devanagari: आयुर्वेद) or ayurvedic medicine is a form of alternative medicine in use primarily in the Indian subcontinent. The word "Ayurveda" is a tatpurusha compound of *āyus* "life" and *veda* "knowledge", and would roughly translate as the "Science of Life". Ayurveda deals with the measures of healthy living, along with therapeutic measures that relate to physical, mental, social and spiritual harmony. Ayurveda is also one among the few traditional systems of medicine involving surgery. It is very important to note that the mainstream scientific community has produced almost no support for the claims of Ayurvedic practitioners.

Ayurveda was first described by Agnivesha, in his book *Agnivesh tantra*. The book was later revised by Charaka, and renamed to Charaka Samhitā.[*citation needed*] Other early texts of Ayurveda include the Charaka Samhitā and the Sushruta Samhitā, which served as textual material in the ancient Universities of Takshashila and Nalanda. [1]The texts are believed to have been written around the beginning of the Common Era, and is based on a holisticapproach rooted in earlier Vedic culture. Its conspicuous use of the word *veda*, or knowledge, reveals its role in early Hinduism and explains its popularity in India.

Ayurveda was practiced during the time of the time of the Buddha (around 520 BC) , and in this period the Ayurvedic practitioners were commonly using mercury-sulphur combination based medicines. In this period Mercury and sulphur and other metals were used in conjunction with herbs to prepare the different medications. An important Ayurvedic practicionner of this period was Nagarjuna, a Buddhist herbologist, famous for inventing various new drugs for the treatment of ailments. Nagarjuna was accompanied by Surananda, Nagbodhi, Yashodhana, Nityanatha, Govinda, Anantdev, Vagbhatta etc. The knowledge of Ayurveda progressed a lot during this period; and is therefore termed as the Golden Period of Ayurveda.

Ayurveda teaches two main principles:

- Preservation of health - how to maintain health and what to do to keep the human body healthy and fit to avoid sickeness
- Methods, medicine, and tactics for management of diseases and ailments to cure and procure a return to normal health.

YOGA

Yoga is a family of ancient spiritual practices that originated in India, where it remains a vibrant living tradition and is seen as a means to enlightenment. Karma Yoga, Bhakti Yoga, Jnana Yoga, and Raja Yoga are considered the four main yogas, but there are many other types. In other parts of the world where yoga is popular, notably the West, Yoga has become associated with the asanas (postures) of Hatha Yoga, which are popular as fitness exercises and also form the basis of an expanding business.
Yoga as a means to enlightenment is central to Hinduism, Buddhism, Jainism and has influenced other religious and spiritual practices throughout the world. Important Hindu texts establishing the basis for yoga include the Upanishads, Yoga Sutras of Patanjali, the Bhagavad Gita, and the Hatha Yoga Pradipika

Modern yoga practice often includes traditional elements inherited from eastern religion, such as moral and ethical principles, postures designed to keep the body fit, spiritual philosophy, instruction by a guru, chanting of mantras (sacred syllables), pranayama (breathing exercises), and stilling the mind through meditation. These elements are sometimes adapted to meet the needs of non-Hindu practitioners, who may be attracted to yoga by its utility as a relaxation technique or as a way to keep fit.
Proponents of yoga see daily practice as beneficial in itself, leading to improved health, emotional well-being, mental clarity, and joy in living. Yoga advocates progress toward the experience of samadhi, an advanced state of meditation where there is absorption in inner ecstasy.

The ultimate goal of yoga is the attainment of liberation (Moksha) from worldly suffering and the cycle of birth and death (Samsara). Yoga entails mastery over the body, mind, and emotional self, and transcendence of desire. It is said to lead gradually to knowledge of the true nature of reality. The Yogi reaches an enlightened state where there is a cessation of thought and an experience of blissful union. This union may be of the individual soul (Atman) with the supreme Reality (Brahman), as in Vedanta philosophy; or with a specific god or goddess, as in theistic forms of Hinduism and some forms of Buddhism. Enlightenment may also be described as extinction of the limited ego, and direct and lasting perception of the non-dual nature of the universe. For the average person still far from enlightenment, yoga can be a way of increasing one's spiritual awareness, or cultivating compassion and insight. While the history of yoga strongly connects it with Hinduism, proponents claim that yoga is not a religion itself, but contains practical steps which can benefit people of all religions, as well as those who do not consider themselves religious.

The Bhagavad Gita famously distinguishes several types of "yoga", corresponding to the duties of different nature of people. Capturing the essence and at the same time going into detail about the various Yogas and their philosophies, it constantly refers to itself as such, the "Scripture of Yoga" (see the final verses

of each chapter). The book is thought to have been written some time between the 5th and the 2nd century BC. In it, Krishna describes the following yogas:

(1) *Karma yoga*, the yoga of "action" in the world.
(2) *Jnana yoga*, the yoga of knowledge and intellectual endeavor.
(3) *Bhakti yoga*, the yoga of devotion to a deity.
(4) *Raja yoga*, the yoga of meditation